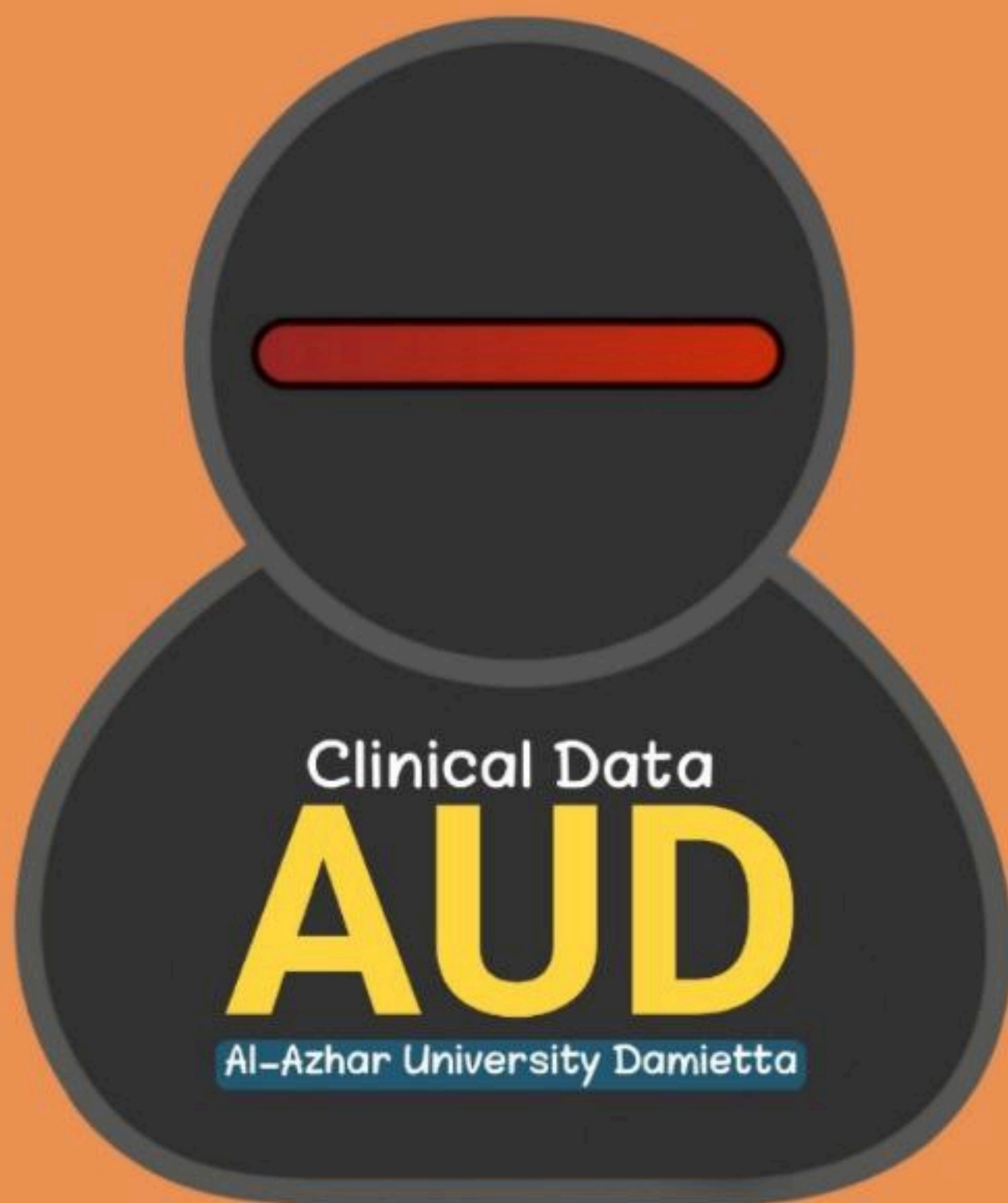




Telegram: @AUData_bot

https://t.me/AUData_bot

LAST UPDATE

THEORETICAL REVISION

Dr. M. Allam

Theoretical revision in 25 days

Telegram: @AUNData_bot

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Cardiology 1

(IHD & Rheumatic fever)

- 1- DD of acute chest pain
- 2- Early complications of acute MI
- 3- Stable angina (management) – chronic stable angina
- 4- Management of acute ST. segment elevation acute myocardial infarction
- 5- Management of acute myocardial infarction
- 6- Variants of angina pain
- 7- Give a comment on therapy in acute MI
- 8- Angina (types, investigations, TTT, C/P)
- 9- IHD (enumerate predisposing factor)
- 10-Short account on cardiac pain
- 11-Diagnosis of rheumatic fever
- 12-IHD (3 clinical presentations)
- 13-Pseudo angina (DD of 3 causes)
- 14-Coronary anatomy
- 15-Unstable angina
- 16-Enumerate predisposing factor of IHD
- 17-Discuss acute coronary syndrome
- 18-Discuss thrombolytic therapy in details
- 19-Rheumatic fever (investigations & management)
- 20- (C/P & TTT) of acute MI

MCQ

1- Drug of choice in IHD:

- a) B blocker b) Nitrate c) Ca++ channel blocker

2- Accelerated atherosclerosis occurs in:

- a) Pan-hypopituitarism b) DM c) DI

3- Cardiac scan in angina is done by:

- a) Thallium b) Technetium c) Both

4- Cold spot in cardiac scan is seen in:

- a) Angina b) MI c) Both

5- What is the first of the following elevated in MI:

- a) Troponin b) Myoglobin c) Lactose

6- Pathological Q wave in ECG indicate:

- a) Unstable angina b) Trans mural infarction c) Sub endocardial MI

7- Which enzyme rises earliest in MI:

- a) SGP T b) LDH
c) SGOT d) CPK

8- All of the following may be caused by MI except:

- a) Arrhythmia b) Shock
c) Aortic incompetence d) Mitral incompetence

9- Early complication of acute MI includes the following except:

- a) Shock b) Pericarditis
c) Embolization d) None

10- Which of the following are major criteria for the diagnosis of rheumatic fever?

- a) Fever b) Raised ESR
c) Poly arthritis d) Erythema margination

11- Complication of RF includes:

- a) Endo carditis b) Chorea
c) Joint deformity d) all of the above

12- Which is not included in minor manifestation of joints criteria in rheumatic fever?

- a) Prolonged PR interval b) Arthralgia
c) Increased ESR d) elevated ASO titre
e) Previous attack of RF

13- Which is not major manifestation of joints criteria in RF?

- a) Chorea b) erythema nodosum
c) Subcutaneous nodules d) Erythema marginatum

14- Diagnosis of AMI within 6 hours depends on:

- a) CPK MB2/CPK.MB ≥ 1.5
- b) increased LDH3
- c) Rise of SGPT >250 IV/L
- d) inverted T wave in ECG

15- Retrosternal chest pain classically occurs in all except:

- a) Acute mediastinitis
- b) Dissecting aneurysm
- c) Bornhelur disease
- d) unstable angina
- e) Esophageal spasm

16- CPK-MB is increased in all except:

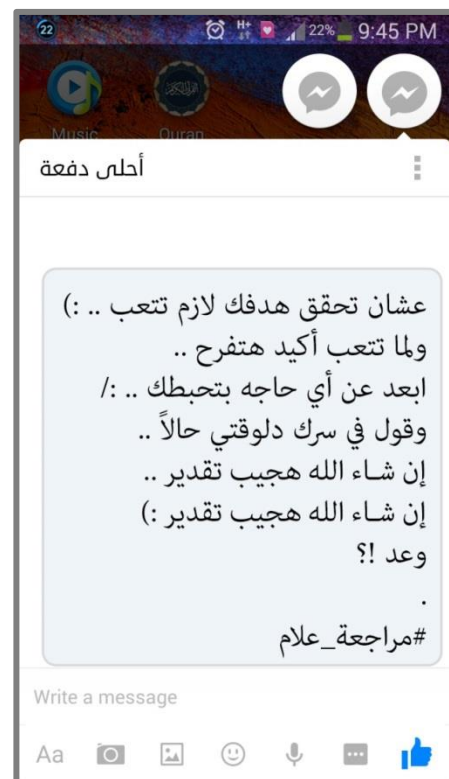
- a) Myocarditis
- b) Rhabdomyolysis
- c) Post-AMI
- d) Post-electrical cardioversion
- e) Unstable angina

17- Which enzyme rises earliest in AMI:

- a) SGPT
- b) LDH
- c) SGOT
- d) CPK
- e) GGT

18- Commonest heart valve abnormality revealed after AMI is:

- a) AI
- b) MI
- c) AS
- d) MS
- e) Ebstein anomaly



Cases

- 1- An obese man 56 y. old presented with a syncopal attack associated with sweating. Results of laboratory investigations were: Hb 14.5 gm, WBCs 12000/cmm, ESR 32 at first hour, SGOT 65 (<25), SGPT 18 (<20).
 - a) *What diagnosis is suggested by these results?*
 - b) *What further investigations would confirm your diagnosis?*
 - c) *Discuss in short the lines management*

- 2- A 12 y. old child presented with fever and joint pains of two weeks duration. Examination revealed that his temp. is 37.5 °C, pulse 110/min. & the tonsils are enlarged and congested. Cardiac auscultation revealed faint heart sounds, and a soft systolic murmur over the apex. ECG revealed prolonged P-R interval.

- 3- A 52 y. old obese & smoker male is under ttt for hypertension presented to the RR with marked upper epigastric pain radiating to the chest & neck, together with palpitation and sense of being about die. This occurred two hours after a heavy meal. Examination revealed an anxious facies, pallor & sweating. The BP is 160/100, and the pulse is irregular

- 4- A 70 y. old man presented with acute pulmonary oedema. BP was 170/100 and ECG showed elevated ST segment in V1 to V6.
 - a) *What is the diagnosis of this case?*
 - b) *What are the immediate lines of management?*
 - c) *What is the diagnostic work up needed to investigate this condition?*

- 5- A 54 y. old man presented to the reception room of a general hospital because severe pain across the chest that lasted for 40 min. clinical examination revealed no abnormalities. ECG was also normal.
 - a) *What advice would you give to the patient?*
 - b) *What particular signs would you look for if you were asked to follow up? Explain the significance of each?*
 - c) *Select the most relevant 3 investigations that you would like to order giving reasons?*

- 6- A man aged 52 y. old was brought to hospital within one hour of having severe chest pain. After doing an ECG, he was admitted to the CCU. A blood sample was sent to the laboratory & proper ttt was started. Two hours later the patient felt sudden severe palpitations. Few minutes later he became severely dyspnic & cyanosed with audible chest bubbling.**
- a) Describe the possible ECG finding that urged admission to the ICU.*
 - b) Choose 3 invest. To be ordered in the laboratory request sent with the bl. Sample, in order to prove the diagnosis & give their possible results.*
 - c) Outline the schedule of treatment that he received in the ICU.*
 - d) What possibly happened two hours later & how would you manage?*
- 7- A 56 y. old male was admitted to the CCU because of severe agonising chest pain that lasted for 30 minutes, BP was 180/100, and pulse was 100/mm with infrequent ectopics.**
- a) Mention 3 diagnostic possibilities.*
 - b) How can you differentiate between them?*
 - c) What treatment would each condition need?*
- 8- 56 y. old, business man, heavy smoker, presented by chest pain of 5 min. duration, which is retrosternal and also have a pressure sensation. High of patient is 173 cm & weight 120 kg.**
- a) What is the diagnosis?*
 - b) What are the investigations to confirm diagnosis?*
 - c) What is the treatment of this case?*
 - d) What are the other predisposing treatments?*
- 9- Male patient presenting to emergency room by chest pain for 5 min. duration, the pain is compressing & retrosternal radiating to his jaw, investigations revealed ST-segment elevation in leads II & III avF**
- a) What is the diagnosis?*
 - b) What are the initial investigations to confirm diagnosis?*
 - c) What are the causes of acute chest pain?*
 - d) Enumerate causes of ST-segment elevation*
-

Oral

- 1) Treatment of MI**
- 2) Investigation of MI**
- 3) Predisposing factor of IHD**
- 4) Atherosclerosis**

Answer

MCQ

- 1- B
- 2- B
- 3- C
- 4- C
- 5- B
- 6- B
- 7- D
- 8- C
- 9- C
- 10- C / D
- 11- A
- 12- D
- 13- B
- 14- A
- 15- C
- 16- B
- 17- D
- 18- B

Cases

- 1- Acute myocardial infarction
- 2- Most probably a case of rheumatic fever
- 3- Most probably a case of inferior wall myocardial infarction
- 4- Acute extensive transmural myocardial infarction complicated by acute pul. Edema
- 5- D.D. of acute chest pain
- 6- Myocardial infarction complicated by pulmonary edema
- 7- Acute chest pain (old male hypertensive):
 - Dissecting aortic aneurysm
 - M.I.
 - Pericarditis.
- 8- Angina pectoris
- 9- Inferior wall MI

Cardiology 2

Infective endocarditis, heart failure

APO- symptoms of CVS

- 1- Acute Lt. sided heart failure (causes & treatment)
- 2- Clinical picture & treatment of Lt. sided heart failure
- 3- Rt. Sided heart failure (causes , clinical picture, management)
- 4- Pathogenesis of dyspnea Lt. sided HF
- 5- Discuss treatment of heart failure
- 6- Refractory HF
- 7- Short account on digoxin
- 8- Digitalis (action – mechanism of action – indication – cont- side effects, toxicity)
- 9- Diuretics (indication & side effects)
- 10- Account on pathogenesis & clinical picture of sub-acute infective endocarditis
- 11-CCB (complication)
- 12- Syncope (definition, DD of 3 causes)
- 13-Discuss causes of central cyanosis
- 14-Give account on clinical picture & management of pulmonary edema

MCQ

1- B-blocker are contraindicated in (recently indicated in HF)

- a) HTN b) HF c) Obstructive

2- Proto diastolic gallop may present in all of the following except:

- a) HF b) AR
c) VSD d) PH

3- Rt. Vent hypertrophy criteria include the following except:

- a) Epigastric pulsation b) apex is localized
c) it has parasternal heave d) precordial bulge

4- All of the following are causes of pulsus paradoxus except:

- a) Severe BA b) cardiac tamponade
c) AR d) advanced Rt. Sided HF

5- Which is false regarding edema in congestive HF?

- a) initially noticed in the morning b) starts in the dependent part
c) pitting edema d) sacral edema in non-ambulatory patients

6- Recent drugs are used in ttt of CHF except:

- a) digoxin b) ACE inhibitor
c) BBS d) diuretics

7- Signs of LV.H

- a) heaving apex b) Epigastric pulsation c) TR

8- Signs of Lt. ventricular hypertrophy include all of the following except:

- a) Apex is diffuse b) Apex is localized
c) heaving apex d) +ve rocking

9- L.V.H. is characterized by the following except:

- a) diffuse apex b) heaving apex c) shifting apex

10- Pulsus alternans is produced by:

- a) Pericardial effusion b) Lt HF
c) COPD d) pulmonary embolism

11- Orthopnea is caused by:

- a) Rt. Sided HF b) Lt. sided HF c) Neither

12- Drug used in treatment of acute Lt. ventric Failure include:

- a) Verapamil b) IV Lasix c) Propranolol

13- Refractory heart failure may result from:

- a) Hyperthyroidism b) massive diuresis c) both

14- The following statement about infective endocardiac are correct except:

- a) The commonest organism is streptococcus haemolyticus
- b) Prolonged fever
- c) Causes more damage to heart valves

15- Infective endocarditis is suspected in cardiac patient:

- a) Haematuria
- b) Palpitation
- c) Neither

16- Incidence of infective endocarditis is least in:

- a) MI
- b) PDA
- c) ASD
- d) VSD

17- Commonest sites of infective endocarditis are all of the following except:

- a) VSD
- b) Prosthetic valve
- c) ASD
- d) PDA

18- Which of the following is the most common cause of injection drug associated infective endocarditis:

- a) Streptococcus (group A)
- b) Staphylococcus
- c) Streptococcus Faecalis
- d) Streptococcus viriden

19- Most common pathogen in infective endocarditis:

- a) Pneumococcus
- b) Staphylococcus
- c) Streptococcus Faecalis
- d) Streptococcus viriden

20- Osler's nodules are common with:

- a) Infective endocarditis
- b) Mitral stenosis
- c) TR
- d) Pericardial effusion

21- Signs of Infective endocarditis:

- a) Erythema marginatum
- b) Arthritis
- c) Splenomegaly

22- Central cyanosis is not present in:

- a) APO
- b) Fallot tetralogy
- c) Lt. to Rt. Shunt
- d) TGA

23- The clinical Picture of pulmonary edema:

- a) Dyspnea
- b) Cough with frothy sputum
- c) bradypnea
- d) all of the above

24- The 1st symptom of digitalis toxicity

- a) Yellow vision
- b) Extra systole
- c) Gynecomastia
- d) anorexia

25- Pulsus bigemini is caused by:

- a) PDA
- b) AS & AR
- c) Digitalis overdose
- d) Constrictive pericarditis

[illegible]

a) Pericardial effusion
b) Lt. sided HF
c) COPD
d) Pulmonary thromboembolism

a) Lt. ventricular failure
b) Rt. Ventricular failure
c) Pulmonary stenosis
d) Lt. to Rt. Shunt
e) Marfan's syndrome

a) Diuretics
b) Trendelburg position
c) Morphine
d) Rotating tourniquets
e) Vasodilator

a) Old age
b) Hypokalemia
c) Renal failure
d) Hepatic encephalopathy
e) Quindine

a) AS b) AI
c) MS d) MI
e) VSD

a) Murmur
b) Osler's nodule
c) Clubbing
d) Splenomegaly
e) Kussmaul's sign

a) Spironolactone
b) Carvedilol
c) Propranolol
d) Digoxin
e) Corptopril

Cases

- 1- 77 y. old man complains of several weeks worsening extensional dyspnea & angina like chest pain & exercise & near syncope, on examination signs of volume overload and late systolic murmur radiated to the neck.
 - a) *What is the diagnosis?*
 - b) *Steps to confirm diagnosis*
 - c) *TTT you suggest*

- 2- 14 y. old girl & known rheumatic mitral valve disease presented & fever
 - a) *DD*
 - b) *Discuss one of them*

- 3- 21 y. old man & known rheumatic valvular disease presented & fever, by examination there is change in the severity of murmur
 - a) *What is the diagnosis?*
 - b) *Other sign to confirm diagnosis*
 - c) *Investigations & complications*

- 4- A 28 y. old manual worker presented with a sense of fever & marked general fatigue in the last 3 weeks. Examination revealed pallor & a toxic look, his temp. 37.8°C, BP 150/50, pulse is 100/min & he has clubbing of fingers. Urine examination revealed many red cells & one plus albumin.
 - a) *What is the diagnosis?*
 - b) *Other sign to confirm diagnosis*
 - c) *Investigations & complications*

- 5- A 26 y. old man complained of general weakness for 3-4 weeks, he sometimes feels himself feverish; he may have lost few kilograms of weight over his period with depressed appetite. On few occasions he had some sweating at night. On E, the pulse was 70 minutes, his BP 110/60 there is an early diastolic murmur heard along the Lt. sternal border. The spleen was palpable 2 cm below the left costal margin.
 - a) *What is the most probable diagnosis?*
 - b) *What do you expect to find in ECG, X-ray chest and urine examination?*
 - c) *How to prove the diagnosis & what is the immediate management?*

6- A 25 y. old female patient known to have a cardiac problem presented with dyspnea (grade IV) & fever for 2 weeks. OIE, she was febrile 38.5°C and pale. Pulse was 110 beats/mm. and her fingers showed second degree clubbing. A soft pain systolic murmur was heard on the cardiac apex

- a) What is your diagnosis?*
- b) Mention 3 important investigations to prove your diagnosis?*
- c) Mention line of ttt?*

7- A 23 y. old male presented with general weakness dyspnea, bone aches & malaise. He gave a past history of palpitation, which was found to be a result of a cardiac valvular lesion. General exam. Revealed that the pt. was pale & toxic, BP was 140/50, temp. was 39°C & his pulse was 118/minute.

- a) What is your diagnosis?*
 - b) What are the required investigations to reach a final diagnosis?*
 - c) How would you treat such patient?*
-

Oral

- **Dyspnea (acute – paroxysmal – PND)**
- **Causes of fever in cardiac patients**
- **Causes of jaundice in cardiac patients**
- **Syncope (definition & causes)**
- **haemoptysis**

Answer

MCQ

1-

1- B	2- D	3- B	4- C	5- A	6- C	7- A	8- A	9- A
10- B	11- B	12- B	13- A	14- A	15- A	16- C	17- C	18- B
19- D	20- A	21- C	22- C	23- D	24- D	25- C	26- A	27- B
28- A	29- B	30- E	31- C	32- A	33- C			

Cases

- 1- AS complicated by HF
- 2- a) Rheumatic activity
b) Infective endocarditis
c) Chest infection
- 3- Rheumatic valvular disease complicated by infective endocarditis
- 4- Most probably a case of infective endocarditis
- 5- Most probably a case of infective endocarditis
- 6- Infective endocarditis
- 7- Most probably a case of infective endocarditis

وڌي ڪل (قمة)
شيء يقول لڪ (قم)
#مراجعة_علام

Cardiology 3

(S. hypertension – P.H. – Valvular disease – Pericardial disease)

- 1) Essential HTN (causes & complications)
- 2) 2ry HTN (causes & management)
- 3) TTT of HTN
- 4) Systemic HTN (causes & complication)
- 5) Treatment of essential HTN
- 6) Hypertensive encephalopathy (CP & TTT)
- 7) Treatment of HTN associated with other medical disease
- 8) Discuss new classifications of grades of HTN with emphasis on TTT in brief
- 9) Systemic HTN (staging, target organ damage)
- 10) In severe HTN explain:
 - a) Acute severe dyspnea
 - b) Central cyanosis
 - c) Convulsion & impaired conc.
 - d) Gradual impairment of renal function
- 11) Short account on captopril
- 12) Indications of BB
- 13) Symptoms & signs of AR
- 14) Discuss physical signs of mitral valve diseases
- 15) DD of diastolic murmur over apex
- 16) DD of ejection systolic murmur over precordium
- 17) Mitral incompetence (hemodynamics & CP)
- 18) DD of diastolic murmur over the heart
- 19) DD & causes of systolic murmur on the apex
- 20) MS (hemodynamics, CP, complication)
- 21) AR (etiology, CP, physical signs)
- 22) Mitral valve prolapse
- 23) Pulmonary HTN (causes & CP)
- 24) Give an account on acute pericarditis
- 25) Pericardial effusion (causes & CP)
- 26) Acute pericarditis (etiology, CP, TTT)
- 27) Constrictive pericarditis (AE, CP, complication, management)

MCQ

1- Malar rash is presented in:

- a) MS b) AR c) AS

2- Signs of MS include except:

- a) Mid diastolic rumbling murmur
b) Opening snap
c) Weak first heart sound
d) Dilated lt. atrium

3- Sign of MS

- a) LVH b) Opening snap c) Pallor

4- The early manifestation of MS:

- a) AF b) exertion dyspnea c) TR

5- Functional MS occurs in:

- a) AS b) AR
c) PS d) PR

6- Causes of LVH include except:

- a) AS b) VSD
c) MS d) systemic HTN

7- MS causes

- a) LVH
b) Rt. ventricular hypertrophy
c) Both
d) None

8- Ortner's syndrome occurs in:

- a) MS b) AS
c) TR d) TS

9- Mid diastolic rumbling murmur is seen in:

- a) MS b) TS c) both

10-Systolic murmur is heard in the following except:

- a) MR
c) PS
- b) TR
d) MS

11-Absence of pre systolic accentuation of rumbling murmurs of MS occurs in:

- a) PH b) Rt. VH
c) Double mitral d) AF

12-MS without murmur may occur in the following except:

- a) LSHF lt. sided heart failure
b) sever PH
c) Rt. Sided heart failure
d) with chest infection

13-Systolic murmur is heard in the following except

- a) MR b) TR
c) PS d) MS

14-Increased pulse volume occurs in the following condition:

- a) Aortic incompetence b) mitral incompetence c) ASD

15-The commonest heart disease associated with Marfan's syndrome:

- a) AI
c) MVP
- b) VSD
d) Aortic coarctation

16-A2 in aortic stenosis is characteristically:

- a) diminished b) ringing in character
c) normal in character d) accentuated

17-Ejection click may be heard in:

- a) HOCM b) Fallot's tetralogy
c) Valvular AS d) Heart failure

18-In aortic stenosis there is:

- a) Apical diastolic thrill b) Hyper dynamic apex c) Basal systolic thrill

19- All of the following produce syncope except:

- a) Cardiac tamponade
b) Adam-stokes syndrome
c) Tight AS
d) Arrhythmia

20- Causes of LVH:

- a) AS b) MS c) Emphysema

21- Giant a wave is seen in the following except:

- a) Pulmonary HTN
b) Pulmonary stenosis
c) AS
d) Rt. Ventricular hypertrophy

22- Propagated systolic murmur over mitral area occurs in:

- A) AS b) ASD c) PS

23- Vigorous arterial pulsation in the neck occurs in:

- a) Aortic stenosis

24- In aortic incompetence the following is true except:

- a) Lit ventricular hypertrophy b) Collapsing pulse c) Loud S2 in aortic area

25- Peripheral signs of aortic regurg include all except:

- a) Water hammer pulse
b) Pistol shot
c) Low BP in LL
d) Capillary pulsation

26- Accentuated S2 in AI occurs in:

- a) Rheumatic b) Congenital
c) Calcific d) Syphilitic

27- Mid diastolic rumbling murmur is seen in:

- a) MS b) TS c) Both

28- Commonest cause of TR is:

- a) Rheumatic fever
- b) Congenital
- c) Functional dilation of value ring

29- Cause of pulsus paradoxus are all of the following except

- a) Pericardial effusion
- b) Adhesive pericarditis
- c) Constrictive pericarditis
- d) Chylo-pericardium

30- Which is true about pulsus paradoxus:

- a) The pulse rate $\uparrow$ $\hat{=}$ inspiration
- b) The pulse rate $\downarrow$ $\hat{=}$ inspiration
- c) Caused by sever AR
- d) Caused by essential HTN
- e) None of the above

31- Peri cardial rub is best audible in all except:

- a) By pressing the chest piece of the stethoscope
- b) After holding breath
- c) On the side of lower sternum
- d) In lying down position

32- The commonest cause of constrictive pericarditis:

- a) Rheumatic
- b) Viral
- c) Rheumatoid
- d) TB

33- All of the following produce systemic HTN except:

- a) Polycystic kidney
- b) Pheochromocytoma
- c) Addison's disease
- d) Conn's syndrome

34- HTN occurs with:

- a) Low Na⁺ intake
- b) Low K intake
- c) High Na intake
- d) All of the above

35- HTN $\hat{=}$ diabetes is best treated by:

- a) B-blocker
- b) Thiazide
- c) ACE inhibitor
- d) CCB

36- Anti-hypertensive drug $\hat{=}$ lupus like symptoms:

- a) Rami pril
- b) Mino xidil
- c) Hydralazine
- d) Thiazide

37- Anti-hypertensive drug contra indicated in renal artery stenosis:

- a) Losantan
- b) Verapamil
- c) Propronalol
- d) Spironolactone

38- In pulmonary HTN the following is true except:

- a) Loud second pulse
- b) Opening snap
- c) Prominent a- wave

39- All of the following are present in PH except:

- a) Ejection systolic murmur
- b) Weak S1
- c) Accentuated S2
- d) early diastolic murmur

40- All of the following are true as regard primary PH except:

- a) Common in female
- b) Resposed to sildenafil
- c) Treated by ACEI
- d) Common after menopause

41- All of the following are sources of recurrent systemic embolism except:

- a) Left ventricular aneurysm
- b) SBE
- c) Tricuspid in competence & occasional ectopies
- d) L.T atrial myxoma

42- Which of the following anti-hypertensive drug is contra indicated in a hypertensive patient & pheochromocytoma

- a) Labetalol
- b) Prazocin
- c) Phenoxybenzamine
- d) guanethidine

43- Patient & mitral valve prolapse has the risk for:

- a) Sever MR
- b) Myocardial
- c) Pulmonary embolism
- d) PH
- e) Wall motion abnormality

44- The least common complication MS is:

- a) Cerebral thrombosis
- b) SBE
- c) Pulmonary hypertension
- d) AF
- e) Haemoptysis

45-Seagull murmur is a feature of:

- a) acute MI
- b) acute rheumatic fever
- c) SBE
- d) Floppy mitral valve
- e) Prothetic valve endocarditis

N.B.

*Prazocin **can't** pass BBB, but phenoxybenzamine **can** pass*

Cases

- 1- Male patient with known heart disease presented with irregular irregularity of the pulse, this patient has history of dyspnea & recurrent chest infection.**
 - 1- What is the diagnosis?*
 - 2- What are the investigations?*
 - 3- What is your treatment?*
- 2- A 42 y. old man with thoracic malignancy & history of radio therapy. Now presented with dyspnea, jugular venous distension distal cardiac sound, pulsus paradoxus & cardiac enlargement.**
 - 1- What is the diagnosis?*
 - 2- What is the next therapeutic step?*
 - 3- Complications?*
- 3- Old man, uncontrolled HTN, presented with acute chest pain radiating to the back, his BP was not equal on both sides**
 - 1- What is the diagnosis?*
 - 2- Investigations*
- 4- 27 y. old woman presented with sharp chest pain which worsened by inspiration & improved by bending forward. Pain is precipitated by breathing deeply or coughing & is relieved by holding the breath. Heart examination reveals coarse, scratchy sound heard in both systole & diastole. ECG reveals elevated ST segment diffusely & PR segment depression**
 - 1- What is the diagnosis?*
 - 2- What are causes of such diagnosis?*
 - 3- What are investigations?*
 - 4- Mention treatment?*
- 5- During evaluation of hypertension, a 54 y. old woman was found to have a BUN of 142 mg/dl and serum creatinine 3.2 mg/dl. She had no known history of renal disease or urinary infection. She has chronic headache. Her physical examination was normal. Her haemoglobin was 9.7 g/dl and creatinine clearance was 20 ml/min.**
 - 1- What is your diagnosis?*
 - 2- What do you expect to find the kidney size by ultrasonography?*
 - 3- What type of renal parenchymal disease (glomerular, interstitial or vascular) do you suspect?*
 - 4- How to manage this case?*

6- You were called to see a 36 y. old hypertensive woman on irregular hypertensive therapy who developed sudden inability to move her left upper and lower limbs. O/E you found the BP 130/100 & she stated that she didn't receive her antihypertensive ttt for several weeks. Her pulse showed an irregular irregularity – 5 days later she complained of severe pain in the Rt. Leg, the foot became swollen pale & quite cold. 4 days later she developed diarrhea & passed about 10 motions of soft, slimy, black stools.

- 1- What particular signs would you look for in examination of the nervous and cardiovascular systems when you first see her? Explain their significance.*
- 2- What particular bedside investigation would you order? Why?*
- 3- What is the explanation of the features, which developed one after the other?*
- 4- What ttt would you advise? How would you monitor your treatment?*

7- A 15 y. old developed fever with cough and chest discomfort. He suddenly became severely dyspnic. Examination showed engorged neck veins with inspiratory filling, pulse of 120/min with a paradoxical character, BP 80/50 mm Hg and distant heart sounds.

- 1- What is the most likely diagnosis?*
- 2- Give 2 investigations to prove the diagnosis?*
- 3- Give 2 possible etiological conditions*
- 4- What emergency management do you suggest?*

8- A 14 y. old boy presented by his mother complaining of increasing fatigability and dyspnea on exertion in the last year. Examination revealed pulse 110/min, paradoxus character, neck veins are congested non-pulsating. Local examination of the heart revealed that the apex is neither visible nor palpable & there is protodiastolic gallop on apical auscultation.

9- A 50 y. old male presented with severe pain in the chest and upper abdomen for 1 hour. On examination, his pulse was absent in one arm and BP was 200/120 in the other arm. While investigations were running & ER management was offered. He developed hemiplegia and ECG showed non-specific changes that may or may not be ischemic. CPK was only mildly increased.

- 1- What is the diagnosis?*
- 2- What is differential diagnosis?*
- 3- What other investigations would you like to do that you expect to find?*

10- A 60 y. old male patient came to ER complaining from severe chest pain of 30 min. duration. BP taken from right arm was 190/90, but the pulse in the left side was much weaker than the right side. Cardiac examination revealed a murmur of aortic incompetence. The patient was admitted to the hospital. Two hours later the pain extended to the abdomen and the patient became oliguric.

- 1- *What is your diagnosis?*
- 2- *What is your differential diagnosis?*
- 3- *What other tests would you like to ask to reach a definitive diagnosis?*

Oral

- 1- Anti-hypertensive drugs
- 2- Causes of 2ry HTN
- 3- TTT of HTN with other diseases
- 4- Peripheral signs of AI

Answer:

MCQ

1- A	2- C	3- B	4- B	5- B	6- C	7- B
8- A	9- C	10- D	11- D	12- D	13- D	14- A
15- A	16- A	17- C	18- C	19- A	20- A	21- C
22- A	23- B	24- C	25- C	26- D	27- C	28- C
29- B	30- E	31- D	32- D	33- C	34- C	35- C
36- C	37- A	38- B	39- B	40- D	41- C	42- B
43- A	44- B	45- C				

Cases:

- 1- MS complicated by AF
- 2- Pericardial effusion, tamponade
- 3- Aortic dissection
- 4- Dry pericarditis
- 5- Hypertensive nephropathy or analgesic nephropathy
- 6- HTN complicated by AF and showers of emboli
- 7- Pericarditis → Pericardial effusion → Tamponade
- 8- Pericardial effusion
- 9- Dissecting aortic aneurysm
- 10- Case of dissecting aortic aneurysm

Cardiology 4

(Arrhythmias – Cardiac arrest – Myocarditis – Cardiomyopathy)

- 1) Brady-arrhythmias (types, diagnosis, TTT in brief)
- 2) Ventricular tachycardia arrhythmia
- 3) Regular tachycardia (DD of two main causes)
- 4) Supraventricular tachycardia (types, TTT)
- 5) Supraventricular tachycardia (give account)
- 6) Supraventricular tachycardia (Pathogenesis & Management)
- 7) AF (causes, diagnosis, TTT)
- 8) PSVT (Pathogenesis & Management)
- 9) DD of supraventricular tachycardia & management one of them
- 10) Irregular irregularity of pulse (causes, DD, TTT)
- 11) Account on AF
- 12) Causes, management of atrial fibrillation
- 13) Heart block
- 14) Account on complete heart block
- 15) Cardiac arrest
- 16) Cardiomyopathy
- 17) Myocarditis (causes, C/P, TTT)
- 18) Mention pathophysiology and TTT of hypertrophic cardiomyopathy
- 19) Dilated cardiomyopathy (causes, C/P, TTT)
- 20) Cardiac tamponade (discuss clinical manifestations)



MCQ:

1- Ventricular fibrillation is best treated by:

- a) IV amiodarone
- b) Carotid massage
- c) IV lignocaine
- d) electrical cardioversion

2- Which isn't a cause of sinus bradycardia:

- a) Myxedema
- b) Complete HB
- c) Hypothermia
- d) Obs. Jaundice

3- Wenckbach's phenomena occurs in:

- a) 1st degree HB
- b) 2nd degree HB
- c) 3rd degree HB

4- PR interval in wenckenbach's phenomena:

- a) Constant
- b) Shortened
- c) Prolonged
- d) Progressively prolonged

5- All of the following may present in AF except:

- a) Giant A wave
- b) Irregular irregularity
- c) Pulse deficit >10
- d) Absent P wave in ECG

6- Dropped beat occurs in:

- a) 1st degree HB
- b) 2nd degree HB
- c) 3rd degree HB

7- In complete heart block neck vein show:

- a) Cannon A wave
- b) R waves
- c) Both

8- Which doesn't produce regularly irregular pulse:

- a) 2nd degree heart block
- b) Atrial fibrillation
- c) Extra systole
- d) Sinus arrhythmia
- e) Atrial tachycardia with block

9- Sudden death may occur in:

- a) AS
- b) ASD
- c) Constrictive pericarditis
- d) PDA
- e) Floppy mitral valve

10- Cardiac arrest may be due to:

- a) Multiple ectopic
- b) Atrial flutter
- c) Pulseless ventricular tachycardia
- d) Wenckebach block
- e) Sinus brady cardia

Cases

1- 55 y. old man with rheumatic mitral stenosis presented with awareness of heart beats with were irregular and by examination. There were congested pulsating neck veins with absence of A wave.

- 1- *What is your diagnosis?*
- 2- *How to manage?*

2- A right handed male aged 22y, presented with sudden RT. Sided hemiplegia & inability speak at all. Cardiovascular examination revealed a double mitral lesion.

- 1- *What other signs & symptoms you look for?*
- 2- *What in your opinions the first ttt of choice?*

3- A 43 y. old female known to have rheumatic heart disease since long time. She suddenly felt syncope. On examination she was found to have hemiplegia. Her BP was 120/75 & pulse was 120 & irregular.

- 1- *What is the diagnosis?*
 - 2- *What are the investigations would you like to order?*
 - 3- *How would treat this lady?*
-

Oral

1- Differential between:

- AF & extra systole
- AF & atrial flutter

2- How to diagnose AF

3- Management of cardiac arrest

4- Causes of brady cardia

Answer:

MCQ

1- D	2- B	3- B	4- D	5- A
6- B	7- A	8- B	9- A	10-C

Cases:

- 1- AF
- 2- AF and showers of emboli
- 3- AF with shower of emboli

Keywords of cardiology cases

I- Murmurs

- 1) MS = mid diastolic localized to the apex**
- 2) MR = systolic & radiating to axilla**
- 3) AR = diastolic in 2nd aortic area**
- 4) AS = systolic radiating to carotid, apex + syncope**

II- Irregular pulse + stroke = AF

III- Ischemic pain > half hour = MI

**Ischemic pain < half hour = Angina
& topography is essential**

IV- Persistence ST elevation in patient with MI = myocardial aneurysm

V- Symptoms of low cop + muffled HS + low voltage ECG = pericardial effusion

VI- Chest pain & ST elevation in all leads = pericarditis

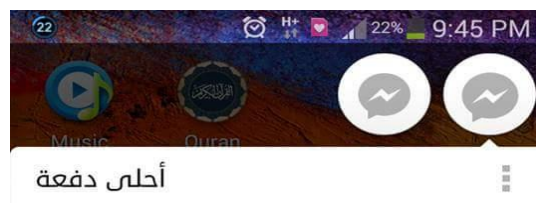
VII- Multi parus female + dyspnea & chest pain + loud S2 = Pulmonary HTN

VIII- Sever HTN + disturbed conscious level without lateralization = Hypertensive encephalopathy

IX- Anorexia – Nausea – vomiting & blurring of vision in pt ê HF = Digitalis toxicity

X- Fever + Arthritis of big joint + tic tac rhythm = RF

XI- Sudden sever chest pain radiating to back + dyspnea + murmur = Dissecting aortic aneurysm



إتحدى نفسك ولاعبها ماتش كورة
واكسبها ..
واوعى تقبل بالتعادل ..
اتحرك ..
وحقق هدفك (:
#مراجعة_علام

Write a message



Blood 1

(Introduction - ↓fe anemia – Megaloblastic – Aplastic a – Haemolytic a)

- 1- Give an account in normal blood picture
- 2- Anemia (Def- classify –investigation)
- 3- Clinical and lab diagnosis of iron deficiency anemia
- 4- Causes and clinical picture and diagnosis of megaloblastic anemia
- 5- Give an account of Vit B12 deficiency anemia
- 6- Folic acid deficiency & Vit b 12 def anemia
(blood film – single test to prove)
- 7- Macrocytic (pernicious) anemia
(clinical picture –blood picture – TTT)
- 8- Deficiency anemia (causes and discuss one of them)
- 9- Microcytic hypochromic anemia (DD & TTT of 2 important causes)
- 10-Sideroblastic anemia
(blood film – single test to prove)
- 11-Clinical and lab diagnosis of thalassemia
- 12-Discuss causes of hemolytic anemia
- 13-Clinical and biochemical manifestation of chronic hemolytic anemia
- 14-Discuss haemolytic crisis
- 15-DD of pancytopenia
- 16-Aplastic anemia
- 17-Describe CBC changes and finding in
 - b- anemia of chronic blood loss
 - c- pernicious anemia
- 18-Complication of blood transfusion
- 19-Non immune complication of blood transfusion

MCQ

1- Anemia of liver cell failure is

- a- microcytic hypochromic anemia
- b- normocytic normochromic anemia
- c- macrocytic
- d- All of the above

2- Auto- immune hemolitic anemia is characterized by all the following except

- a- +ve comb s test (direct – indirect)
- b- abnormal HB electrophoresis
- c- splenomegaly
- d- increase retics

3- Iron deficiency anemia may be caused by

- a- chronic blood loss
- b-ancylostoma
- c-malnutrition
- d- all of the above

4- Iron deficiency anemia may be caused by all the following except

- a- decrease intake
- b- chronic blood loss
- c-decrease folate
- d- pregnancy

5- In iron deficiency anemia there is

- a- high ferritin
- b- high TIBC
- c- both

6- In chronic iron deficiency anemia there is

- a- reticulocytosis
- b- high TIBC
- c- high serum ferritin

7- Which of the following is X- linked disease

- a- G6PD deficiency
- b-pyruvate kinase deficiency
- c-sickle cell anemia
- d-spherocytosis

8- Aplastic anemia is characterized by all except

- a- microcytic hypochromic anemia
- b- low reticulocyte count
- c-thrombocytopenia
- d- leukopenia

9- Sickle cell anemia is associated with

- a- high ESR
- b- diastolic murmur over precordium
- c- high serum iron
- d- fish mouth vertebra

10- Splenectomy is indicated in the following except

- a- hypersplenism
- b- spherocytosis
- c- sickle cell anemia
- d- ITP

11- Feature of sickle cell anemia include the following except

- a- leg ulcer
- b- aseptic bone necrosis
- c- huge spleen

12- All of the following is pancytopenia causes except

- a- megaloblastic
- b- aplastic
- c- hypersplenism
- d- sideroblastic anemia

13- Sign of vit b 12 deficiency anemia

- a- peripheral neuropathy b- tremors c- diarrhea

14- In pernicious anemia there is

- a- deficient intrinsic factor b- high vit b 12
c- abnormal BM picture d- all of the above
e- none of the above

15- Senile neutrophil < segmental > are commonly seen in

- a- spherocytosis b- megaloblastic
c- lymphoma d- acute appendicitis

16- In TTT of megaloblastic anemia

- a- folic acid should be given before vit b 12 b- vit b 12 should be given before folic acid
c- given together d- oral iron should be given at first

17- One of the following is not consider a sign of hemolytic anemia

- a- pallor b- splenomegaly c- itching
marking

18- Hemolytic anemia is characterized by the following except

- a- hemolytic jaundice b- gall stone
c- hemolytic crisis d- normal life span of red cell

19- The feature of hemolytic anemia is

- a- leucopenia b- reticulocytosis c- both

20- Complication of hemolytic anemia

- a- gall stone b- renal failure c- HTN

21- Red cell osmotic fragility is increased in

- a- thalassemia major b- hereditary spherocytosis
c- iron deficiency anemia d- HbC disease

22- Splenectomy is virtually curative in

- a- G6PD b- ITP
c- thalassemia d- hereditary spherocytosis

23- Ham's test used in diagnosis of

- a- spherocytosis b- paroxysmal nocturnal hemoglobinuria
c- anemia with chronic disease d- lymphoma

24- Causes of abdominal pain in thalassemia caused by all except

- a- vasculitis b- splenic infarction
c- dragging pain due to splenomegaly d- pigment stone due to induced biliary cyst

25- Hba2 is high in

- a- thalasemia
- b- vit b12 deficiency
- c- sideroblastic anemia
- d- iron deficiency anemia

26- Thalassemia major is inheritant as

- a- autosomal recessive
- b- autosomal co-dominant
- c- x- linked
- d- mitochondrial inheritance

27- Thalassemia major may be associated with all except

- a- cardiac arrhythmia
- b- cardiac tamponade
- c- congestive heart failure
- d- cardiomegaly

28- All of the following are seen in intravascular hemolysis except

- a- high urinary urobilinogen
- b- reticulocytosis
- c- high plasma stercobilinogen
- d- high urinary hemosiderin

29- Which of the following is associated with splenomegaly

- a- chronic renal failure
- b- aplastic anemia
- c- hereditary spherocytosis
- d- sickle cell anemia

30- All of the following produce microcytic anemia except

- a- sideroblastic anemia
- b- pernicious anemia
- c- lead poisoning
- d- iron deficiency anemia

Cases

1- Male patient has known history of iron deficiency anemia presented with stomatitis and spooning of nails, splenomegaly & dysphagia

- a- What is your diagnosis?
- b- What are the investigations?
- c- TTT you suggest?

2- Male patient has known history of peptic ulcer and gastrectomy presented with weakness and stiffness in leg, gloves & stock sensory loss

- a- What is your diagnosis?
- b- What are the investigations?
- c- TTT you suggest?

3- A 10 y. old child presented complaining from stunted growth & bouts of fever which recurs frequently since a long time. Examination revealed pallor, enlarged liver 4 fingers below the costal margin & enlarged spleen 5 fingers below the costal margin.

- a- What is your diagnosis?
- b- What are the investigations?
- c- TTT you suggest?

4- A 19 y. old medical secretary was admitted to hospital after she had fainted at work. She mentioned feeling tiredness in the last 6 months. On examination she was pale with no other remarkable physical signs. Her pulse was 100/min, BP was 100/50 & temp. 37°C. inv.: Hb: 7 gm/dl. RBCs hypochromic & microcytic, chest X-ray & ECG were normal.

- *What are other clinical & laboratory findings may help in the diagnosis of this girl's anemia?*

5- Young female aged 32 y. consulted her general practitioner because of getting pale and unable to perform her usual activities. He found her Hb. 8.7. gm/dl, reticulocytic count 0.3 % and microcytosis in the blood film. Serum iron was 28 Hg/100 and total iron binding capacity (IBC) 498 Hg/100 ml.

a- What is the diagnosis and why?

b- Mention three common causes for this condition in this age group.

6- A 55 y. old male presented with gradually increasing easy fatigability, shortness of breath on effort & numbness of both hands & feet for the last few months. He gave a history of subtotal gastrectomy for gastric carcinoma many years ago. General examination revealed only pallor. Chest examination was unremarkable while cardiac examination revealed ejection systolic murmur over the base. Apart from the scar of the previous surgery, abdominal examination revealed no abnormality

a- What is the most probable diagnosis?

b- What investigations would you order to reach the final diagnosis?

c- How would you treat this pt.?

7- A 13 y. old boy had an upper respiratory tract infection for which he received medications. 1 day later he came to you complaining of shortness of breath on exertion. Also he noticed that his urine became dark. His mother said that he had similar attacks during early childhood. O/E there was pallor, mild jaundice. Heart rate 110/min., regular. Abdominal and chest was normal.

a- What's your diagnosis (explain why)?

b- How do you investigate this case?

c- What's your management?

Oral

- 1- Complication of blood transfusion
- 2- Crisis of hemolytic anemia
- 3- B-thalassemia
- 4- Causes of iron deficiency anemia
- 5- Anemia (def – Type – invest)

Answer

MCQ

1- D	2- B	3- D	4- C	5- B	6- B	7- A	8- A	9- D	10- C
11- C	12- D	13- A	14- A	15- B	16- B	17- C	18- D	19- B	20- A
21- B	22- B	23- B	24- A	25- A	26- B	27- B	28- C	29- C	30- B

Cases

- 1) Plummer vinson disease
- 2) SCD on top of vit B12 deficiency
- 3) Most probably a case of chronic haemolytic anemia mostly thalassemia major
- 4) DD of microcytic hypochromic anemia:
 - I. Iron deficiency anemia
 - II. Thalassemia minor
 - III. Anemia of chronic disease e.g. CRF, TB
 - IV. Sideroblastic anemia (most common DD)
- 5) A case of microcytic hypochromic anemia
- 6) B 12 deficiency (megaloblastic anemia)
- 7) Hemolytic anemia e.g. drugs, G6PD

القوة انك تتحدى نفسك وانت تحت ضغط

القوة انك تتحدى الملل والاصباط

القوة ف الاستمرار .. ف المحاولة .. ف الثقة

قوتك انك تعمل اللي عليك وترضي ربنا وترضي النفس الخيرة اللي جواز

ذاكر .. وانجح .. وتفوق .. واثبت انك قوي

#قول_انا_قوي

#مراجعة_علام

Blood 2

(Poly cysthemia – Leukemia – Hypersplenism – bleeding disorders)

- 1- Polycythemia (causes – c/p)
- 2- Polycythemia rubra vera
- 3- Complication of polycythemia and mangement
- 4- Causes of splenomegaly
- 5- Causes / clinical picture / diagnosis of throbmocytopenic purpura
- 6- Hemorrhage (2 causes and DD of them)
- 7- Give account on ITP
- 8- Hemophilia
- 9- Account on hemophilia
- 10-Non- throbmocytopenic purpura
- 11- Vascular purpura (causes)
- 12-Platelet fraction
- 13- Thrombocytopenia
- 14-Discuss diagnosis of CML
- 15-Clinical and laboratory diagnosis of acute leukmia
- 16-Discuss diagnosis of CLL
- 17- Acute leukemia
- 18- Acute lymphoplastic leukmia (clinical and lab feature)
- 19- Describe CBC changes & finding in:
 - a- Acute lymphoblastic leukemia
 - b- Anemia of chronic blood loss
 - c- Pernicious anemia
- 20- Chronic lymphoblastic leukemia
- 21- Chronic myeloid leukemia (diagnosis / CP / TTT)
- 22- Chronic lymphatic & myeloid leukemia
- 23- Chronic granulcytic leukemia

MCQ

1- Causes of leukocytosis include all the following except

- a- acute myeloid leukemia
b- acute bacterial infection
c- hypersplenism
d- cushing syndrome

2- Extra- medullary relapse of acute lymphoblastic leukemia is common in

- a- CNS b- testicles c- Both

3- Relapse of acute lymphoblastic leukemia is commonly initiated in

- a- bone marrow b- meninges c- spleen

4- Feature of CML include all the following except

- a- huge spleen
b- hyperurecemia
c- thrombocytopenia
d- increase lymphocyte in BM

5- CML the following are seen except

- a- huge splenomegaly
b- leukocytosis
c- leukostasis
d- normal bone marrow

6- Splenomegaly is by far the most consistent physical sign in

- a- ALL b- CML c- CLL

7- Alpha interferon is the thraputic option in

- a- thalassemia b- Hogkin's lymphoma c- chronic myeloid leukmia

8- Chronic lymphatic leukemia is characterized by

- a- sever anemia
b- young age onset
c-generalized lymphadenopathy
d- rapidly progressive course

9- Skin lesion may occur in henoch-sholein purpra

- a- vesicular bullous b- pustular c- Both

10-In thrombocytopenic purpura there is

- a- incense clotting time b- increase bleeding time c- Both

11-In idiopathic primary cytopenic purpra the spleen is

- a- enlarged in all case b- abscent un all case
c- enlarged in 20% of case

12- Treatment of autoimmune thrombocytopenic purpura

- a- steroids b- ampicillin c- blood transfusion

13- Splenectomy is indicated in all except

- a- hypersplism b- spherocytosis
c- sickle cell anemia d- ITP

14- Which one of the following is false in hemophilia

- a- normal prothrombin time
- b- decrease fact. VIII Ag antigen level
- c- high partial thromboplastin time
- d- absent factor VIII activity

15- Gum bleeding is characteristic of all except

- a- aplastic anemia
- b- scurvy
- c- chronic phenothiazine therapy
- d- hemophilia

16- Which is not Vitamin K dependent factor

- a – factor 8
- b- factor 7
- c- factor 9
- d- factor 2

17-DIC is a complication of

- a- septicemia
- b- typhoid fever
- c- brucellosis

18-Thrombocytopenia is seen in the following except

- a- iron deficiency
- b- autoimmune
- c- hypersplenism
- d- bone marrow

19-The typical feature of polycythemia rubra vera include all the following except

- a- splenomegaly, leukocytosis, thrombocytosis
- b- headache, pruritis, peptic ulcer
- c- decrease leukocyte alkaline phosphatase

20- Which is not true in polycythemia rubra vera

- a- increase red cell mass
- b- thrombocytopenia
- c- marked hypercellular bone marrow
- d- basophilia

21-ESR decrease in:

- a- Collagenosis
- b- Infection
- c- Polycythemia

22-Complications of polycythemia vera

- a- Vascular thrombosis
- b- peripheral neuritis
- c- Edema of LL

Cases

- 1- 20 years old girl complain of metro-menorrhea and spontaneous gum bleeding, on examination there is petechi all over the body
 - a- Write DD of the cause?
 - b- Discuss one of them
- 2- 12 years old girl presented with bleeding tendency and exertional dyspnea , on examination there is marked pallor , eccymetic skin and cervical lymph node enlargement
 - a- What is the diagnosis?
 - b- What are the investigations? And other manifestation?
 - c- TTT you suggest?
- 3- A 25 y. old male who was working abroad for 2 years started to complain from irregular fever since one month with joint & bone pain. Examination revealed fever of 38°C together with generalized lymphadenopathy (1-2 cm in size) & a palpable spleen
 - a- What is the diagnosis?
 - b- What are the investigations? And other manifestation?
 - c- TTT you suggest?
- 4- A 23 y. old nurse recently developed multiple spontaneous bruises over her trunk and limbs and has been having heavy menses. She has also noted sore throat with painful ulcers. O/E, she is pale and there are multiple bruises over her limbs & trunk & there is a deep ulcer over the soft palate with oral candidiasis, her gums are bleeding spontaneously. Examination otherwise is normal. Blood picture was done which was found to be diagnostic.
 - a- Mention 2 possible causes their characteristic blood picture
- 5- A 60 y. old man complaining of increasing fatigue, excessive sweating and loss of weight. He was found to be pale with multiple skin bruises and massive splenomegaly.
 - a- What is the most likely diagnosis?
 - b- What other clinical features to search for?
 - c- How blood examination can help you in diagnosis?
 - d- How to confirm diagnosis?
- 6- A 65 y. old male patient complained of ill health, fatigue & lethargy over the last 4 months. He also had spontaneous bruising. On examination he was pale & jaundiced. Few significant axillary & cervical lymph nodes were palpable. Hepatosplenomegaly was evident. His CBC showed hemoglobin level of 8 mg/dl, WBCs 50,000/cmm with 90% small lymphocytes, platelets were 40,000/cmm.
 - a- What is your diagnosis?
 - b- What is your differential diagnosis?
 - c- Mention the possible causes of jaundice in this patient

Oral

- 1- Causes of bleeding tendency
- 2- Causes of thrombophilia
- 3- Classification of leukemia
- 4- Difference between hemophilia and purpura
- 5- How to diagnosis polycythemia

Answer

MCQ

1. C	2. A	3. B	4. D	5. D	6. B	7. C	8. C
9. C	10. B	11. C	12. A	13. C	14. B	15. C	16. A
17. A	18. A	19. C	20. B	21. C	22. A		

Cases

- 1) DD of purpura
- 2) Acute leukemia
- 3) Acute lymphoblastic leukemia
- 4) **Aplastic anemia:**

CBC:

- Pancytopenia: normocytic normochromic anemia
- ↓ Reticulocytic count. – TLC < 2.000/cmm - Thrombocytopenia

Acute leukemia: (myeloblastic)

- RBCs: Normocytic normochromic anemia
- WBCs: 20-100.000/cmm = variable blasts up to 90%
- Platelets: Thrombocytopenia

- 5) Myelofibrosis or chronic myeloid leukemia
- 6) Chronic lymphocytic leukemia

" يَرْفَعِ اللَّهُ الَّذِينَ آمَنُوا مِنْكُمْ وَالَّذِينَ أُوتُوا الْعِلْمَ دَرَجَاتٍ ۚ وَاللَّهُ بِمَا تَعْمَلُونَ خَبِيرٌ "

تأمل الآية وي كده

بالا بقى .. اعمل اللي يخلي ربنا يرضى عنك ويرفعك درجات

#مراجعة_علام

Infection and blood

(Lymphoma – Multiple myeloma – Infectious diseases)

- 1- Give an account on generalized lymphadenopathy
- 2- Give an account on fever with lymphadenopathy
- 3- Clinical picture and diagnosis of IMN
- 4- TTT of malaria
- 5- Discuss clinical picture and diagnosis and complication and TTT of typhoid fever
- 6- Diagnosis of brucellosis
- 7- Complication and TTT of brucellosis
- 8- Multiple myeloma (complication & TTT)
- 9- Enumerate
 - a- five important feature of MM
 - b- causes of secondary polycythemia
 - d-five general signs of Hodgkin lymphoma
- 10- Salmonellosis
- 11- Cerebral malaria TTT
- 12- Malignant malaria
- 13- Malaria (prophylaxis & TTT)
- 14- Malaria flacipurum (complication and mangement)
- 15- Giardiasis (TTT)
- 16- Septic shock
- 17- Fever with skin rashes
- 18- DIC
- 19-DD of cervical lymphadenopathy
- 20-Iron deficiency anemia (causes – diagnosis - TTT)
- 21-Auto immune hemolytic anemia (types – C/P - diagnosis)
- 22-Polythemia (types – diagnosis - TTT)
- 23-Bleeding from orrifles (causes)
- 24-Case of purpuric eruption (causes & diagnosis)
- 25-Criteria for diagnosis of chronic myeloid leukemia
- 26-Discuss diagnosis of hemolysis

MCQ

1- The most common feature of Hodgkin lymphoma is

- a- generalized lymphadenopathy
- b- anterior mediastinal lymphadenopathy
- c- bleeding tendency

2- Investigation of choice for diagnosis of lymphoma

- a- CBC
- b- blood film
- c- LN biopsy
- d- bone marrow biopsy

3- In multiple myeloma

- a- increase platelet
- b- high ESR
- c- low serum calcium

4- Scalene LN is present in

- a- above clavicle
- b- around jaw
- c- between 2 head of sternomastoid
- d- in occipital region

5- Specific feature of diphtheria include

- a- cervical lymphadenopathy
- b- high fever
- c- exudate beyond the tonsil

6- Clinical finding in typhoid fever

- a- skin rash
- b- coated tongue
- c- splenomegaly
- d- all of the above

7- Rose spot if present is diagnostic of

- a- glandular fever
- b- enteric fever
- c- malaria fever
- d- scarlet fever

8- Hemorrhage is a complication of typhoid fever occur in

- a- first week
- b- 2nd week
- c- 3rd week

9- Characteristic of brucellosis are except

- a- increase sweating
- b- spondylitis
- c- mental depression
- d- leucopenia

10- Recognized feature of brucellosis include all except

- a- high fever, night sweating, back pain
- b- hepatosplenomegaly
- c- oligoarthritis & spondylitis
- d- leukocytosis

11- Paul bunnell test is positive in infection with

- a- cytomegalovirus
- b- infectious mononucleosis
- c- rubella virus

12- Koplik's spot is diagnostic of

- a- enteric fever
- b- lobar pneumonia
- c- pulmonary TB
- d- measles

13-Triad of fever and hemorrhagic rash and retinal changes occur in

- a- meningitis
- b- typhus
- c- encephalitis
- d- rift valley fever
- e- all of the above

14- Praziquantel is drug of choice of

- a- bilharrzia mansoni
- b- fasciola hepatica
- c- amebiasis

15-Fungal infection complicaed hemodialysis is treated by

- a- phenothiazide
- b- amphotercine
- c- INH

16-In amebiasis we use the following

- a- metronidazole
- b- quinolone
- c- amoxicilline

17-Malaria falicpuram causes

- a- nephrotic syndrome
- b- nephritic syndrome
- c- non

18-Clinical feature of malaria include

- a- fever
- b- headache
- c- Chill's and rigor
- d- clubbing

19-Cerebral malaria is except

- a- treated oral quinine sulfate
- b- often complicated by multiple organ failure
- c- associated by sign of meningeal irritation
- d- IV dexamethasone reduce mortality

20-WBC in stool is not found in

- a- giardiasis
- b- shigella
- c- entero-invasive E-coli
- d- helicobacter.pylori

Cases

1- Old male 70 years old presented with bonyaches , ESR >100 with high calicium level

- a- *What is your diagnosis?*
- b- *What are the investigations?*
- c- *TTT you suggest*

2- A young 12 y. old girl presented with pyrexia of 38°C, malaise, anorexia, and hypochondrial pain. Examination revealed mild jaundice and enlarger tender lever. Blood picture showed eosinophilia of 20%

- a- *Give me most likely parasitic infection giving this picture*
- b- *Give any further investigations you would like to do*
- c- *What is the best drug for treatment?*

- 3- A Nigerian girl 11 y. old visiting Egypt was cough with fever & shivering. O/E she showed pallor & splenic enlargement. Blood examin. Clinched the diagnosis, and fever subsided after specific therapy.
- a- *What were the possible diagnostic features on blood examination?*
 - b- *What was specific management given?*
 - c- *Is a further management needed? Why?*
- 4- A Sudanese student 21 y. old, studying in Egypt was brought by his friend to the ER in a semicomatosed state since one day. They noticed that he passed too dark urine this day. History taking revealed that he returned from vacation which he has spent in this country since 3 weeks.
- 5- A 25 y. old female presented with irregular fever & pain in the Rt. Upper part of the abdomen of 1 month duration. Examination revealed temp. 37.7°C, enlarged tender liver & no palpable spleen. Blood picture showed RBCs 15000 / cm, 75% of the WBC were eosinophils & S. bilirubin was 2 mg/dl
- 6- A 15 y. old male pt. presented with fever 38.5°C for one week. He has enlarged discrete firm LNs on both sides of the neck. His tonsils are enlarged & covered by extensive white membrane. He was given ampicillin (500 mg/6h) on assumed streptococcal tonsillitis. After the first dose he developed severe generalized, maculopapular rash.
- a- *Was the diagnosis of streptococcal tonsillitis correct?*
 - b- *What is the most likely diagnosis?*
 - c- *What is the causative agent(s)?*
- 7- An adult Egyptian man came back recently from a visit to South Sudan. There he suffered from fever, rigors & sweating and was diagnosed as malaria but received irregular treatment. Now he complains of fever, rigors and passage of very dark urine.
- a- *What are the complications he developed after he returned back to Egypt?*
 - b- *What is the most likely drug he received in Sudan?*
 - c- *How can you manage this complication?*
- 8- A 19 y. old pt. presented with one week history of malaise and anorexia followed by fever and sore throat. On examination, the throat was inflamed without exudates but with few palatal petichae. There was cervical lymphadenopathy. Abdominal examination revealed enlarged liver and spleen. Throat culture was negative for group A streptococci. Total leukocytic count was 14.000/mm³ (30% segmented neutrophils, 60% lymphocytes, and 10% monocytes). Serum total bilirubin was 2 mg/dl, SGOT was 60 U/L, SGPT was 70 U/L. Serum ALP was normal serology for hepatitis A, B and C were all negative.
- a- *What is the most likely diagnosis?*
 - b- *Give one differential diagnosis?*
 - c- *Give 2 important investigations to confirm*

9- A 65 y. old male complained of easy fatigue and effort intolerance for the last 2 months. The patient denied any bleeding tendency or G.I.T. symptoms apart from poor appetite and some weight loss. O/E there was pallor & tenderness on the bones. Blood chemistry analysis revealed a serum creatinine of 3 mg/dl, S.globulin was 5.5 gm/dl, S.phosphorus was 4 mg/dl, S. albumin was 3.4 gm/dl, S.globulin was 5.5 gm/dl, S. potassium was 4 mEq/Land, S. sodium was 134 mEq/L. CBC revealed Hb. Of 8 gm/dl with normocytic normochromic RBCs. ESR was 140 1st hour.

- a- What is your possible diagnosis?*
- b- What is your differential diagnosis?*
- c- What investigations would you suggest to reach a final diagnosis?*

10- A 20 y. old female consulted her doctor for lumps in the neck. On examination, these lumps were found to be enlarged cervical lymph nodes, tender on both sides of the neck. There was also enlarged axillary lymph nodes but to lesser extent. There was sore throat with some reddish spots in the pharynx. Her temperature was 38°C. Chest and heart examination were unremarkable & showed no abnormalities. CBC showed a total leucocytic count of 25.000/cc with predominant mononuclear cells rather than polymorph nuclear cells.

- a- What is your diagnosis?*
- b- What is your differential diagnosis?*
- c- What other tests would you like to ask to reach a definitive diagnosis?*

11-A 16 y. old female patient presented with one week history of malaise & anorexia & bone pains followed by fever & sore throat. On examination, the throat was inflamed without exudates, with few palatal petichae. There was cervical lymphadenopathy. Abdominal examination revealed enlarged liver and spleen. Throat culture was negative for group A streptococci. Total leucocytic count was 12,000/mm³ (30% segmented neutrophils, 60% lymphocytes, and 10% monocytes). Serum total bilirubin was 2 mg/dl, SGOT was 60 U/L, SGPT was 70 U/L. Serum alkaline phosphates was normal. Serology for hepatitis A, B and C were all negative

- a- What is the most likely diagnosis?*
- b- Give one differential diagnosis?*
- c- Give 2 important investigations to confirm*

12-A 36 y. old butcher presented with fever 38.5°C marked sweating & polyarthralgia & low back pain for 1 week. O/E there was bilateral small axillary's non-tender LN. The liver & spleen were mildly enlarged.

- a- What is the most probable diagnosis?*
- b- How would you confirm your diagnosis?*
- c- What is your suggested treatment?*

Oral

- 1- Complication of blood transfusion
- 2- Generalized lymphadenopathy
- 3- Fever of unknown origin
- 4- Causes of splenomegaly
- 5- Difference between hodjkin and non hodjkin lymphoma

Answer

MCQ

1- B	2- C	3- B	4- C	5- A	6- D	7- B	8- C	9- C	10-D
11-B	12-D	13-D	14-A	15-B	16-A	17-C	18-C	19-A	20-A

Cases

- 1) Multiple myeloma
- 2) Fasciola infestation (Hepatitis like + eosinophilia)
- 3) Malaria
- 4) Diagnosis: Malaria complicated by black water fever
- 5) Most probably a case of fascioliasis (parasitic infestation of the liver by fasciola hepatic)
- 6) A case of IMN
- 7) Diagnosis: Malaria complicated by black water fever
- 8) Most probably a case of IMN
- 9) Multiple myeloma
- 10) Most probably a case of IMN (glandular fever)
- 11) Most probably a case of IMN
- 12) Most probably a case of Brucellosis

Key words for blood cases

- 1- Petechiae or rash = ITP
- 2- Sever hemoptysis, hematemesis= DIC or Vit K deficiency
- 3- Pallor + fatigue and low HB and MCV = iron deficiency anemia
- 4- Low HB , low RBC and high MCV , low WBCs = megaloblastic anemia
- 5- Low HB and reticulocytosis , Heinz bodies = G6PD deficiency
- 6- Low HB and reticulocytosis and pain over the body = sickle cell anemia
- 7- Low HB and WBC and platelet (or two of them) and decrease bone marrow cellularity = aplastic anemia
- 8- Prolonged fever +lymphadenopathy& hepatosplenomegaly = lymphoma
- 9- Increase WBCs and blasts> 20 % = acute leukemia
- 10-Blasts < 10% =chronic leukemia

Keywords for infection

- 1- Fever + constipation + rash +relative bradycardia = typhoid
- 2- Fever + GIT symptom + tender hepatomegaly + basal lung crepitation = amobeic liver abscess
- 3- Fever + chills + sweating + trip of Africa = malaria
- 4- Fever + visual manifestation + lymphadenopathy = toxoplasmosis
- 5- Fever + sore throat + lymphadenopathy and may be rash after ampicillin = IMN
- 6- Fever for 3 weeks – 10 days free + lymphadenopathy or HSM = brucellosis
- 7- Epigastric pain + preserved appetite + anemia = ankylostoma
- 8- Sever watery diarrhea – vomiting + sign of dehydration = cholera

مش لاصق..المذاكرة كثير..المنهج كبير..الكتب مش بتخلص
عايز أنا..عايز أخرج..الجو حر..الناس كلها أجازه وأنا لأ
لو عندك ارادة قوية لتقدر تتغلب على كل المعوقات دي
لو استسلمت متلومش إلا نفسك
مكانك بايدك لتعدوه
#حقي_حلمك
#مراجعة_علام

Chest 1

(Pleural disease – Pneumonia – Symptoms – Mediastinal S.)

- 1- Causes & c/p of pleural effusion
- 2- Clinical diagnosis of massive pleural effusion
- 3- Pleural effusion (causes / c-p / diagnosis / investigation / DD)
- 4- Discuss c/p – management of tension pneumothorax
- 5- Types and c/p of spontaneous pneumothorax
- 6- Pneumothorax (causes / c-p / types / diagnosis / TTT)
- 7- C/P & diagnosis of lobar pneumonia
- 8- give an account on community acquired pneumonia (def., causes, inust., TTT, etiology)
- 9- give an account of pneumococcal pneumonia
- 10-causes & DD of hemoptysis
- 11-DD of acute chest pain
- 12-Bronchiectasis (def., types, management, C/P, TTT)
- 13-Discuss clinical and radiological diagnosis of suppurative lung disease
- 14-(Causes, C/P, management) of lung abscess
- 15-Cough (enumerate causes and discuss on of them)
- 16-DD of unilateral effusion
- 17-Causes of acute dyspnea

MCQ

1- Bilateral pleural effusion is seen in

- a- nephrotic syndrome
- b- constrictive pericarditis
- c- congestive heart failure
- d- all of the above

2- The most common cause of pleural effusion

- a- liver cirrhosis
- b- myxedema
- c-SLE
- d- pulmonary tuberculosis

3- Haemorrhagic pleural effusion occur in

- a- liver cirrhosis
- b- myxedema
- c-SLE
- d- pulmonary tuberculosis

4- The most common cause of community acquired pneumonia is

- a- group A streptococcus
- b-hemophilus influenza
- c- strept pneumonia
- d- klebsiella pneumonia

5- Sign of pleural effusion

- a- absent breath sound
- b-increase TVF
- c- trachia shifted to same side

6- Causes of transudate in cases of pleural effusion

- a- nephrotic syndrome
- b- TB
- c- bronchogenic carcinoma

7- The clinical sign of pleural effusion are

- a- dyspnea
- b- clubbing of fingers
- c- cough and expectoration
- d- stony dullness in percussion

8- Sign of pleural effusion

- a- dullness in the same side
- b- increase TVF
- c-normal breath sound

9- Malignant pleural effusion is characterized by

- a- haemorrhagic
- b- rapid accumulation on tapping
- c- has all the lab feature of exudate
- d-all of the above

10- Absent litten sign is found in

- a- pneumonia
- b- pleural effusion
- c- TB
- d- chronic bronchitis

11- Exudative pleural effusion occur in

- a- pulmonary infraction
- b- nephrotic syndrome
- c- HF

12- Exudative pleural effusion is seen in

- a- constrictive pericarditis
- b- nephrotic syndrome
- c- bronchogenic carcinoma
- d- rt venticular failure

13- Massive pleural effusion is seen in

- a- constrictive pericarditis
- c- bronchogenic carcinoma

- b- nephrotic syndrome
- d- right ventricular failure

14- In haemoptysis blood usually comes from

- a- bronchial vein
- c- cavitory lesion

- b- pulmonary edema
- d- bronchoectasis

15- Brassy cough is seen in

- a- recurrent laryngeal nerve injury
- c- heavy smoker

- b- acute laryngitis
- d- carcinoma of larynx

16- Pink, frothy, profuse sputum is seen in

- a- pneumoconiosis
- b- acute pulmonary edema
- c- aspergilloma

17- Kussmaul breathing is characterized of

- a- metabolic acidosis
- c- metabolic alkalosis

- b- respiratory acidosis
- d- respiratory alkalosis

18- Finger clubbing is caused by the following except

- a- chronic bronchitis
- c- bronchogenic carcinoma

- b- bronchiectasis
- d- cryptogenic fibrosis alveolitis

19- Creptiation uninfluenced by cough is found in

- a- acute pulmonary edema
- c- lung abscess

- b- consolidation
- d- fibrosing alveolitis

20- Tracheal deviation occurs in except

- a- sarcoidosis
- b- acute pneumonia
- c- pleural effusion

21- Increased TVF due to one of the following

- a- pleural effusion
- b- lung consolidation
- c- pulmonary fibrosis

22- Clubbing of the finger is caused by the following except

- a- suppurative lung disease
- c- lung consolidation
- b- bronchogenic carcinoma
- d- mesothelioma

23- Clubbing is present in all except

- a- fibrosing alveolitis
- c- emphysema
- b- cystic fibrosis
- d- lung abscess

24- Causes of dull trapezoidal area is seen in all except

- a- left lobe hepatomegaly
- c- pericardial effusion
- b- Pancoast tumor
- d- splenomegaly

25- The most dangerous pneumothorax is

- a- tension
- b- traumatic
- c- primary spontaneous

26- Sign of pneumothorax

- a- retracion of the affected side b- stony dullness c- TVF decrease

27- Drug of choice in mycoplasma pneumonia

- a- penicillin b- tetracycline
c- cefuroxime d- erythromycin

28- Apical bronchiactsis indicate

- a- foreign body b- tumor
c- TB d- fibrosis

29- The main feature of chronic lung abscess in clude

- a- hemoptysis b- chest wheeze
c- cough and expectorant d- none of the above

30- Suppurative syndrome is charactrized by the following except

- a- excessive expectoration of foeted sputum b- finger clubbing
c- bronchial hyper- reactivity

31- All are causes of bronchiectsis except

- a- inhaled foreign body b- primary hypogamma-globulinemia
c- cystic fibrosis d- sarcoidosis

32- Complication of bronchiectesis include

- a- cerebral abscess b- pneumonia
c- empyema d- all of the above

33- Atypical pneumonia is caused by

- a- chlamidia pneumonia b- mycoplasma pneumonia
c- legionella bacilli d- all of the above

34- Pneumatocele is formed in pneumonia caused by

- a- staphylococcus aureus b- klebsilla pneumonia
c- streptococcal pneumonia d- mycoplasma pneumonia

35- In acute pneumonia the following is required except

- a- sputum examination b- chest x- ray
c- pulmonary function test d- blood picture

36- Most common cause of hospital acquired pneumonia

- a- gram +ve b- gram -ve c- atypical organism

37- Drug of choice of pneumococcal pneumonia is the following except :

- a- beta lactam antibiotics b- aminoglycoside c- sulphonamide

38- Worldwide commonest cause of hemoptysis

- a- bronchogenic carcinoma
 - b- pulmonary TB
 - c- pneumonia
 - d- acute bronchitis
-

Cases

- 1- Patient with FAHM, chest pain, cough, expectorant with large foeted big amount of sputum increased on lying on RT side
 - a- what is your diagnosis?
 - b- c/p & investigation?
 - c- TTT you suggest?
- 2- Patient with FAHM, dyspnea, chest pain, cough, expectorant of rusty blood stained sputm?
 - a- what is your diagnosis?
 - b- c/p & investigation?
 - c- TTT you suggest?
- 3- 32 years old patient with history of community acquired pneumonia without improvement in examination: diminish breath sound and dullness of LT lung
 - a- What is your diagnosis?
 - b- C/P & investigation?
 - c- TTT you suggest?
- 4- A 25 y. old man presented with generalized edema, pallor and a blood pressure of 170/110. He also had chronic cough of 12 years duration with expectoration of more than glassful of mucopurulent sputum during exacerbations of the illness
 - a- How to investigate this case
 - b- How to investigate the possible relationship between the presenting features and its original disease condition?
 - c- Discuss the lines of management
- 5- A 24 y. old male complained of cough with blood streaked sputum, fever & chills of 2 days duration. There was stitching pain in the right infrascapular area. Physical findings revealed dullness and crepitations in in the right infrascapular area.
 - a- What is the probable diagnosis?
 - b- How would you investigate such case?
 - c- What is your suggested treatment?

- 6- A 65 y. old engineer used to smoke 40 cigarettes/day for last 40 years. He presented with gradually increasing shortness of breath & coughing of blood streaked sputum. On examination, he had central cyanosis & 2nd degree clubbing. His blood pressure was 130/90, pulse rate was 90/min, regular & he has a febrile. Chest examination revealed respiratory rate of 20/min, harsh vesicular breathing with expiratory bronchi on both lungs except on the lt. infraclavicular area where bronchial breathing was audible.
- a- *What are the possible causes of his complaint?*
 - b- *What investigations would you order for this patient?*
 - c- *How would you treat this patient?*
- 7- A 55 y. old male, smoker with a long history of productive cough and exertional dyspnea, presented to the emergency room with sudden onset of pain at the right side of the chest that increases with breathing. He also complained of increasing dyspnea. O/E he was in respiratory distress with mediastinal shift to the left.
- a- *What's your diagnosis?*
 - b- *What are the expected physical signs on local chest ex.?*
 - c- *What's the single immediate inv. To confirm the diagnosis?*
 - d- *What's your management?*
- 8- A 25 y. old male felt acute stitching pain in the left side of the chest while driving a bicycle, followed shortly by marked dyspnea. He presented himself to the doctor, who discovered that the left side of the chest is larger than the right and shows decreased air entry.
- a- *What is the probable diagnosis?*
 - b- *How would you investigate such case?*
 - c- *What is your suggested treatment?*
- 9- A 60 y. old male on oral treatment for diabetes got a sudden sense of coldness and chilling, followed shortly by the development of pyrexia. Next day cough started to appear, increased rapidly in severity, and became painful. By the fourth day, he became markedly ill, dyspnic & the cough became very distressing, rusty sputum started to appear and increased progressively. Examination on the 6th day revealed 38°C, 110/70, 110/min. pulse, marked dullness on the right lung base with decreased air entry and medium sized consonating crepitation. The WBC was 14000/min
- a- *What is the probable diagnosis?*
 - b- *How would you investigate such case?*
 - c- *What is your suggested treatment?*

Oral

- 1- DD of chest pain
- 2- Causes of sudden dyspnea
- 3- Causes of acute chest pain
- 4- Types of pneumonia
- 5- Cough (def- types)
- 6- Mediastinal syndrome
- 7- How to diagnosis suppurative lung disease

Answer

MCQ

1- D	2- D	3- D	4- C	5- A	6- A	7- D	8- A	9- D	10- B
11- A	12- C	13- C	14- A	15- D	16- B	17- A	18- A	19- D	20- B
21- B	22- C	23- C	24- B	25- A	26- C	27- D	28- C	29- C	30- C
31- D	32- D	33- D	34- A	35- C	36- A	37- C	38- D		

Cases

- 1- Lung abscess of LT side
- 2- Pneumonia
- 3- Post-pneumatic effusion
- 4- A case of suppurative lung disease complicated by amyloidosis kidney
- 5- Most probably a case of labor pneumonia
- 6- Pneumonia with underlying bronchogenic carcinoma
- 7- Pneumonia with pleurisy → pleural effusion
- 8- Most probably a case of open pneumothorax mostly due to ruptured blep
- 9- Most probably, it's a case of labor pneumonia complicated by pleurisy (immune compromised pt)

Chest 2

(BA – Resp. function – Resp. F. – COPD – Corpalmonale)

- 1- Chronic asthmatic bronchitis
- 2- Chronic bronchitis (predisposing factor – c/p –complication – TTT)
- 3- Bronchial asthma (pathogenesis –A/E –c/p –diagnosis - management – types – def.)
- 4- Status asthmatics (definition –c/p – TTT)
- 5- TTT of bronchial asthma (how to assess the severity of BAR)
- 6- TTT of acute severe asthma
- 7- Management of acute attack of BA
- 8- Emphysema (diagnosis)
- 9- Abnormal pulmonary function test of patient of COPD
- 10-COPD (def., diagnosis)
- 11-COPD (aetiology – C/P – complication – TTT)
- 12-Clinical presentation of COPD
- 13- Core pulmonale (def., causes, C/P, TTT)
- 14-Smoking (diagnosis –measure to reduce it)
- 15-Acute respiratory failure
- 16-Respiratory failure (types, C/P, TTT)

دلوقتي وقتها

المذاكرة

#مراجعة_علام

MCQ

- 1- All are the typical feature of asthma except**
a- eosinophilic bronchial infiltration
b- air way macrophage
c- epithelial shedding
d- goblet cell hypoplasia
- 2- In bronchial asthma**
a- reduce FEV1
b- increase PCO₂
c- increase VC
- 3- Breathing in bronchial asthma is**
a- harsh vesicular
b- bronchial
c- cavernous
- 4- Main Antibody in BA is**
a- IgG
b- IgM
c- IgE
- 5- Internsic asthma is characterized by all except**
a- on onest of adult life
b- high circulating IgE
c- sputum eosinophilia
d- good respond to steroid
- 6- Immune inflamatory cells in BA**
a- esinophillia
b- mast cell
c- Both
- 7- BA wheeze is**
a- inspiratory sibilant
b- sonorous
c- expiratory sibilant
- 8- Spirometer of BA reveals**
a- restriction
b- obstructive
c- both
- 9- Which of the following is not seen in BA**
a- dyspnea
b- cough
c- chest pain
d- wheeze
- 10- The following suggest sever BA in**
a- central cyanosis
b- hyperinfilated chest
c- increase breath sound
- 11- All drugs used in TTT of chronic presistent asthma except**
a- inhaled B₂ agonist
b- systemic steroids
c- propranolol
d- inhaled steroids
- 12- In sataus asthmaticus which of the following is bad prognosis sign**
a- sever tachycardia
b- central cyanosis
c- absent wheeze
d- all of the above
- 13- In emphysema there is enchroachment on**
a- hepatic dullness
b- cardiac dullness
c- Both

14-Causes of emphysema

- a- acute bronchitis
- b- pneumonia
- c- chronic bronchitis
- d- BA

15- Breath sound is in emphysema

- a- diminish
- b- absent
- c- normal

16- Aveolocapillary block may occur in

- a- COPD
- b- interstitial lung disease
- c- both

17- The specific diagnosis method in early COPD

- a- xray
- b- blood gases
- c- spirometer

18- Which is correct in type 2 respiratory failure

- a- diminished P_{O_2} and diminished P_{CO_2}
 - b- diminished P_{O_2} and normal P_{CO_2}
 - c- normal P_{O_2} and increase P_{CO_2}
 - d- diminished P_{O_2} and increase P_{CO_2}
-

Cases**1- 60 years old male with COPD with peripheral edema and tender liver**

- a- What is the diagnosis?
- b- C/P and investigation
- c- TTT of case

2- Patient 65 years old heavy smoker suffer from cough & dyspnea and expectoration, by examination show hyperinflated chest with palpable wheeze all over the chest and percussion note on Bare area was resonant

- a- What is the diagnosis?
- b- C/P and investigation
- c- TTT of case

3- A 30 y. old male gives history of continuous wheezing & breathlessness for four days. He had frequent attacks of asthma since childhood and maintained on bronchodilators by his physician.

- a- What three physical signs will help in assessing the severity of this patient asthma?
- b- If the diagnosis was established mention three immediate lines of management.

Oral

- 1- Difference between BA & COPD
 - 2- Investigation of obstructive air way disease
 - 3- TTT of BA
 - 4- Respiratory failure
 - 5- Core pulmonale
-

Answer

MCQ

1- D	2- A	3- A	4- C	5- B	6- C	7- C	8- B	9- C
10- A	11- C	12- D	13- C	14- C	15- A	16- C	17- C	18- D

Cases

- 1- (COPD with RVF) corpulmonale
- 2- COPD
- 3- Acute severe asthma

آمن بحلمک .. طریقہ طویل
احلم ہتوصل .. مفیش مستحیل
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#مراجعة_علام

Chest 3

(T.B. – Fibrosis – Collapse – Sarcoidosis – Pul. embolism – Bronchial carcinoma)

- 1- Give an account of one of anti TB drug programme
- 2- Discuss management of pulmonary TB
- 3- Clinical presentation and method of diagnosis of post primary pulmonary TB
- 4- Bronchogenic carcinoma (C/P and diagnosis)
- 5- Pulmonary embolism (causes, predisposing fact, management)
- 6- Lung collapse (causes and C/P –radiological finding)
- 7- Pulmonary TB (PTT factor – pathogenesis – C/P – investigation and management)
- 8- Ghon's focus
- 9- Bronchogenic carcinoma (C/P – extra thoracic & intrathoracic manifestation – investigation and TTT)
- 10- Sarcoidosis
- 11- Lung fibrosis
- 12-Primary & chronic pulmonary TB
- 13-Interstitial pulmonary fibrosis (causes, C/P, TTT)

MCQ

- 1- Mycobacterium TB lives in**
a- macrophages b- lymphocyte c- both
- 2- All about pulmonary TB are false except**
a- Is highly infectious b- produce cavitation
c- may be asymptomatic d- commonly associated with negative tuberculin test
- 3- In post primary pulmonary TB there is**
a- enlarged hilar LN b- lung cavitaion c-+ve tuberculin test
- 4- Sign of lung fibrosis**
a- increase TVF b- decrease breath sound c- trachea shifted to other side
- 5- In TB meningitis CSF cholorides**
a- reduce moderately b- increase c- normal
- 6- Steroids is given in TB cases with**
a- all patient b-core pulmonale c- pericardial effusion

- 7- Steroids are indicated in TB in all except**
 a- pericardial effusion b- sever illnes c-first stage meningitis
- 8- Most effective anti-tuberculus drugs are**
 a- rifampicin b- acetohexamide c- ciprofloxacin
- 9- BCG vaccine is**
 a- killed vaccine b- attenuated strains c-purified protein dervative
- 10- Complication of advanced pulmonary TB may include**
 a- massive hemoptysis b- amyloidosis
 c- congestive HF d- none
- 11- TB plueral effusion**
 a- exudative b- transuditave c- mixed
- 12-The dose of INH in TTT of TB is**
 a- 5 mg/kg b- 10mg/kg
 c- 50mg/kg d- none
- 13-False -ve tuberculous test seen in**
 a- AIDS patient b- miliary TB c- both
- 14-All the following are causes of false –ve tuberculine test except**
 a- atypical mycobactria b- lymphoma c- in active tuburclin
- 15-Mantox test is used to diagnosis**
 a- TB b- systemic sclerosis c- sarcodosis
- 16- Sarcodiosis**
 a- caseating granuloma b- non caseating granuloma c- both
- 17- In sarcodiosis the following is present except**
 a- erythema nodusum b- pleurla effusion c- thrombocytosis
- 18- Sarcodiosis occurs due to defective**
 a- humural immunity b- cell medited c- both
- 19- Pulmonary infiltration in sarcodiosis occur in**
 a- stage 1 b- stage 2 c- stage 3
- 20- Bronchial adenoma most commonly present with**
 a- cough b- stridor
 c- recurrent hemoptysis d-chest pain
- 21- The commenst benign pulmonary neoplasm is**
 a-hamartoma b- fibroma
 c- adenoma d- lipoma

22- Bronchial adenoma is commonly presented with

- a- cachexia b- hemoptysis c- hematemesis

23- The clinical picture of oat cell carcinoma is

- a-cough and expectorant
b- cushinoid feature of Pt.
c- hemoptysis
d- all of the above

24- Which of the following is hypersensitivity type 1

- a- pulmonary TB b- anaphylaxis c- cardiac asthma

25- Bilateral hilar lymphadenopathy is seen in all except

- a- sarcoidosis
c- bronchogenic carcinoma
- b- lymphoma
d- pneumoconiosis

26- Cor- pulmonale means

- a- RVF due to MS
b- RVF due to LVF
c- RVF due to lung disease
d- RVF due to VSD

27- Anaphylaxis is

- a- type 1 hypersensitivity b- type 2 c- type 3

28- Cavitory lung lesion is seen in

- a- pneumococcal b- sarcoidosis c- staph pneumonia

29- Pulmonary diffusion defect occur in

- a- acute pulmonary edema b- bronchial obstruction c-sarcoidosis

Cases

1- Diabetic patient had past history of cough and night sweating and fever and recently has suffered from dyspnea. On examination there is limitaion on movement on RT side

- a- What is your diagnosis?
- b- C/P & investigation
- c- TTT you suggest

2- Farmer 52 y. old presented to outpatient clinic by an exertional dyspnea. cough and lower limb edema.

On examination pulse is 73 beat/min, temp 36.9, Bp 130/80 mmHG, RR 29cycle/min with blue tongue and clubbing, chest examination reveal elevate crepitation in all over the lung

- a- What is your diagnosis?
- b- What is the cause?
- c- C/P & investigation
- d- TTT you suggest

3- Old male 70 years old heavy smoker with sudden change in cough pattern and presented by hemoptysis and clubbing

a- What is your diagnosis?

b- C/P & investigation?

c- TTT you suggest

4- Five days after on operation for fracture neck of femur, a 73 y. old man developed fever and shivering. A week later he started to cough and brought up hemorrhagic sputum. He had sharp pain in the lower right side of the chest and right upper abdomen.

a- What are points in the clinical examination would you specifically look for & Why?

b- How would you manage the patient?

c- What is special advice would you give him in order to avoid recurrence if he is to have further operations?

5- A 55 y. old man, smoking for 30 years and having “smokers cough” developed acute chest infection last September and another one last December. After the second episode cough changed in character and the sputum started to become blood tinged. He presented to you on account of progressive weakness and numbness in hand and feet which were progressive over the previous 8 weeks.

a- What particular points in the general examination and examination for the various systems should you look for? Why?

b- Select 3 important investigations that would help you to reach a definitive diagnosis and might help your choice of the line of management. Explain how?

c- What particular consultant might be needed for further management?

d- What additional investigations would you prepare before calling this patient?

6- A 60 y. old man with chronic cough that he attributes to heavy smoking for many years started to get frank haemoptysis.

a- How can sputum analysis and chest radiology suggest the most important causes of his recent haemoptysis?

b- Mention another additional investigation necessary to reach the proper diagnosis

7- A 58 y. old man who smokes 40 cigarettes per day gives a history of chronic cough for many years. In winter, he brings up purulent sputum with episodes of fever & ill health. Prior to the present admission he had lost 8 kg of body weight and his breathing had become more diff. & he started to bring bloody sputum. O/E he had pyrexia 38°C, finger clubbing & he was breathless at rest & chest expansion was poor with signs of pleural effusion on the Rt. Side.

a- What is your interpretation of this pt. history?

b- Give 3 possible causes for this pt. haemoptysis

c- Suggest 3 important investigations to diagnose this case

8- A 25 y. old female presented with sudden sever dyspnea & haemoptysis of frank blood following effort. She mentioned history of rheumatic fever during childhood.

a- What is your possible diagnosis?

b- What are the clinical findings you expect to find on examination of the pre cordium?

c- Mention one investigation that probably assesses her illness

9- A 40 y. old obese lady had performed cholecystectomy for calcular gall bladder disease. She complained of acute dyspnea and haemoptysis on the 8th day after operation.

a- What is the important possible diagnosis?

b- Mention the diagnosis investigations

c- What is the line of treatment

Oral

1- TTT of TB

2- Difference between TB & sarcoidosis

3- Management of pulmonary embolism

4- Para malignant syndrome of bronchogenic carcinoma

5- Bronchogenic carcinoma

Answer

MCQ

1- A	2- C	3- B	4- B	5- A	6- C	7- B	8- A	9- B	10- B
11- A	12- A	13- C	14- A	15- A	16- B	17- C	18- B	19- B	20- C
21- A	22- B	23- D	24- B	25- D	26- C	27- A	28- B	29- A	

Cases

1- Bronchogenic carcinoma

2- Interstitial lung fibrosis

3- Post TB Fibrosis

4- DVT lead to pulmonary embolism

5- Bronchogenic carcinoma and presented by paramalignant syndrome

6- A case of bronchogenic carcinoma

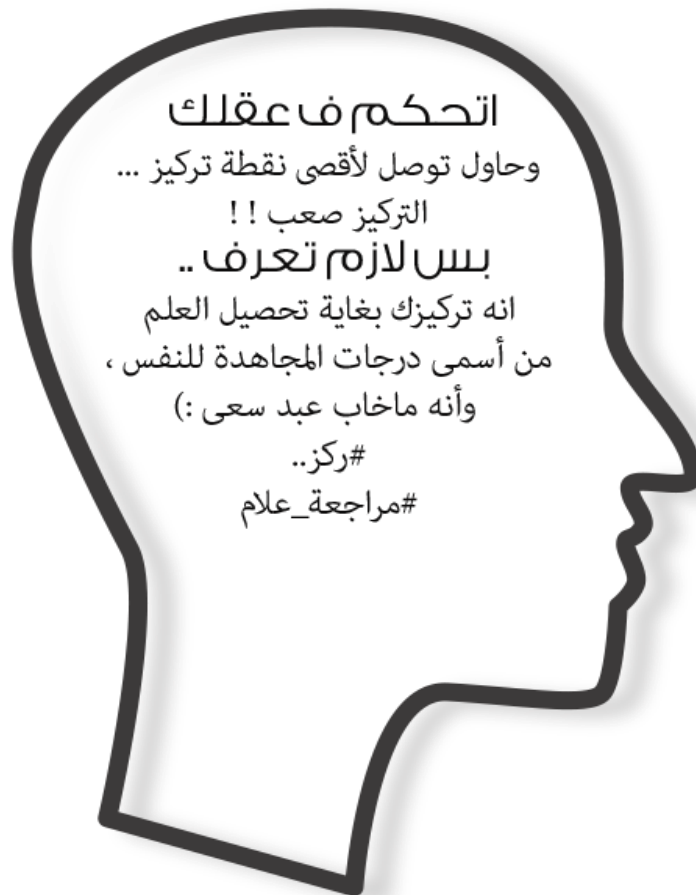
7- Most probably a case of bronchogenic carcinoma

8- Most probably it's a case of pulmonary infarction

9- DVT pulmonary embolism or pneumonia

Keywords of chest

- 1- Smoker + hemoptysis+ clubbing = bronchogenic carcinoma
- 2- Smoker + dyspnea + expiratory wheeze = COPD
- 3- Hemoptysis + loss of body weight + night sweating = TB
- 4- Pulurent sputum = suppurative and if after operation suggest primary lung abscess
- 5- Wheezy chest in young age = bronchial asthma
- 6- Acute dyspnea and shortening of breath sitting too much in bed , taking CCP = pulmonary embolism
- 7- Tall and thin patient with sudden dyspnea , smoker , chest pain = primary spontaneous pneumothorax
- 8- Dyspnea and cough and wheeze 1 hour after asprin (asprin induce asthma)
- 9- Dyspnea and dry cough with fascial congestion and bluish tounge = superior mediastinal syndrome
- 10- *Chest pain + cough + dyspnea + Hemoptysis + ting of jaundice = pulmonary infarction*
- 11- *Acute shock + cyanosis + dyspnea + low COP = massive pulmonary embolism*



Hepatology 1

(Liver f. test – Hepatitis – Liver Ø – L.C.F. – H. encephalopathy – Acute hepatic failure)

- 1- Enumerate 3 liver function test and discuss their diagnostic value
- 2- Viral hepatitis (types / complication)
- 3- Acute viral hepatitis (causes /clinical picture / management)
- 4- Serological and biochemical diagnosis of HBV
- 5- Clinical picture and lab diagnosis of chronic viral hepatitis
- 6- Auto immune hepatitis
- 7- Chronic hepatitis (causes / clinical picture / investigation / types)
- 8- Extra hepatic manifestation of HCV
- 9- Prevention of HAV & HBV
- 10- Chronic HCV (C/P, management)
- 11- Compare below Acute hepatitis A,B,C in prevention, serological diagnosis
- 12- Complication of hepatitis B infection
- 13- Complication and treatment of hepatitis
- 14- Non-viral hepatitis (causes)
- 15- Liver cirrhosis
- 16- Clinical picture and treatment of hepatic encephalopathy
- 17- Causes and Type of liver cirrhosis
- 18- Describe clinical feature of liver cell failure
- 19- Malignant liver (clinical picture / diagnosis)
- 20- Liver failure (aetiology / clinical picture / treatment / investigation)
- 21- In hepatic failure comment on
 - A- spider nevi
 - B- flapping tremors
 - C- gynecomastia
 - D- caput medusa
- 22- Hepatic encephalopathy (causes / pathogenesis /clinical picture / treatment / diagnosis)
- 23- Hepatic coma (predisposing factors / treatment)
- 24- Hepatocellular carcinoma (diagnosis)

MCQ

- 1- **Main causes of edema on liver cirrhosis is:**
a) hypo-albuminemia b) salt & water restriction c) portal hypertension
- 2- **In patient with hepatic cirrhosis the determine of ascites may contribute to following except :**
a) portal hypertension b) hypo albumenimia
c) salt retention d) porto systemic shunt
- 3- **All of the following is feature of hepatocellular faliure except :**
a) foetar hepaticus b) flapping tremors
c) Ascites d) hematemesis
- 4- **Liver cell failure is characterized by all except :**
a) spider nevi b) hypoalbumenimia
c) esophageal varices d) jaundice
- 5- **Important sign in LCF :**
a) flapping tremors b) fine tremors c) intention kinetic tremors
- 6- **Clinical manifestation of LCF include :**
a) flapping tremors b) spider nevi
c) hepatomegaly d) all of the above
- 7- **All of the following are manifestations of liver cell failure except :**
a) fever b) flapping tremors
c) spider nevi d) arthritis
- 8- **All of the following are prsentation of hepatic coma except :**
a) Asterixis b) abscent deep reflex
c) abnormal EEG d) increase ammonia
- 9- **One of the following is important manifestaion of hepatic pre-coma**
a) fever b) flapping tremors c) anemia
- 10- **Hepatic coma is precipetated by all except:**
a) hematemesis b) hypokalemia
c) high protien intake d) hyperglycemia
- 11- **The earliest sign of pre-hepatic coma is:**
a) mental and psychatric change b) jaundice c) spider naevi
- 12- **Treatment of hepatic coma**
a) neomycine b) blood transfusion c) high protein intake
- 13- **Early HCV infection may cause:**
a) fluminant hepatic failure b) liver cirrhosis c) no symptom

14-The following are common feature of chronic viral hepatitis except

- a) enlarged soft tender liver
- b) jaundice
- c) fever
- d) elevated plasma liver enzyme

15-Chronic viral hepatitis may benefits than therapy with:

- a) corticosteroids
- b) cyclosporine
- c) interferon

16-Interferon may be used in

- a) autoimmune hepatitis
- b) Chronic viral hepatitis
- c) liver cirrhosis

17-Liver chronicity is present in

- a) hepatitis A
- b) hepatitis B
- c) Both

18-The commonest cause of marked rise liver enzyme

- a) hepatitis A
- b) hepatitis B
- c) hepatitis C

19-Chronic active hepatitis occurs in

- a) hepatitis A
- b) hepatitis B
- c) hepatitis bilharziasis

20-In Chronic active hepatitis one of the following is found

- a) deep jaundice
- b) splenomegaly
- c) intermitted fever
- d) all of the above

21-Hepatitis C infection:

- a) the virus belong DNA virus
- b) short incubation period
- c) patient are often Asymptomatic
- d) chronic hepatitis is rare

22-Hepatic failure may produce:

- a) flapping tremors
- b) fine tremors
- c) intention kinetic tremors

23-Suppressive treatment for viral hepatitis include

- a) antibiotics
- b) ribavirin
- c) Both

24-Treatment of autoimmune hepatitis include

- a) ribavirin
- b) interferon
- c) corticosteroids

25-Serum of patient contain only anti HBs he is

- a) acutely infected by HEV
- b) suffering from chronic HEV infection
- c) Vaccinated
- d) low level of HBs Ag carrier

26-Interferone therapy for virus C hepatitis may result in

- a) fever
- b) depression
- c) Both

27-Which hepatitis C genotype is common in Egypt

- a) genotype 1a
- b) genotype 1b
- c) genotype 2
- d) genotype 3
- e) genotype 4

28- Which of the following hepatitis virus has no vaccination:

- a) hepatitis A virus
- b) hepatitis B virus
- c) hepatitis C virus
- d) all of these have vaccines

29- The presence of hepatic bruit over liver suggest

- a) recent liver biopsy
- b) perihepatitis
- c) hepatoma
- d) portal HTN

Cases

1- 50 years old male presented to hospital with mild jaundice and edema of lower limb & ascites and disturbed consciousness. Pulse 100 /min regular and blood pressure 110/70 mmHg with free chest and heart.

- a- What's your diagnosis?*
- b- How to manage this case?*

2- Female patient 30 years old presented with palpable tender liver & jaundice & pain in joint and muscles.

Investigation reveal elevated transaminase with positive viral marker

- a- What is your diagnosis?*
- b- Treatment you suggest?*

3- 55 years old male presented with history of HCV presented with jaundice and fever and foetor hepaticus with ascites and bleeding tendency and spider nevi & palmar erythema and flapping tremors

- a- What is your diagnosis?*
- b- Treatment you suggest?*

4- A 45 y. old female used to self-administer pain killers for her joints pain in the last 6 months started to notice yellow discoloration of her eyes. In the last 2 weeks examination revealed hepatic enlargement four fingers below the costal margin. In addition to the jaundice, there are no spider nevi & no oedema other lower limbs

- a- What is your diagnosis?*
- b- Treatment you suggest?*

5- A 48 y. old man who was in previous good health started to notice bilateral ankles oedema, easy fatigability & some few bruises in his legs. On examination, the spleen was found enlarged & the liver palpable 3 cm. below the costal margin which was confirmed by abdominal sonography which altered hepatic echogenicity. There was also mild ascites.

- a- What are the possible causes for this patient's condition?*
- b- What are other investigations needed to settle the diagnosis?*
- c- What is the explanation of this patient's bruising?*

- 6- An elderly man with established cirrhosis & ascites deteriorated recently with drowsiness, hand tremors and temperature 37.8°C.**
a- How can you explain the clinical findings?
- 7- An elderly man aged 55 y. with established cirrhosis and ascites deteriorated recently with drowsiness, hand tremors & temperature 37.8°C.**
a- What is your diagnosis?
b- Name five precipitating high factors for deterioration
c- What is your immediate diagnosis?
- 8- A 50 y. old female patient known to have liver cirrhosis +ve HBsAg since 10 years. during the last 2 months, she noticed increased fatigability & marked loss of weight. Abdominal examination showed hard hepatomegaly & ascites with audible bruit over the liver**
a- What is the most likely diagnosis?
b- What are the investigations you suggest to this patient?
- 9- A 45 y. old male farmer came to ER with disturbed level of conscious. His family said that he vomited blood before bringing him to the hospital. O/E, the pt. was drowsy & jaundiced. There were ecchymotic areas over his back, right shoulder and legs. Heart exam. Showed no abnormalities.**
a- What is your diagnosis?
b- What investigations would you like to perform to this patient?
c- What your suggested therapy to manage this patient?
- 10- A female patient, 44 y. old, presented with severe pain, tenderness & swelling of lower part of Lt. forearm & around the wrist after a sudden fall. She gave a long past history of persistent jaundice, itching & passage of dark urine & big amounts of pale stools. Examination showed many bruises & firm liver. Investigations revealed S. total bilirubin of 3 md/dl, S. calcium of 9 mg/dl & S. phosphorus of 3.5 mg/dl. X-ray revealed fracture of forearm.**
a- What is your diagnosis?
b- How can you confirm it?
c- How can you ttt the case?
- 11- A 60 y. old man was brought to ER impaired consciousness of day duaration. He had abdominal pain and vomiting for 4 days. Examination revealed a tinge of jaundice and ascites. He was delirium & his temperature was 38.4°C. Chest and heart were normal.**
a- What is the most likely causes of his delirium?
b- What is the differential diagnosis?
c- What are the investigations you need to reach diagnosis?
d- What ttt would you like to prescribe?

- 12- A 14 y. old boy came to ER for persistent vomiting and anorexia of 1 week duration. His family reported that his temperature was 38°C in the first 3 days & then subside after that. O/E, he was a febrile but jaundiced. There was some tenderness in upper abdomen but the liver was not felt. Urine was dark. Serum total bilirubin was 4 mg/dl, serum transaminases were elevated 10 folds more than normal & serum alkaline phosphatase was double its normal value.**
- a- What is your diagnosis?*
 - b- What is your differential diagnosis?*
 - c- What other test would you like to ask to reach a definitive diagnosis?*
- 13- A 16 y. old student developed malaise, anorexia, nausea & right upper quadrant discomfort. He also had dark urine & pale stools. O/E, he was jaundiced, with 2 small enlarged cervical lymph nodes. His liver and spleen were enlarged. There was no cutaneous stigmata of liver disease & no ascites. After 3 weeks his symptoms resolved & jaundice improved.**
- a- What is your likely diagnosis?*
 - b- What is your differential diagnosis?*
 - c- What are the investigations you need to establish the exact cause of the jaundice?*
- 14- A 12 y. old boy complaining of anorexia, nausea, vomiting and mild rise of temp, on the third day he developed jaundice and dark urine discoloration.**
- a- What is your probable diagnosis?*
 - b- What is the cause of dark urine?*
 - c- What is the inv. To confirm your diagnosis?*
 - d- 3 weeks later he improved, his appetite returned but unfortunately jaundice deepened, stools became pale, urine darkened, itching and diarrhea occurred. What is your new diagnosis?*
 - e- What inv. would you ask again?*
- 15- A male patient 45 y. old, presented to the reception room in hospital with delirium. His wife told you that he caught influenza and chest infection three days ago. She also told you that in the last few months he was irritable & lost concern to the family. On examination you found that he had tremors. The liver was felt 10 cm below costal margin.**
- a- What is the diagnosis?*
 - b- What is the ppt factor for present condition?*
 - c- Which types of tremors did the patient have?*
 - d- Mention other 5 causes of tremors you know*
 - e- What are the essential lines of treatment of the patient's emergency condition?*
 - f- Can you give him morphia to sedate?*
-

Oral

- 1- Sign of LCF
 - 2- Causes of hepatitis and investigation
 - 3- DD of hepatitis
 - 4- Management of hepatic encephalopathy
 - 5- Causes of hepatomegaly
 - 6- DD of pain in right hypochondrium
-

Answers

MCQ

1- A	2- D	3- D	4- C	5- A	6- A/B	7- D	8- B	9- B	10- D
11- A	12- A	13- C	14- A	15- C	16- B	17- B	18- A	19- B	20- D
21- C	22- a	23- B	24- C	25- C	26- C	27- E	28- C	29- C	

Cases

- 1) hepatic encephalopathy
- 2) auto-immune hepatitis
- 3) LCF
- 4) Primary biliary cirrhosis or lipoid hepatitis
- 5) Liver cirrhosis, Bilharziasis or GNS or secondary to virus B or C
- 6) Hepatic encephalopathy
- 7) Hepatic encephalopathy
- 8) Hepatocellular carcinoma
- 9) A case of chronic liver disease complicated by haematemesis due to rupture esophageal avarices
- 10) Most probably a case of 1ry biliary cirrhosis
- 11) Hepatic encephalopathy
- 12) Acute viral hepatitis
- 13) Hepatitis A virus
- 14) a - Acute viral hepatitis A
b- Prolonged cholestasis
- 15) Hepatic encephalopathy

Hepatology 2

(P.H. – GIT. Bleeding – Ascites – Jaundice – Amoebic abscess – Miscellaneous)

- 1- Jaundice (causes and diagnosis)
- 2- Discuss (causes, DD, types, diagn.) of jaundice
- 3- Deep jaundice with itchy (causes, DD of them)
- 4- Intrahepatic obstructive jaundice (discuss and causes)
- 5- Obstructive jaundice (causes / clinical picture / invest / DD)
- 6- Recurrent jaundice (2 causes and DD)
- 7- Clinical and lab diagnosis of obstructive jaundice
- 8- Cholestatic jaundice
- 9- Give an account on hepatic ascites (causes, pathogenesis)
- 10-Short account and causes of ascites and DD
- 11-Transudative ascites (3 causes – DD)
- 12-Ascites (hepatic ascites – pathogenesis – c/p – invest- paracentesis)
- 13-Treatment of acute variceal bleeding
- 14-Treatment of ameobic liver abscess
- 15-Ameobic hepatitis (clinical sign)
- 16-Amoebic liver abscess (+TTT)
- 17-Portal hypertension (causes, C/P, invest., TTT/ classefication)
- 18-Upper GIT bleeding
(Causes –how to management patient with bleeding from esophageal varices)
- 19-Hematemesis (causes, management)
- 20-Causes of hepatomegaly
- 21-Management of esophageal varices
- 22-DD & enlarged tender liver
- 23-Causes of bleeding per-rectum.

MCQ

1- The causes of ascites in liver cirrhosis

- a- Primary aldosteronism
- b- High salt intake
- c- Proteinuria
- d- Hypoalbuminemia

2- Itching is characteristic symptom in the following except

- a- uremia
- b- leukemia
- c- obstructive jaundice

3- All are associated with obstructive jaundice except

- a- oral contraceptive
- b- criggle- najjar type 2
- c- pregnancy
- d- 2ry carcinoma of liver

4- In obstructive jaundice bilirubin in serum mainly

- a- direct
- b- indirect
- c- both

5- Which is incorrect about jaundice

- a- caused by hemolysis
- b- caused by viral hepatitis
- c- caused by stone in GBD
- d- appears in sclera when bilirubin 1.5 mg%

6- Unconjugated hyperbilirubinemia occurs on following

- a- hemolytic anemia
- b- obstructive jaundice
- c- aplastic anemia
- d- primary biliary cirrhosis

7- Which of the following drugs has direct effect on hepatocyte

- a- acetaminofen
- b- halothane
- c- isoniazide
- d- rosuvastatin

8- The following are true regard Serum alkaline phosphatase concentration except

- a- it is derived from liver & bone & small bowel
- b- it is derived from hepatic sinusoid and canalicular membrane
- c- typically increase to more than six times normal in viral hepatitis

Cases

1- Male patient with known HCV related cirrhosis presented with abdominal pain, malaise, nausea, on examination he had moderate ascites, mild generalized abdominal tenderness. Investigation revealed HB= 11.2gm/dl , WBCs 1500/cc .Pt21/sec serum albumin 25 gm/dl , serum bilirubin 56 mg/dl

ascetic fluid for protein was 26 mg/dl WBCs 500/ML and amylase is normal

- a- What is cause of patient current clinical presentation?
- b- TTT you suggest?

- 2- 50 y female with new onset of ascites & jaundice, she gave history of previous blood transfusion, lab test show S. albumin 2.8mg/dl, platelet count 78000/mms.**
- a- What is your diagnosis?*
 - b- What is your investigation?*
 - c- How to manage?*
- 3- Male patient 55 years old presented with coma 2 days ago, he was admitted to ER for an attack of hematemesis.**
- Examination show jaundice and shrunked liver & huge splenomegaly**
- a- What is your diagnosis?*
 - b- TTT you suggest?*
 - c- Plan to prevent recurrence?*
- 4- Male patient suffer from fever , severe headache, anorexia, no jaundice and presented by pain in RT hypochondria**
- a- What is the diagnosis?*
 - b- TTT you suggest?*
- 5- Male patient 40 years old has history of recurrent hepatitis he presented with abdominal involuntary movement. On Examination, ophthalmologist discover Kayser fleischer ring**
- a- What is your diagnosis?*
 - b- TTT you suggest?*
 - c- What is investigation?*
- 6- A young man was admitted to hospital on account of collapse after passing black soft stools. His pulse was 110/min. regular, BP was 90/60 min HG**
- a- What are the points in history and clinical examination will help you reach the diagnosis?*
 - b- In spite of treatment the patient developed bouts of irritability & coma. How this development would affects management?*
- 7- A 23 y. old man was admitted in shock after vomiting a large amount of dark brown vomitus. He was transfused with three units of whole blood. Next morning he was found confused and irritable. His BP was 110/70.**
- a- What are the points in history of this patient would you like to enquire about?*
 - b- What signs would you help you to arrive to a diagnosis? Explain how?*
 - c- How would you manage this patient?*

8- A 45 y. old business man taking aspirin for occasional headache, presented with fainting and melena.

- a- What is the diagnosis?*
- b- Give one investigation to assess the case*
- c- What is your management?*

9- A 44 y. old male was admitted to hospital with a two weeks history of fever and right upper quadrant abdominal pain. There are no urinary symptoms, physical examination revealed a chronically ill appearing man with temperature 38°C, there was no jaundice, there was moderate hepatomegaly with tenderness all over the right upper quadrant & intercostal tenderness, and there was no splenomegaly. On examination of the chest there was a dullness and diminished air entry over the base of the right lung.

- a- What is the most probable diagnosis?*
 - b- Mention 2 possible D.D.*
 - c- How to reach the proper diagnosis?*
-

Oral

- 1) DD of jaundice
 - 2) Management of attack of hematemesis
 - 3) Difference between Wilson & Hemochromatosis
 - 4) Mechanism of Ascites
-

Answer

MCQ

1- D	2- B	3- B	4- A	5- D	6- a	7- C	8- C
------	------	------	------	------	------	------	------

Cases

- 1) Spontaneous bacterial peritonitis
- 2) Post hepatitis cirrhosis with PH & LCF
- 3) Hepatic encephalopathy
- 4) Amoebic liver Abscess
- 5) Wilson Disease
- 6) Black soft stool (Melena = upper GIT bleeding)
- 7) A case of upper GIT bleeding (P.U. – esophageal varices)
- 8) Most probably a case of upper GIT bleeding may be due to gastritis or peptic ulcer which is likely to occur in this case
- 9) Amoebic liver abscess. Complicated by Rt. Sided pleural effusion

Keywords for liver cases

- 1- dark urine – clay color stool = obs. Jaundice or HC jaundice
- 2- dark stool – normal urine – hemolytic Jaundice
- 3- jaundice – lower limb edema –ascites – decreased albumin = liver cirrhosis
- 4- jaundice – LL edema- ascites + decrease albumin + hematemesis = complicated portal hypertension
- 5- jaundice – LL edema – ascites - spidernevi – gynecomastia - bleeding tendency = LCF
- 6- If abdominal pain – tenderness –fever -& may be encephalopathy = SBP

كونك طبيب

فده معناه انك ممكن تتسأل في احي وقت
وخصوصا في امراض الكبد لانتشارها في بلونا - للأسف الشديد

المذاكرة بقى -غير انها بتخليك تجيب تقرير-

بتنقذ في مواقف كثير وبتخلي ثقتك في نفسك تزيد والأهم من كده انك
تكون شخص مفيد لكل اللي حواليك وتكون عند حسن ظنهم

#الطب_مهنة_سامية
#المذاكرة_مهمة

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#مراجعة_علام

GIT

(GIT diseases)

- 1- Causes & diagnosis of chronic diarrhea
- 2- Discuss acute diarrhea
- 3- Malabsorption Syndrome (C/P . investigation)
- 4- Management of steatorrhea
- 5- Causes, management and DD of acute dysentery
- 6- Dysentery (DD of 2 main causes)(C/P, management & complications)
- 7- GERD (C/P, invest., TTT)
- 8- Short account on reflux esophagus
- 9- Barrett's esophagus & mention it's risk factor
- 10- Achalasia of cardiac (Diagnosis – TTT)
- 11- Hiatus hernia (A/E – CP – complications & TTT)
- 12- C/P & TTT of PU
- 13- Diagnosis and treatment and complication of PU
- 14- C/p – complication & TTT of duodenal ulcer
- 15- Duodenal ulcer (pathogenesis – CP – management, complication)
- 16- Acute pancreatitis
- 17- Chronic pancreatitis (diagnosis)
- 18- Ulcerative colitis (+TTT)
- 19- Medical cause of acute abdominal pain
- 20- Give an account on 3 medical causes of acute abdomen
- 21- Give an account on Epigastric pain
- 22- Non-surgical recurrent abdominal pain (5 causes & DD of 2)
- 23- Causes of dysphagia
- 24- Dysphagia (causes – DD of 2 medical causes & investigations)
- 25- Repeated vomiting (diagnosis – causes – DD- investigations & TTT)
- 26- Persistal vomiting (2causes & DD between them)
- 27- Hematemesis (3 causes – DD between them)
- 28- DD of hematemesis- early management
- 29- DD of bleeding per rectum
- 30- Haematochezia (causes & discuss one of them)
- 31- Melena (Def. – Causes – investigations.

MCQ

1- Gastrin is predominant secretion from

- a- antral mucosa
- b- fundus of stomach
- c- 2nd part of duodenum
- d- jejunum

2- All are absorbed maximally in upper small intestine except

- a- Ca
- b- Fe
- c- VIT B12
- d- folate

3- Causes of intestinal malabsorption include:

- a- colonic diverticulosis
- b- Amebiasis
- c- Acromegaly
- d- intestinal lymphoma

4- Angular stomatitis present in all except

- a- vitamin deficiency
- B- malabsorption syndrome
- c- GERD

5- Malabsorption is usually presented with except

- a- flat nails
- B- hyperpigmentation
- c- nausea

6- Traveler's diarrhea can be treated by

- a- doxycycline
- b- bismuth
- c- lepromide

7- Steatorrhea is associated with the following except

- a- villous atrophy
- b- malabsorption
- c- intestinal obstruction

8- Steatorrhea is associated with

- a- increased fecal fat
- B- obesity
- c- tenesmus

9- The most reliable screening test for malabsorption is

- a- quantitative determination of fecal fat
- b- d-xylose absorption test
- c- radioactive triolein absorption test
- d- small intestine x-ray

10- Celiac disease is caused by

- A- bacterial infection
- b- Gluten hypersensitivity
- c- viral infection

11- The most specific treatment for tropical sprue is

- a- corticosteroids
- B- anti-biotic
- c- gluten free diet
- d- folic acid

12- Cardiac cachexia is due to

- a- intestinal congestion
- b- protein losing enteropathy
- c- Both

13- Cardiac cachexia may occur due to except

- a- anoxia
- b- low metabolic rate
- c- protein losing enteropathy

14- Which of the following causes bleeding diarrhea

- a- Crohn's disease
- b- U.colitus
- c- salmonella food poisoning
- d- giardiasis

15- Common feature of Crohn's disease

- a- bleeding per rectum
- b- hepatitis
- c- anal fistula

16- Ulcerative colitis causes the following is complication

- a- intestinal stricture
- b- peri-anal fistula
- c- toxic megacolon

17- Common complication of ulcerative colitis

- a- psuedopolyposis b- polyneuritis c- anal fistula

18- Complication of ulcerative colitis

- a- cancer colon b- polyneuritis c- edema of lower limb

19- Bacillary dysentery can be differentiated from UC by

- a- barrium enema B- stool smear
c- stool culture d- segmeidoscopy

20- In ulcerative colitis cause the following drugs except

- a- salazopyrine b- methotrexate c- corticosteroids

21- Retrosternal burn evoked by spicy food and relived by nitroglycerin

- a- angina b- esophageal spasm c- niether

22- Gerd is complicated by

- a-esophageal varices b- barret esophagus c- esophageal perforation

23- Reflux esophagitus without H.pylori infection is treated by

- a- omeprazole b- metronidaxole c- niether

24- The commenst cause of antral gastritis

- a- alcohol b- H.pylori infection
c- pernicious anemia d- herpes virus infection

25- Helicobacter pylori may be pre cause of

- a- ulcerative colitis b- crohns disease c- deudenal ulcer

26- Deudenal ulcer is releived by

- a- fasting b- food c- neither

27- In peptic ulcer the following drugs are contraindication

- a- corticosteroids b- proton pump inhibitor
c- antacid d- all of the above

28- Regard melena which statement is false

- a- at least 60ml of blood is required b- blood should remain at least 4 hourwith gut
c- black tarry semisolid stool d- offensive odour

29- Symptoms of acute pancreatitis include

- a- diarrhea b- bulky offensive stool
c- bleeding tendency d- none of the above

30- In pancreatitis amylase is raised in

- a- serum b- urine c both

Cases

1. **Male patient 28 years old presented with crampy abdominal pain , tenesmus, bloody mucoid stool , x-ray revealed colonic dilation and he gave history of similar attack.**
 - a- What is your diagnosis*
 - b- What are the investigations?*
 - c- TTT you suggest?*
2. **A 55 years old female presented with epigastric pain and melena & hypotension. She had history of long standing therapy of rheumatological disease.**
 - a- What is your diagnosis?*
 - b- What are the investigations?*
 - c- TTT you suggest?*
3. **Male patient presented with recurrent abdominal pain in Rt. iliac fossa relieved by defecation or passing flatus.**
 - a- What is your diagnosis?*
 - b- What are the investigations?*
 - c- TTT you suggest*
4. **A 44 y. old business man taking aspirine for occasional headache presented with vomiting and melena**
 - a- What is your diagnosis?*
 - b- What are investigations & treatment?*
5. **A 67 y. old man complains of difficulty of swallowing of 3 months duration. The condition was mild at the beginning with difficult in eating semisolid foods only. At the time of presentation he could only swallow fluid. He lost 10 kg of his weight during the illness and started to complain of hoarseness of voice.**
 - a- What is the most likely diagnosis?*
 - b- Mention 3 tools of investigation to reach definite diagnosis*
 - c- Mention 5 conditions that may lead to this definitive condition*
 - d- Give 3 diff diagnosis and how they can be exclude?*
6. **A 68 y. old man with history of progressive dysphagia to solid food of 6 months duration was admitted to hospital. On admission he could only swallow fluids. He complained also of irritative cough & chest lost 12 kg during the last 6 months. His Hb. Was 6.8 gm/dl & ESR 92 mm (1 sthr)**
 - a- What is your provisional diagnosis?*
 - b- Mention 5 points in favour of your diagnosis*
 - c- What would you investigate this case?*
7. **A 50 y. old male came complaining from dark brown urine since 2 weeks. He could notice that he has lost 8 kg of weight in the last 4 months. Examination revealed that he is markedly jaundiced; the liver is enlarged and not tender. The gall bladder could be palpated.**

8. A 45 y. male complaining from difficulty in swallowing of 4 years duration, more to fluids than solids & felt behind the lower part of the sternum, commonly accompanied by retrosternal pain with radiation both arms.
9. A 45 y. old female was suffering from abdominal discomfort & fat intolerance for the last 2 years. She developed upper abdominal pain. N & V in the last 10 days followed by abdominal distention & fever. On examination, she was pale & tetanic spasm was easily evoked in her hands, temp. 38.5 °C, a swelling was seen & felt in the epigastric region. Immediate U/S showed calculi gall bladder & dilated common bile duct & a large heterogeneous mass was seen inferior to the pancreas.
- a- How would you correlate the pt. history, physical signs & U/S findings?*
 - b- What is the cause of the patient carpal spasm?*
 - c- Do you accept the physician attitude upon conservative management of this case?*
10. A 70 y. old male, who had very regular bowel habits all over his life, started to have constipation of increasing severity in the last 6 months not responding easily usual laxatives. He lost 6 kg of weight during this period. Examination revealed exhausted face but no other signs.
11. A 23 y. old male patient with general weakness, dyspnea, bone aches & malaise. He gave a past history of palpitation, which was found to be a result of cardiac valvular lesion. General examination revealed that the patient was pale & toxic, BP was 140/50, temp. was 39°C & his pulse was 118/min.
- a- What is your diagnosis?*
 - b- What are the required investigations to reach a final diagnosis?*
 - c- How would you treat such patient?*
12. A 55 y. old male started to feel Epigastric pain both when hungry and after meals since 4 months. This was accompanied by marked loss of appetite particularly to meat, which resulted in 10 kg loss of weight at the end of this period. Examination revealed marked pallor, tender epigastric region, and enlarged left supraclavicular lymph node.
- a- What is the diagnosis?*
 - b- What is the D.D.?*
 - c- Mention investigations to be done to confirm your diagnosis*
 - d- Treatment of the case based on your most probable diagnosis*
13. A lady 58 y. of age presented with jaundice, for 1 week duration she has been complaining of flatulent dyspepsia over the last two years, occasional abdominal pain. She had marked anorexia for the last 2 weeks.
- a- What are the symptoms & signs are care to look for in this patient?*
 - b- What are the investigations required proving this diagnosis?*

Oral

- 1- Vomiting
 - 2- Diarrhea (acute-chronic)
 - 3- Constipation
 - 4- Dysentery
 - 5- Melena-hematemesis
 - 6- Bleeding per rectum
 - 7- Compare between UC & Crohn's disease
 - 8- Compare between gastric ulcer & duodenal ulcer
 - 9- DD of acute epigastric pain
-

Answer

MCQ

1- A	2- C	3- D	4- C	5- C	6- C	7- C	8- A	9- B	10- B
11- b	12- C	13- B	14- B	15- C	16- C	17- A	18- A	19- C	20- A
21- B	22- B	23- A	24- B	25- C	26- B	27- A	28- B	29- D	30- C

Cases

- 1) Crohn's disease
- 2) GIT bleeding – PU
- 3) Irritable bowel
- 4) PU
- 5) Most likely cancer esophagus → dysphasia
Is smoker → cancer lung → Mediastinal (squamous cell carcinoma)
- 6) Most probably it is a case of cancer esophagus
- 7) A case of cancer head of the pancreas complicated by obstructive jaundice & cachexia
- 8) Achalasia of esophagus
- 9) Most probably a case of acute pancreatitis complicated by hypocalcaemia and pseudo pancreatic cyst
- 10) Most probably a case of cancer colon
- 11) Most probable a case of infective endocarditis
- 12) Cancer stomach
- 13) Chronic calculous cholecystitis

Key words of GIT

- 1- Epigastric pain = PU or pancreatitis
- 2- Bulky diarrhea+ loss of vit = malabsorption
- 3- Bloody diarrhea + tenesmus + mucous in stool = dysentery
- 4- Abdominal pain increased by meal & relieved by defecation = IBS
- 5- Lower quadrant abdominal pain + non bleeding diarrhea+ urinary tract fistula = Crohn's disease
- 6- Occult blood in stool + anemia + progressive constipation = cancer colon

استعن بالله ولا تعجز

حقق أمل باباك ومامتك فيك

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#مراجعة_علام

(Introduction – C.CX – Speech – Blood supply of brain – Brain tumours)

- MCQ**

- 81

9- In monoplegia usually the site of the lesion in

- a- pons
c- internal capsule
b- cortex
d- midbrain

Cases

Diabetic female patient, 53 years old presented by symmetrical, bilateral weakness of both LL. with acute onset and regressive course. On examination there are bilateral extensor planter, hyposthesia to the level of costal margin & intact position and movement sensation

- a- what is the cause?*
b- Investigation & complication?

Female patient smoker + history of OCP with frequent chest infection then marked weakness in rt arm , difficulty on walking and difficult in expressing words

- a- What is your diagnosis?*
b- What is the most probable cause?

Oral

- 1- Blood supply of capsule**
- 2- Difference between UMNL & LMNL**
- 3- Discuss vascular lesion of hemiplegia**

Answer

MCQ

1- B	2- D	3- C	4- B/D	5- B	6- D	7- A	8- B	9- B
------	------	------	--------	------	------	------	------	------

Cases:

- 1) Anterior spinal artery occlusion**
- 2) Hemiplegia - vascular cause**

CNS 2

(Cr. Nerves – Hemiplegia – Paraplegia)

- 1- Write on 7th cranial n paralysis
- 2- Bell's palsy
- 3- Bells phenomena
- 4- Lower motor neuron facial nerve palsy (causes & C/P)
- 5- C/P of 3rd nerve lesion
- 6- trigeminal neuralgia
- 7- vertigo
- 8- Causes of hemiplegia
- 9- Causes – C/P of acute hemiplegia
- 10- Capsular hemiplegia (C/P – management - causes)
- 11- Crossed hemiplegia (site of lesion – c/p - causes)
- 12- DD of hemiplegia according to site of lesion (+C/P)
- 13- Systemic paraplegia (causes – c/p)
- 14- Causes of paraplegia
- 15- Paraplegia with sensory level
(causes – clinical feature –investigation)
- 16- Flaccid paraplegia (DD)
- 17- Focal paraplegia (aetiology – clinical diagnosis - DD)
- 18- Causes – investigations to reach diagnosis of paraplegia with level
- 19- Compression paraplegia
- 20- Causes of spastic paraplegia
- 21- Short account on compression of cord
- 22- Clinical diagnosis if extra-medullary compression of spinal cord
- 23- Clinical diagnosis of transverse myelitis
- 24- Cauda equina syndrom (causes – clinical manifestation)
- 25- Short account on urinary bladder disorder in different neurological disease

MCQ

1- All of the following are causes of hypertonia except

- a- potts disease
- b- syringomyelia
- c- shock stage
- d- disc prolapse

2- Brown sequard syndrome is characterized by

- a- contralateral deep sensory loss
- b- ipsilateral superficial sensory loss
- c- ipsilateral hemiplegia
- d- contralateral hemiplegia

3- Transient hemiplegia occurs in

- a- disseminated sclerosis
- b- mitral stenosis
- c- cerebral thrombosis

4- Causes of transient hemiplegia

- a- MND
- b- SCD
- c- todd's paralysis

5- Capsular hemiplegia is presented by except

- a- complete hemiplegia
- b- hypo-reflexia
- c- extensor planter reflex

6- Crossed hemiplegia indicate site of lesion in

- a- internal capsule
- b- cortex
- c- brain stem
- d- cervical spine

7- Weber's syndrome in crossed hemiplegia with involvement

- a- facial nerve
- b- abducent nerve
- c- oculomotor nerve
- d- vagus nerve

8- Causes of flaccid paralysis

- a- peripheral neuritis
- b- pellagra lateral sclerosis
- c- cervical spondylitis

9- Radicular sensory loss results from lesion in

- a- conus medullaries
- b- posterior column
- c- cauda equina

10- Extra- medullary compressive paraplegias characterized by all except

- a- early bladder affection
- b- painful onest
- c- asymmetrical
- d- early affection of saddle shaped area

11- Paraplegia in flexion is characterized by all except

- a- clonus
- b- automatic bladder
- c- mass reflex
- d- pyramidal & extra-pyramidal affection

12- Morvans syndrome (trophic changes) may occur in

- a- intramedullary compressive paraplegia
- b- extra-medullary compressive paraplegia
- c- both
- d- none

13- 3rd cranial nerve paralysis is manifested by

- a- ptosis
- b- myosis
- c- convergent squint

14- Corneal reflex tests the integrity of

- a- optic nerve
- b- trochlear nerve
- c- trigeminal nerve
- d- oculomotor nerve

15- Causes of unilateral facial nerve palsy include all except

- a- trigeminal neuralgia
- b- herpetic neuralgia
- c- systemic HTNs
- d- migraine

16- Signs of Bell's palsy

- a- affection of upper face b- affection of upper and lower face c- ptosis

17-The following are feature of pseudobulbar palsy

- a- exaggerated jaw reflex b-tounge atrophy c- nasal tone

18-Pseudobulbar palsy is charactrized by all except

- a- hypotonia b- bilateral +ve babinski
c- quadriparesis d- nasal tone of voice

19-10th cranial nerve palsy cause

- a- squint b- dysphagia c- dysarthria

20- LMNL of the 12th cranial nerve causes

- a- deviation of tounge to opposite side
b- deviation of tounge to same side
c-dysphagia

21- UMNL of hypoglossal nerve cause

- a- deviation of tounge to opposite side b- dysphagia d- tongue tremors

22-Regarding acute transverse myelitis which is false

- a- viral or post vaccinate b- bladder involvement is very late
c- absence of root pain d- definite upper level of sensory loss

Cases

- 1. Male 40 years old presenting by weakness in his LL. Examination reveals +ve Babinski sign with hypertonia & hyper-reflexia. There is no autonomic affection but sensation were lost in saddle area and have history of cough and hemoptysis since 2 years with night sweating**

- a- What is your diagnosis?*
b- What is investigation required to confirm the diagnosis?
c- What is TTT?

- 2. Diabetic hypertensive male patient 55 years presented with complete closure of left eye and weakness of RT side of the body**

- a- What is your diagnosis?*
b- On examination, what are other signs to confirm the diagnosis?

- 3. Male patient 23 years old complain of inability to close his LT eye. Examination reveals deviation of the LT jaw, the condition occurred after train travel with exposure to cold air**

- a- What is your diagnosis?*
b- How can you localize the site of lesion?
c- What is the TTT?

- 4. Female 28 years old presented by RT site hemiplegia with sudden onset since 2 hours, the family gave no history of any previous disease except 3 recurrent abortion. Examination reveals aphasia along with conjugated eye deviation to the LT**
- a- What is your diagnosis?*
 - b- What is investigation required to confirm the diagnosis?*
 - c- What is the artery which occluded?*
- 5. A heavy smoker lady on contraceptive pills used to feel dyspnea on moderate exertion & frequent bronchitis in winter, which she attributed to smoking in the last 2 years. She came to ER with sudden marked weakness in her Rt. arm with difficulty in walking & difficulty in expressing herself in words.**
- a- What is your diagnosis?*
 - b- What is investigation required to confirm the diagnosis?*
- 6. A 65 y. old man presented to ER. In coma, examination revealed that this blood pressure is 130/80 the heart, chest & abdomen are free. Neurological examination revealed deviation of the mouth to the Lt. side, and Rt. extensor plantar response.**
- a- What is your diagnosis?*
 - b- What is investigation required to confirm the diagnosis?*
- 7. A 55 y. old male patient came to the emergency room complaining of falling and abnormal gait and inability to walk properly with vertigo. O/E, his BP was 170/120. Neurological examination showed nystagmus & in coordination of movement. CT scan of the brain was done immediately and the pt. was given anti-coagulants. On the same evening, the pt. improved markedly and asked for discharge.**
- a- What is the most likely diagnosis?*
 - b- What is the differential diagnosis?*
 - c- What is ttt you advise?*
- 8. An 18 y. old male, presented with gradual progressive weakness of both lower limbs with diminished pain sensations at the umbilicus and localized tenderness over the mid dorsal spine associated night fever**
- a- What is your provisional diagnosis?*
 - b- Choose one lab. test to prove your diagnosis?*
 - c- What is the best radiological imaging to confirm the diagnosis?*
 - d- What is the line of treatment?*

Oral

- 1- compare intra & extramedullary compression
- 2- types and DD of hemiplegia
- 3- Bell's palsy & trigeminal neuropathy
- 4- how to detect level of hemiplegia & paraplegia
- 5- vascular lesion of hemiplegia
- 6- Brown sequard syndrome
- 7- Causes of flaccid paralysis
- 8- Argyll-Robertson pupil

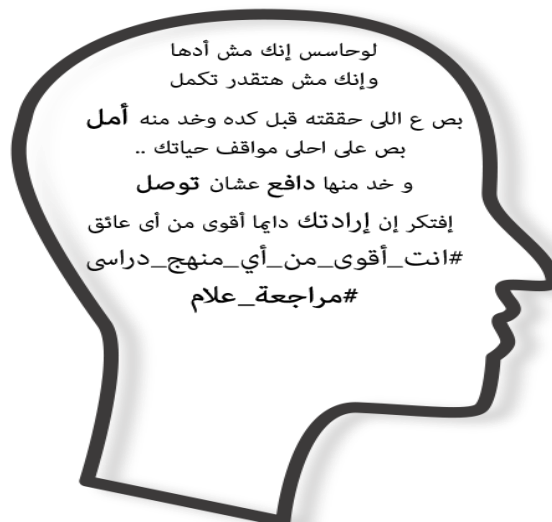
Answer

MCQ

1- C	2- C	3- A	4- C	5- B	6- C	7- C	8- A
9- C	10- A	11- A	12- A	13- B	14- C	15- A	16- B
17- C	18- A	19- B/C	20- B	21- A	22- B		

Cases

- 1) Paraplegia due to extra-medullary compression 2ry to Pott's (TB)
- 2) Crossed hemiplegia
- 3) Bell's palsy
- 4) Vascular hemiplegia... anti phospholipid syndrome
- 5) Most probably a case of cerebrovascular stroke mostly vascular in nature & leading to left hemiplegia
- 6) Most probably a case of cerebrovascular stroke mostly vascular in nature & leading to left capsular hemiplegia
- 7) A case of TIAS affected vertebrobasilar
- 8) Focal spinal cord lesion mostly due to Pott's disease



CNS 3

(P.N. – Ms. dis. – Spondylosis – MND – D.D. of wasting of small ms. Of hand – Sciatica)

- 1- Write on peripheral poly neuropathy
- 2- Diabetic neuropathy
- 3- DD of PN
- 4- DD of metabolic polyneuropathy
- 5- Infective poly neuropathy
- 6- Gullian barre syndrome
- 7- Causes of atrophy of small muscles of hand
- 8- Unilateral wasting (level of lesion and causes)
- 9- Myasthenia graves (aetiology / c-p / TTT)
- 10- C/P of myasthenia graves
- 11- Give an account on myopathy
- 12- Duchenne muscle dystrophy
- 13- DD of wasting small muscle of hand
- 14- C/P of MND

MCQ

1- Proximal muscle wasting is not produced by

- | | |
|-----------------|-------------------------|
| a- leprosy | b- GBS |
| c- polymyelitis | d- diabetic nephropathy |

2- Management of choice of GBS

- | | |
|---------------------|----------------|
| a- immunoglobulin | b- cyclosporin |
| c- corticosteroids | d- interferon |
| e- cyclophosphamide | |

3- Wrist drop is commonly seen in

- | | |
|------------|----------------|
| a- arsenic | b- alcohol |
| c- lead | d- vincristine |

4- All of the following can occur in MND except

- | | |
|---------------------------------|--------------------------|
| a- lateral sclerosing | b- pseudobulbar palsy |
| c- progressive muscular atrophy | d- peripheral neuropathy |

5- Diabetic autonomic PN characterized by all the following except

- | | |
|-------------------------|------------------------------|
| a- postural hypotension | b- quadriceps wasting |
| c- impotence | d- gastro-paresis diabetorum |

6- Myasthenia gravis can be treated by the following except

- | | |
|--------------------|----------------|
| a- corticosteroids | b- thymectomy |
| c- atropine | d- prostigmine |

7- The following lesion occur in MND

- | | | |
|--------------|---------------|---------------------|
| a- pyramidal | b- cerebellar | c- peripheral nerve |
|--------------|---------------|---------------------|

8- Signs of MND

a- polyneuritis

b- fasciculation

c- CNS affection

Cases

- 1- male patient has inability to sign his name and walks slowly and has mask face by neurological examination no sensory manifestation

a- What is your diagnosis?

b- C/P and investigation

c- TTT of the case

- 2- male patient 63 years old presented with gradual onset and progressive course of symmetrical weakness of 4 limbs of 45 years duration with marked muscle wasting and bilateral pes-cavus and there are gloves and stocks of hypoesthesia

a- What is your diagnosis?

b- C/P and investigation

c- TTT you suggest

- 3- Male patient 35 years old presented by quadriplegia and bilateral babinski sign and fasciculation

a- What is your diagnosis?

b- c/p and investigation

c- TTT you suggest

- 4- A mother presented male child 12 y. old to the clinic. She noticed that he climbs the stairs slowly & with difficulty & that he tries to support himself by grasping his legs with his hands on standing. Examination revealed that the muscles of the lower limbs are bulky, but the ankle & knee reflexes are very weak.

- 5- A 40 y. old house-wife started notice that she feels more than usual fatigue specially at the end of the day. The condition became progressively severe, so that within 6 months she can only open her eyes with difficulty at the end of the day & have difficulty in swallowing the food at the end of the meal & her voice changes if she speaks long enough

a- What is most probable diagnosis?

b- Investigations necessary to prove your diagnosis

c- Treatment in short

- 6- An 18 y. old female presented 4 weeks after a cold like illness with rapid onset of weakness the lower and upper limbs associated with glove and stocking numbness of the limbs.

a- What is the most probable diagnosis?

b- What are the possible lines of ttt?

c- What serious complication this pt. may have?

- 7- A 25 y. old female was presented to ER with deterioration in the level of consciousness. Her family gave a history that she had acute febrile illness 2 weeks before & quadriparesis that started 3 days ago. O/E, she had central cyanosis with flaccid weakness of the four limbs.
- a- What is your differential diagnosis?*
 - b- What investigations need to settle the diagnosis?*
 - c- What emergency treatment you would like to be carried out to this patient?*
- 8- A 23 y. old female developed rapid paralysis in the LL then UL within 2 days. This was preceded a week earlier with influenza like picture. O/E, the physician found glove & stocking sensory loss with flaccid paralysis.
- a- What is the diagnosis?*
 - b- What is the expected abnormality of the CSF on lumbar puncture?*
 - c- What is the treatment of choice?*
- 9- A 26 y. old female began to complain insidiously of abnormal fatigability. She begins the morning well with the fatigability increasing by effort. She also complains of double vision while her mother noticed drooping of the daughter's upper eyelids. She presented to the physician when she noticed increasing difficulty in swallowing while eating. When she had an injection in the physician's clinic, all her symptoms improved dramatically.
- a- What is the most probable diagnosis?*
 - b- What injection is given to improve her that quickly?*
 - c- What are the required investigations to confirm the diagnosis?*
 - d- Discuss the management of this condition*
-

Oral

- 1- DD of PN
- 2- Causes of lost ankle preserved knee reflex
- 3- Sciatica
- 4- How to diagnosis subacute combined degeneration
- 5- Difference between duchenne and bakers myopathy
- 6- Gower (sign – test – diagnosis)

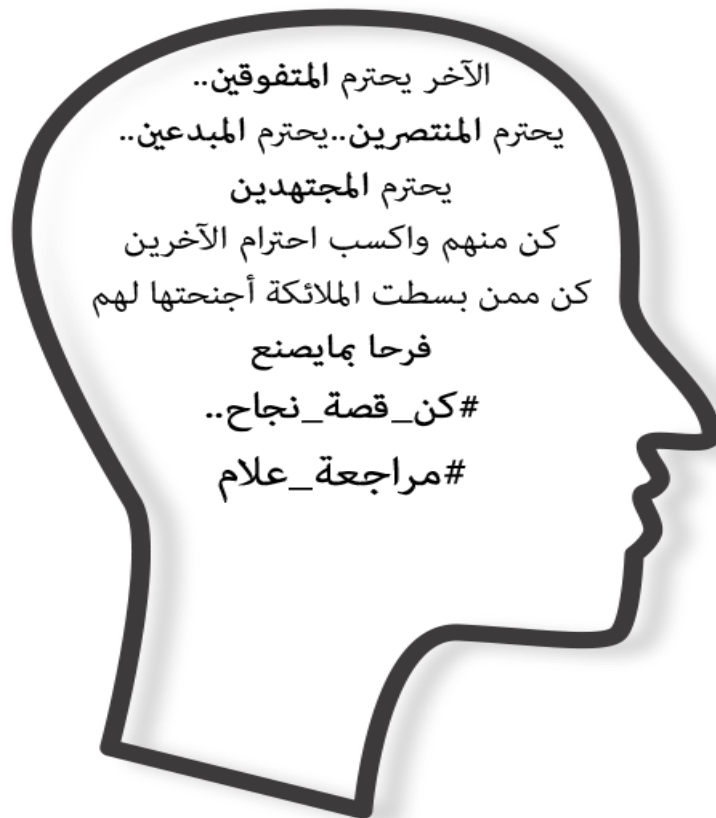
Answer

MCQ

- 1) A
- 2) A
- 3) C
- 4) D
- 5) B
- 6) C
- 7) A
- 8) B

Cases

- 1) Parkinsonism
- 2) Peroneal muscle atrophy
- 3) MND
- 4) Most probably a case of muscular dystrophy mostly duchenne
- 5) Most probably a case of myasthenia gravis
- 6) G.B. syndrome
- 7) Guillian - barre syndrome
- 8) Guillian - barre syndrome
- 9) Myasthenia-gravis



CNS 4 & CNS 5

**Headache – epilepsy – Meningitis – Encephalitis – Comas
SA he - Cerebellar ataxia - M.S. - extra pyramidal diseases**

- 1) Give an account in headache (+ causes)
- 2) Migraine (theories – c/p – TTT)
- 3) C/P and TTT of grand mal epilepsy
- 4) Temporal lobe epilepsy
- 5) Partial seizure (def - types – presentation)
- 6) (TTT – types – C/P) of epilepsy
- 7) Anti-epileptic drugs
- 8) TTT of status epilepticus (management)
- 9) Repeated seizures
- 10) Causes of meningitis
- 11) Septic meningitis
- 12) Clinical diagnosis of septic meningitis
- 13) Meningococcal meningitis (aetiology – c/p – prophylaxis - investigations)
- 14) Encephalitis
- 15) Increase ICT (c/p – investigation)
- 16) Parkinsonism
- 17) Chorea
- 18) Involuntary movement
- 19) Abnormal movement (causes – c/p – DD)
- 20) Ataxia (causes – c/p – investigation – types - DD)
- 21) Causes - management – C/P) of sub arachnoid age
- 22) Cerebellar ataxia (causes & C/P) and describe the neocerebellar manifestation
- 23) MS (clinical picture – investigation - TTT)

MCQ

- 1- Statics tremors occur in
 - a- parkinsonism
 - b- marie's ataxia
 - c- friedrich ataxia
- 2- Parkinsonism is caused by lesion in
 - a- caudate nuclus
 - b- olive nuclus
 - c- internal capsule
 - d- red nuclus
 - e- none of the above
- 3- Clinical picture of parkinsonism
 - a- spasticity
 - b- bradykinesia
 - c- poly-neuritis
- 4- Pattern of gait in parkinsonism
 - a- spastic
 - b- shuffling
 - c- drunken
- 5- All are true about chorea except
 - a- irregular
 - b- jerky
 - c- present during sleep
 - d- semipurpose
- 6- In grand mal epilepsy there is
 - a- clonic convulsion
 - b- amnesia
 - c- automatism
- 7- Psychomotor epilepsy there is
 - a- motor fits
 - b- automatism
 - c- akinesia
- 8- Simple partial motor seizure is diagnosed by
 - a- MRI
 - b- EEG
 - c- CT scan
- 9- Patient with migraine may need
 - a- CT brain
 - b- lumbar puncture
 - c- none
- 10- Causes of acute headache include the following
 - a- psychogenic
 - b- subarachnoid Hge
 - c- hydrocephalus
 - d- all of the above
- 11- The most common cause of meningitis is
 - a- viral
 - b- TB
 - c- bacterial
 - d- malignancy
- 12- Which is not a symptoms of raised ICT
 - a- altered consciousness
 - b- headache
 - c- non-projectile vomiting
 - d- convulsion
- 13- Viral meningitis is characterized by the following CSF changes
 - a- mononuclear cell
 - b- increase protien
 - c- decrease sugar
 - d- xanthochromia
- 14- CSF protien level is normal in the following type of meningitis
 - a- viral
 - b- TB
 - c- bacterial
 - d- malignancy
- 15- Chemoprophylaxis of meningococcal meningitis the best drug is
 - a- rifampicin
 - b- chloramphenicol
 - c- sulphonamides
- 16- Prophylaxis of meningococcal meningitis
 - a- vaccination
 - b- gentamycin
 - c- rifampicin

17-Viral encephalitis is most commonly due to

- | | |
|-------------------|------------------|
| a- HIV | b- poliomyelitis |
| c- herpes simplex | d- herpes zoster |
-

Cases

Young female with unilateral throbbing headache preceded by flash of light and nausea and vomiting

- a- What is your diagnosis?*
- b- What are investigations?*
- c- TTT you suggest*

2- Male patient has history of splenectomy presented by headache and neck rigidity

- a- What is your diagnosis?*
- b- What are investigations?*
- c- TTT you suggest*

3- A male patient aged 65 y. presented with gradually increasing difficulty in signing his name. O/E, no weakness, reflexes changes or sensory abnormalities were recorded. His blood pressure was 100/60 mmHg and his pulse showed multiple extra systoles.

- a. Would you expect any changes of muscle tone? If so which tone abnormality would you expect mostly?*
- b. What further sign would you expect to detect in this patient during the next year if she is not treated?*
- c. During the management of this case, is percussion of lower part of the abdomen important, and why?*

4- A married female aged 20 y. developed irregular involuntary movements of the left upper limb. The proximal muscle was more affected than the distal. Her husband reported that she became irritable & dropped objects from her hand.

- a. What would you expect to find on neurological examination?*
- b. What further manifestations would you like to search for to help define the type and cause of this complaint?*
- c. What treatment would you give her?*
- d. What is the prognosis in this case?*

5- A 60 y. old man presented in coma and urinary incontinence since one week. His coma varies in severity during this period with occasional hours of improved consciousness. He is neither diabetic nor hypertensive, but one month ago he fell in the bathroom and wounded his head with a resulting hematoma which resolved after 2 weeks. Examination revealed semi-coma and the reflexes are exaggerated all over but there are no other signs.

- a- What is your diagnosis?*
- b- What are investigations?*
- c- TTT you suggest?*

- 6- A 21 y. medical student presented to you with a complaint of frequent severe headache at 2 yrs intervals. On each occasion the headache was associated with nausea, vomiting and prostration & was preceded by flashing stars in her eye, worse in her left although not as severe, on examination there was no abnormality & all investigations were –ve
- What is the cause of this girl's headache?*
 - As simple analgesics are seldom effective, how would you manage the problem?*
 - Why is this type of headache preceded by flashes?*
- 7- A 30 y. old man presented with severe headache, vomiting, pyrexia and photophobia of 2 days duration. O/E, there was neck stiffness but his heart, chest and abdomen were free except for a scar of previous splenomegaly 5 years previously following a road traffic accident. There was no neurological focal signs and his fundi normal
- What is the most logical provisional diagnosis?*
 - Mention 2 other possible diagnosis*
 - Mention a single investigation that can confirm the diagnosis and mention the characteristic findings in each?*
 - What treatment should be given immediately for the most logic diagnosis?*
- 8- A 14 y. old boy was brought to the hospital with rapid deterioration in the level of consciousness. His family said that he had fever & sore throat few days ago. He had convulsion over the last day. O/E, his temperature was 39°C & his blood pressure was 80/40. There was neck rigidity & bilateral +ve extensor planter reflex.
- What is your diagnosis?*
 - What is your differential diagnosis?*
 - What investigations you would like to do to reach a final diagnosis?*
 - What antibiotics would you like to give to this patient?*
- 9- A 12 y. old boy was admitted with a 2 days history of fever, headache & vomiting. Consciousness was impaired. His temp. was 39.7°C, Petichae were present in conjunctiva & palatal mucosa. There was neck stiffness with +ve Kering's sign. The pupils were seen dilated, fixed & unresponsive to light. The eye was deviated laterally & didn't move vertically. Fundi were normal.
- What is your most likely diagnosis?*
 - What is your D.D.?*
 - What is the cranial nerve lesion?*
 - What investigations would you like to order*
 - What treatment would you prescribe?*

10- A 20 y. old male was brought to ER for rapid deterioration of his level of consciousness over the last 24 hours. He had been febrile for the last 3 days with some soreness in throat & was diagnosed by a general practitioner as having flue. O/E, his BP was 90/60 mmHg & his temp. was 39°C. There was neck stiffness. Hemorrhagic rash was noticed on the skin. Chest, heart and abdominal examination showed no abnormalities. There was polymorph nuclear leukocytosis in his CBC.

- a. *What is your diagnosis?*
- b. *What is your differential diagnosis?*
- c. *What other tests would you like to ask to reach a definitive diagnosis?*

11-A 60 y. old male presented with pain in the lower limb joints with difficulty on walking and stiffness. His wife noticed that his speech changed & seem very slow in everything. O/E, he appeared masked with general flexion and static tremors. There was also rigidity of the four limbs.

- a. *What is you diagnosis?*
- b. *What are the expected types of rigidity in this condition?*
- c. *What is the best medicine that can be prescribed?*
- d. *Can surgery be of any help to his symptoms?*

Oral

- 1) Headache
- 2) Water-house friedreichson \$
- 3) Coma
- 4) Status epilepticus
- 5) Value of CSF analysis in diagnosis of neurological diseases

Answer

MCQ

1- A	2- E	3- B	4- B	5- C	6- A
7- B	8- B	9- C	10-B	11-C	12-C
13-B	14-D	15-A	16-C	17-C	

Cases

- 1) Migraine
- 2) meningococcal meningitis
- 3) Parkinsonism
- 4) A case of chorea
- 5) Chronic subdural haematoma
- 6) Migraine
- 7) Meningococcal meningitis
- 8) Meningococcal meningitis complicated by acute adrenal insufficiency
- 9) Most probably a case of encephalitis
- 10) Meningococcal meningitis complicated by purpura fulminant or meningioencephalitis
- 11) Parkinsonism

Key words for CNS cases

- 1) Weakness of LL + micturition troubles + no cranial nerve affection = paraplegia
- 2) LMN facial paralysis after cold exposure = Bell's palsy
- 3) Increase ICT + sever HTNS + no meningeal sign = intracranial He
- 4) Back pain + loss of lordosis = disc degeneration
- 5) Increase ICT + fever + rash + neck rigidity = meningitis

اتعلم تعمل كل حاجة بطريقة مميزة
لهذاكر عشان أكون راضي عن نفسي
وعشان أنا مش حد عادي
أنا مش تحت رحمة الامتحان ..
الامتحان ف حياتي مجر و خطوة
#انت_ابداع_ربنا_في_أرضه
#مراجعة_علام

RENAL 1

(G.N. – Nephrotic – Nephritic – ARF – CRF)

- 1- GN (c/p –causes- diagnosis)
- 2- Management of post streptococcal GN
- 3- IgA nephropathy
- 4- C/P & laboratory inves. Of acute GN
- 5- Discuss clinical and laboratory diagnosis of nephritic syndrome
- 6- Causes and diagnosis of nephrotic syndrome
- 7- Pathogenesis and TTT of edema of nephrotic syndrome
- 8- Clinical and biochemical change in Renal failure
- 9- Acute tubular necrosis
- 10- Acute renal failure
- 11- Discuss clinical & biochemical change of oliguric, polyuric phase of ARF
- 12- ARF (causes, investigation)
- 13- In ARF explain oliguric phase & polyuric phase
- 14- Causes of CRF
- 15- End stage of chronic kidney dis.(C/P, management)
- 16- CRF (calcium, ph, abnormality)
- 17- CRF. Explain vomiting, anemia, odema, itching, coma, bone aches
- 18- C/P & biochemical inves. Of chronic renal failure
- 19- Pathogenesis, TTT of CRF odema & nephroticsty
- 20- DD of polyuria
- 21- DD of oliguria
- 22- Discuss proteinuria
- 23- Enumerate causes of hematuria
- 24- DD of proteinuria
- 25- Granular cast
- 26- Short account in renal edema (types & DD of them)

MCQ

- 1- In acute papillary necrosis there is**
a- pyuria b-polyuria c- loin pain
- 2- In ARF the following are correct except**
a- anuria b- Ut obstruction
c- hematuria d- hypophosphatimia
- 3- Oliguria more likely to be due pre-renal failure than intrinsic renal failure**
a- urine free of red blood loss
b- urine plasma urea ratio <3
c-urine osmolarity <350mosml/kg
d- urine sodium >10 mmol/l
e- in the prescence of HTNs ,raised JVP & go peripheral circulation
- 4- All the following types of GN cause nephrotic syndrome except**
a- minimal GN b- membranous GN
c- focal segmented d- cresentic GN
- 5- TTT of oliguric phase of acute renal faliure include all except**
a- restriction of diatery protein to 40 gm/day
b- calcium gluconate to reduce hyperkalemia
c- restriction of fluid intake to be total volume of daily loses
d- avoidance of dilaysis if pulmonary edema supervers
e-tetracycline therapy if anterocolis supervers
- 6- Minimal change nephropathy**
a- is the commonest cause of nephrotic syndrome in childhood
b- doesn't relapse after remission
c- causes depression of the serum complement level
d- must always be formed by renal biobsy
- 7- Drugs causes nephrotics syndrome**
a- penicillin b-metronidazole c- ampicillin
- 8- Heavy proteinurea is a feature of**
a-nephrotic syndrome b- nephritic syndrome
c-pyelonephritis d- typhoid fever
- 9- Typical feature of acute glomerulonephritis include**
a- bilateral renal angle pain and tenderness
b- hypertension and periorbital and fascial edema
c- oliguria <800 ml and hematuria
d- history of allergy with edema of lips
- 10- Protenuria more than 3 gm/ day is a feature of all the following except**
a- cardiac faliure b- nephrotic syndrome
c- minimal lesion glomerulonephritis d- chronic pyeonephritis
- 11-Acute GN is caused by**
a- beta hamolytic streptococci b- strept. Fecalis c- staph aurus

12- AGN is not characterized by

- a- macroscopic hematuria
- b- systemic HTNs
- c- massive preteinuria
- d- oliguria

13- Blood urea in acute glomerulonephritis

- a- sever rise
- b-moderate rise
- c- abscent
- d- none of the above

14- Acute GN include all except

- a-hyper-aldostrenoism
- b- high cholesterol level
- c- hypoalbuminuria
- d- none of the above

15- Patient presented by hemoptysis& RF witj anti-basement membrane antibody has

- a- Good pauster syndrome
- b- wengers syndrome
- c- interstitial nephritis
- d- henoch-scholien purpura

16- All are true about minimal changes GN except

- a-selective protenuria
- b-IgG deposition in the mesangiam
- c-common in age group 2 years
- d-respond to steroids

17- Minimal change GN is characterized by :

- a- protenuria
- b- HTNs
- c- microscopic hematuria

18- Minimal change GN is characterized by

- a- symptomless hematuria
- b- HTNs
- c- microscopic hematuria

19- GN that develop in AIDS is

- a- focal segmented GN
- b- MP GN
- c- IgA nephropathy
- d- good posture

20- Thickning of basment memebrane of glomerulus is seen in

- a- IgA nephropathy
- b- membrane-proliferativeGN
- c- lipid nephrosis
- d- post- streptococcal GN

21- Rapidly progressive GN is

- a- focal segmented
- b- cresentric
- c- membranous
- d- membrano-prolifrative

22- The commenst glumerulopathy all over the world is

- a- IgA nephropathy
- b- post- streptococcal GN
- c- MPGN
- d- FS GN

23- Recognized feature for glomelural disease include all except

- a- HTN
- b- asymptomatic
- c- hypokaemia
- d- hyperuricemia
- e- recurrent hematuria

24- In chronic glomerulonephritis which of the following is true

- a- the urine is smoky
- b- there is polyurea
- c- BP is low
- d- none of the above

25- Renal tubular acidosis may be caused by

- a- GN
- b- renal artery stenosis
- c- pyelonephritis

26- Which of the following doesn't give red urine

- a- haemoglobinuria
- b- myoglobinuria
- c- microscopic hematuria
- d- acute intermittent porphria

27- Polyuria with high specific gravity present in

- a- GN
- b- pyelonephritis
- c- DM
- d- DI

28- Poly-uria may occur in the following

- a- hypercalcemia
- b- hypocalcemia
- d- hyperkalemia

29- Risk factor for renal calculi

- a- renal tubular acidosis
- b- myxedema
- c- hypoparathyroidism
- d- DM

30- All of the following stones are radio-opaque except

- a- cystine
- b- oxalate
- c- urate
- d- phosphate

Cases

1- 35 years old man developed bilateral loin pain and frank hematuria, his symptoms started 24 hours after developing a sore throat his BP 138/88 mmHG. Urine analysis is positive for blood (+++++) & protein (++)

- a. What is the diagnosis?
- b. Investigation & TTT

2- 55 years old male presented with acute chest pain then shock examination and investigation revealed oliguria & urea and creatinine

- a. What is the diagnosis?
- b. Investigation after manifestation?
- c. TTT you suggest?

3- A 12 y. old girl presented after a fit. O/E, she was puffy, BP 170/100, temp. 37.5°C, exam. Of the chest, heart, abdomen and nervous system were unremarkable. Her mother gave a history of fever & sore throat 10 days previously for which the girl received 3 injections of penicillin.

- a. What is the most important bed-side test you should do? Explain the significance of the findings.
- b. Explain the line of treatment you should advise
- c. What particular complications would you like to look for? Describe their early diagnosis and your response to their development?

4- A young girl aged 6 y. presented with pyrexia, rigors, headache, vomiting and left loin pain. She had a history of 2 similar attacks in the last few months.

- a. What is the only important investigation you recommended in such circumstances?
- b. What is the probable cause of fever in this case?
- c. What further investigations you advise to reach final diagnosis?

- 5- A 34 y. old lady presented with one week's history of severe vomiting, malaise, hic cough, nocturia, and bilateral loin pain. Little urine had been passed over the last two days. These loin pains had been occurred during her two pregnancies, together with dysuria, and were also repeated 3 times over the last 2 years. On each occasion she was given an antibiotic. O/E, she looked ill, flushed, dehydrate, but a febrile. BP 100/70, pulse 100/min regular, bilateral loin tenderness was present. Heart & lungs were free. Investigations showed: Hb 14.2 gm – WBCs 16000.cmm – urea 412 mg% - plasma sodium 120 mEq/L – K 8.3 mEq/L – Hco3 10 mEq/L/
- What are the 4 essential points that necessitate immediate management in this pt?*
 - How will you achieve your aim to correct them?*
 - Could you advise I.V.P.? Give reasons*
- 6- A 35 y. old man presented with chest pain, marked dyspnea and fatigue due to one week duration. Three months earlier he had anorexia, vomiting & hiccough. Ten years earlier he was operated up on for bilharzial ureteric strictures. Examination revealed marked pallor, air hunger, tremors of hands, BP 160/120 and an extensive pericardial rub heard.
- What is your diagnosis?*
 - How would you investigate this patient?*
 - Discuss the management*
- 7- A 54 y. old male on irregular ttt of hypertension presented complaining from general fatigue dyspnea, polyuria & mild diarrhea in the last 2 weeks. Examination revealed BP 210/120, heaving apex & mild skin dehydration. Urine examination revealed pale color & 1+ albumin
- 8- A 67 y. old man attended his physician with persistent vomiting as the main complaint over the last 4 months, he sometimes vomit food eaten many hours previously. His past medical history is unremarkable except for some dyspepsia. There are no abnormalities to find O/E except a succession splash in the upper abdomen. His BP was 100/60, blood urea was found to be 98 mg/dl, creatinine was 8 mg/dl, sodium was 128 mmol/l, chloride was 82 mmol/l and bicarbonate was 40 mmol/l.
- What is your provisional diagnosis?*
 - How to diagnose the case and etiology?*
 - What is your comment on the bio-chemical findings?*
- 9- A 54 y. old female patient came to the hospital with post-traumatic fracture femur. She was admitted & received proper surgical treatment, analgesics & antibiotics. Few days later, the patient became febrile, with cough & expectoration & another antibiotic & antipyretics were added. The patient improved dramatically, however 2 weeks after admission she developed oliguria & mild edema with skin rash & arthralgia. Her urine analysis showed microscopic hematuria, mild proteinuria, increased WBCs & increased eosinophils. CBC showed some eosinophilia. Her renal functions were impaired with serum creatinine 5.6 mg/dl & BUN 63 mg/dl.
- What is the most likely diagnosis?*
 - What is your differential diagnosis?*
 - What investigations would you like to do to reach a diagnosis?*

- 10- A 65 y. old obese female complained of swelling of both lower limbs of 2 weeks duration. She also developed easy fatigue and dyspnea on very mild exertion. She gave a past history of DM & hypertension for the past 23 years for which she was receiving irregular treatment. She also had joint pains for 7 years, specially affecting the knee joints, and was received by continuous analgesic use. O/E, her BP was 190/110, pulse was 92/min, and she was a febrile. There was marked pallor and bilateral lower limb edema.**
- What is your provisional diagnosis and differential diagnosis?*
 - What investigations would you like to order?*
 - What is the line of treatment that you suggest to such patient?*
- 11- A 60 y. old lady presented with fatigue, shortness of breath and swelling of both lower limbs. She had a long history of DM on irregular treatment with sulfonylurea drugs. Over the last 3 years she used to take NSAIDs for her joint pains. O/E, she was pale and had congested neck veins. Investigations revealed normocytic normocromic anemia (Hb: 9 gm/d) and her serum creatinine was 4 mg/dl**
- What is the probable diagnosis?*
 - How would you investigate such a case?*
 - What is your suggested treatment?*
- 12- A 23 y. old male presented with shortness of breath & puffiness of his eyelids for the last days. He noticed that his urine became reddish & of little amount & diminished frequency. O/E, he was pale & mild pitting edema of both legs. His BP was 150/100, pulse rate 90/min. & temp. was 37°C. urine examination revealed leukocyturia. Hematuria & red cells cast.**
- What is the most probable diagnosis?*
 - What investigations would you order for this pt.?*
 - How would you treat this pt.?*
- 13- A 55 y. old male presented with gradually increasing easy fatigability, shortness of breath on effort & numbness of both hands & feet for the last few months. He gave a history of subtotal gastrectomy for gastric carcinoma many years ago. General examination revealed only pallor. Chest examination was unremarkable while cardiac examination revealed ejection systolic murmur over the base. Apart from the scar of the previous surgery, abdominal examination revealed no abnormality.**
- What is the most probable diagnosis?*
 - What investigations would you order to reach the final diagnosis?*
 - How would you treat this pt.?*
- 14- A 50 y. old lady came to you saying that she was feeling unwell for the last few months. She complained of pedal edema, malaise, fatigue loss of energy, loss of appetite, nausea and vomiting, nocturnal and polyuria. O/E, there was pallor, itching marks, soft pitting edema in the lower limbs. BP was 200/110 mmHg. Cardiac exam. showed friction sound and soft systolic murmur over the base.**
- What is your diagnosis (explain why)?*
 - How would you investigate this case?*
 - What are the lines of management?*

Oral

- 1- DD of hematuria
- 2- DD of protienuria
- 3- DD of polyurea
- 4- Causes & TTT of ARF
- 5- Causes / cp / TTT of GN
- 6- Diffrence between nephritic and nephrotic

Answer

MCQ

1- C	2- D	3- A	4- D	5- D	6- A	7- A	8- A	9- B	10- D
11- A	12- C	13- B	14- B	15- A	16- B	17- A	18- C	19- A	20- B
21- B	22- A	23- C	24- A	25- B	26- C	27- C	28- A	29- A	30- C

Cases

- 1) IgA nephropathy
- 2) ARF (pre renal)
- 3) Nephritic syndrome
- 4) Acute pyelonephritis
- 5) Congenital polycystic kidney
- 6) Chronic renal failure on top of obstructive uropathy → pericarditis & pericardial effusion
- 7) Chronic renal failure on top of longstanding & uncontrolled hypertension
- 8) Pyloric obstruction complicated by pre renal failure
- 9) Acute renal failure
- 10) Chronic renal failure due to HTN, DM, analgesic
- 11) Chronic renal failure (by combined analgesic nephropathy with or without diabetic nephropathy)
- 12) Acute glomerulonephritis (Nephritic \$)
- 13) B12 deficiency (megaloblastic anemia)
- 14) CRF

كل ساعة بتذاكرها مهمة جدا.. ممكن تكون فيها بتذاكر سؤال لبيجي ف الامتحان
سيب الفيس والواتس .. انسى أحلام اليقظة شوية .. ملكش دعوة بأي مشكلة حواليك
السنة دي كانت طويلة .. ولوقتي خلاص ..

#نصبوا_الصوان

#كل_دقيقة_بتفرق

#مراجعة_علام

(Pyelonephritis – Interstitial nephritis – Electrolytes – PH – Rheumatoid arthritis)

- MCQ**

- 105

7- Which of the following is correct for CRF

- a- anemia
- b- hyperparathyroidism
- c- metabolic acidosis
- d- flapping tremors
- e- vomiting
- f- all of the above
- g- none of the above

8- Granular cast is presented in

- a- ARF
- b- Oxalate stone
- c- GN
- d- nephrotic syndrome

9- In CRF the following is present except

- a- acidosis
- b- hypokalemia
- c- hyperuracemia

10- Manifestaion of uremia include

- a- polycythemia
- b- hypokalemia
- c- acidosis
- d- hypophosphatemia

11- Typical biochemical features of CRF include

- a- hypophosphatemia
- b- hypercalcemia
- c- metabolic acidosis
- d- proteinuria > 3.5 gm/day

12- Metabolic acidosis with high anion gab is seen in all of the following except

- a- lactic acidosis
- b- ARF
- c- keto acidosis
- d- ammonium chloride poisoning

13- Hemodialysis is indicated in a patient with renal failure when

- a- there is fluid over load
- b- serum K is 5 mEq/L
- c- serum creatinine 3 gm/dl

14- Peritoneal dialysis is preferable than hemodialysis in

- a- ascites
- b- hemophilia
- c- emergency
- d- obesity

15- Acute pyelonephritis usually caused by

- a- E-cloi
- b- staph
- c- pseudomonas
- d- proteins

16- All of the following may cause respiratory alkalosis except

- a- pulmonary embolism
- b- hysterical hyperventilation
- c- high altitude
- d- Conn's disease

17- Hypokalemia can be induced by

- a- CRF
- b- addison disease
- c- excessive use of spironolactone
- d- none of the above

Oral

- 1) Causes of CRF
- 2) Hypokalemia
- 3) Hyperkalemia
- 4) Hypo-hyper/calcemia
- 5) Hyponatremia
- 6) Metabolic acidosis and alkalosis

Answer

MCQ

1- B	2- D	3- A	4- A	5- C	6- E	7- F
8- A	9- B	10- C	11- C	12- D	13- A	14- B
15- A	16- D	17- D				

Key words for renal cases

- 1- RBCs + oliguria + HTN + Edema = nephritic syndrome
- 2- Lipid cast + increase cholesterol = nephrotic syndrome
- 3- DM + edema + anorexia & nausea = renal failure
- 4- Anorexia + nausea , fatigue , high urea & creatinine , hypokalemia , hypoglycemia = ESRF

هفاكر عشان وه واجبي ولوقتي بعد طاعة ربنا

ولو مفاكر تش انا عايش ليه ؟؟

من الاخر هفاكر عشان مفيش بديل غير اني افواكر

#مراجعة_علام

Rheumatology

- 1- Clinical picture and diagnosis of RA & TTT
- 2- New criteria for diagnosis of RA
- 3- Extra-articular manifestation of RA
- 4- TTT of RA
- 5- Compare of RA & rheumatic fever as regard c/p & invest
- 6- Articular manifestation of RA
- 7- Rheumatoid nodule
- 8- DMARDS in RA
- 9- Pulmonary manifestation of RA
- 10- Clinical manifestation of SLE & management
- 11- Lab diagnosis of SLE
- 12- Treatment of SLE
- 13- Lupus nephritis
- 14- Pulmonary manifestation of SLE & renal complication
- 15- Antiphospholipid syndrome
- 16- Seronegative arthritis
- 17- C/P & mangement of acute gout
- 18- Acute gouty arthritis(c/p- TTT)
- 19- Chronic gout
- 20- Chronic hyper uricemia
- 21- Dermatomyositis
- 22- Limited scleroderma
- 23- Scleroderma (C/P, TTT)
- 24- Systemic sclerosing (c/p- complication)
- 25- Heberden's nodule
- 26- Neurological manifestation of Collagen disease
- 27- Erythema nodosum
- 28- Raynad phenomenon
- 29- Neurological manifestation of Behcet dis.
- 30- Investigation for collagen Disease
- 31- DD of mono arthritis
- 32- DD of poly arthritis
- 33- Causes of polyarthritis
- 34- Low back pain (causes & DD)

MCQ

1- Articular cartilage is characterized by all of the following except

- a- composed of chondrocyte
c- rich in proteoglycose
- b- extremely vascular
d- devoid of a nerve supply

2- The following statement right except

- a- anti-Ds DNA +ve SLE
- b- HLA B27 associated with AS
- c- painful oral ulceration associated with Behcet's disease
- d- anti RNP -ve in mixed CT dis.

3- Hematological manifestation associated with arthritis include all of the following except

- a- iron deficiency anemia
b- sickle cell anemia
c- leukemia
d- hemophilia

4- The following are non organ spread auto-immune disease

- a- pernicious anemia b- SLE c- RA

5- Polyarthrititis is feature of

- a- gonorrhea b- TB c- RA

6- Poly-arthritis is said to be present when joint affected are

- a- > one joint c- >4 joint
b- 2-4 joint d- all joint

7- Morning stiffness is manifestation of one the following

- a- SLE b-RA c-scleroderma

8- Erosive arthritis is common in

- a- SLE b- RA c- systemic sclerosing

9- Ulnar deviation in RA due to affection of

- a- MCP joint b- MTP
c- PIP d- DIP

10-In RA the cervical spine affection include

- a- atlanto-axial subluxation
b- vertebral artery compression
c- spinal cord compression
d- all of the above

11- Acute pain & swelling of calf muscle in patient with RA is due to

- a- achillis tendonitis
c- Anserine bursitis

12- Rheumatoid nodule are characterized by all except

- a- big b- fixed to skin
c- tender d- ulcerative

13- Extra- articular manifestation of RA include the following except

- a- peripheral neuropathy
c- G.N
- b- amyloidosis
d- pleural effusion

14- In RA the following systemic manifestation may be found

- a- pericarditis
b- amyloidosis
c- aortic incompetence
d- all the above

15- The following are associated with RA except

- a- osteoporosis
- b- splenomegaly
- c- pulmonary fibrosis
- d- dementia

16- Measurement of disease activity in RA depend on

- a- pain scale
- b- morning stiffness
- c- deformity

17- Feltys syndrome include all except

- a- normal spleen
- b- +ve RF
- c- leg ulcer
- d- skin pigmentation

18- Sero –ve arthropathies commonly affected the following joint

- a-sacro- iliac
- b- spine
- c- both

19- Enthesitis commonly in

- a- sero +ve arthritis
- b- sero –ve arthritis
- c- both

20- Sero –ve arthritis are all this disease except

- a- reactive arthritis
- b- bahcet s disease
- c- Rieters disease
- d- psoriatic arthritis

21- Planter fascitis occur in

- a- RA
- b- Rieters disease
- c- both

22- In Rieter disease all may present except

- a- iridocyclitis
- b- circinato balanitis
- c- morning stiffness
- d- arthritis

23- In Rieter disease all may present except

- a- arthritis
- b- conjunctivitis
- c- cerebritis
- d- urethritis

24- Rheumatoid factor usually absent in

- a- Rieter S syndrome
- b- RA
- c- SLE

25- Behcet's disease may occure due to

- a- viral infection
- b- auto immunity
- c- cross reactive
- d- all
- e- niether

26- Erythema nodusum is present in

- a- sarcoidosis
- b- Bahcet's disease
- c- both

27- Relapsing uvetis is the charactristic of

- a- scleroderma
- b- Behcet disease
- c- sjogran S

28- Behcet's disease is characterized by the following except

- a- oro-genital ulcer
- b- thrombotic manifestation
- c- arthargia
- d- skin atrophy

29- Raynaud s phenomenon is presented in following except

- a- SLE
- b- osteoarthritis
- c- Behcet's disease

30- Behcet's disease is associated with

- a- HIAb51
- b- HLAB27
- c- HLADR4
- d- HLA B3

31- Pathergy's test is helpful in

- a- ankylosing spondylitis b- Behcet's disease d- scleroderma

32- Oral ulcer of Behcet's disease first TTT by

- a- topical TTT b- systemic steroids c- cyclosporin

33- Patient with ankylosing spondylitis may present with all of the following except

- a- chronic hepatitis b- urethral discharge c- sacroiliac

34- Extra- articular manifestaion of ankylosing spondylitis invlude all except

- a- iritis b- AR
c- U. colitis d -anemia

35- With ankylosing spondylitis is associated by all except

- a- peripheral arthritis b AR
c- +ve RF d- pulmonary fibrosis

36- HLA B27 is commonly presentation

- a- with ankylosing spondylitis b- RA c- gout

37- Test confirm ankylosing spondylosis

- a- Shouber's test b- Schimmer's test c- Pathergy's test

38- Radiological finding of spine ic As include

- a- squaring of spine b- syndesmophyte
c- Bambo spine d- all of above

39- Joint involvement in SLE

- a- erosive b- non- erosive c- both

40- Skin manifestation of SLE include one of the following

- a- malar rash of skin b- livedo reticularis
c- photosensetivity d- discoid rash
e- all of the above

41- Anti-Ds- DNA help in diagnosis of

- a- gout b- RA
c- SLE d- psoriatic arthritis

42- The single most useful laboratory test for diagnosis of SLE is

- a- LE cell b- fluorescence
c- anti-DS DNA d- ANA

43- ANA is present in all following except

- a- rheumatoid fever b - SLE
c- polymyositis d- systemic sclerosis

44- The following drugs induce SLE except

- a- NSAID exposure b- oral contraceptive
c- phynetoin d- phenohiazine

45- Drugs induce SLE diagnosed by

- a- +ve ANA b- +ve anti DNA
c- +ve decrease c3 & c4 d- +ve anti-histone

46- Drugs used in management on acute gout include

- a- NSAID
- b- allopurinol
- c- salicyate
- d- propencid

47- TTT on acute gout arthritis is

- a- colchicine
- b- allopurinol
- c- paracetamole

48- The mangment of OA include all the following except

- a- weight reduction
- b- Methotrexate
- c-physiotherapy
- c- NSAID

49- Theraputic useful in the TTT of osteoprosis all except

- a- regular exercscise
- b- Bisphosphante
- c- VIT D & C
- d- glucocorticoids

50- Which is the best for diagnosis of low back pain

- a- history of clinical examination
- b- MRI
- v- plain X ray
- d- CT spine

Cases

1- 55 Years old women presented by Fever & dysphagia & raynauds disease with digital infarcts

- a. *What is your diagnosis?*
- b. *What is your investigation & TTT?*

2- 35 y. old male presented with recurrent oro-gintal ulcerations. Eye examination revealed acute uveitis

- a. *What is your diagnosis?*
- b. *C/P & management*

3- A 6 y. old girl complained of pain in her left shoulder & right knee for 10 days, she was febrile. She presented to you a laboratory result of ESR of 100 mm.

- a. *What particular points in the history should you enquire about? And what should you specifically look for in the physical examination?*
- b. *Select the most important 4 additional laboratory tests that should help you in follow up*
- c. *Comment on the immediate and remote prognosis?*

4- A 20 y. old girl presented with ankle edema & puffiness of the eyelids, fever 37.8°C & joint pain. Urine analysis showed a protein urea of 5.5 gm/lit. She gave the history of an erythematic rash on the cheeks & nose

- a. *What is your provisional diagnosis?*
- b. *State the appropriate investigations, and their expected results?*

The condition of the pt. markedly improved by proper treatment, and the dose of the main drug was reduced gradually to a maintenance smaller dose. 2 years later the pt. developed severe stitching pain in the right side of the chest examination revealed dullness on the right side of the chest Rub heard at its upper level.

- a. *Give an explanation for this data*
- b. *What are the investigations needed now?*

Six years later the patient was admitted to a hospital in a semi-comatosed state with marked hunger and blood pressure of 180/120 mmHg. Her relatives stated that she suffered from marked anorexia, vomiting and general weakness during the last month.

- c. *What is your diagnosis in this stage?*
- d. *What are the urgent investigations to be done?*
- e. *Outline the necessary treatment at this stage?*

5- A 40 y. old woman suffered from arthralgia affecting the small joints for the last 15 years received various drugs for treatment of her disease. Last month she developed generalized edema and heavy proteinuria.

- a. *Discuss the possible diagnostic diagnosis*
- b. *What investigations are required to reach the diagnosis?*
- c. *Describe the management according to the diagnosis*

6- A 22 y. old girl presented with fever and joint pains since one month. Examination revealed T. 38°C, pulse 110/mm, malar flush, and silky light hair. Her skin shows reddish skin mottling. The knee, ankles and wrists are swollen and tender. Urine examination revealed heavy albuminuria.

7- A 30 y. old unmarried male presented complaining from joint pains in both knee & fingers since three weeks, together with some dysuria during micturition. Examination revealed that his temp. 37.3°C, he has congested eye, a mild macular skin rash, & the sacroiliac joints are tender. The WBC count is 11000/cmm and the sedimentation rate is 85/mm in the first hour.

8- A 35 y. old lady complained of morning stiffness of the small joints of the hand and feet since 10 years. Recently she developed fever & bilateral symmetrical swelling of both wrist and ankle joints.

- a. *What are your probable diagnosis and your differential diagnosis?*
- b. *What investigation would you order to reach a final diagnosis?*
- c. *What is your proposed treatment?*

- 9- A 22 y. old girl presented with fever & joint pains since 1 month, later on she developed lt. side chest pain followed by progressive dyspnea. O/E, temp. was 38°C, pulse 100/min, tachypnea & malar rash. The knee, ankles & wrists were swollen, tender & there was basal dullness on the lt. side of the chest. Urine examination revealed heavy proteinuria, RBCs & red cell casts.
- What is the most likely diagnosis?*
 - How would you investigate this case?*
 - What are the most important lines of therapy?*
- 10- A 55 y. old lady was referred to a medical clinic complaining of breathlessness on exertion with pain & colour change of her hands in cold weather for the last years. During the last year she started to complain of difficulty in swallowing. O/E, there was bluish discolouration & tapering of the finger with tight skin. Tachypnea & basal chest crepitations.
- What is the diagnosis of the color change in this lady's hand & Mention other causes of these phenomena?*
 - How do you account for the chest symptoms & signs?*
 - Why does this patient complain of dysphagia?*
- 11- A 22 y. old girl presented with fever and joint pains since one month. Later she developed left sided chest pain followed by progressive dyspnea. Clinical examination revealed temp. 38°C, pulse 110/min, malar flush, reddish skin mottling, in addition to swelling and pain in the knee, ankles and wrists. Urine examination revealed heavy albuminuria
- 12- A 38 u. old lady consulted her doctor for the past three years with painful joints. These joints were knee, shoulders & wrist and later on the fingers & temporomandibular joints were affected. In the morning her doctor had prescribed NSAIDs for her with some benefit. Prior to admission she developed a fever & a sharp pain in the left axillary region.
- What are the possible causes for her polyarthritiis?*
 - How do you account for the last complication?*
 - What investigations are indicated?*
- 13- A 22 y. old female presented with fever and joint pains of one month duration. For the last two days, she developed left sided chest pain followed by progressive dyspnea. Clinical examination revealed that her temp. was 38°C and her pulse was 110/min. She was tachypnic. There was malar flush & was reddish skin mottling. Knees, ankles & wrists were swollen and tender. There was basal dullness on the left side of the chest. Urine examination revealed heavy proteinuria.
- What is the most likely diagnosis?*
 - What is your differential diagnosis?*
 - What further investigations would you like to do this patient?*

14- A 42 y. old male, smoker complained of sudden pain of his Rt. ankle that started at night one day ago. He gave no history of trauma. He gave history of hypertension for which he was receiving ttt. He also gave history of repeated renal colic followed by passage of renal stone. Last stone that he passed was 2 years ago. O/E, BP was 170/110, temp. was 37.5°C and his pulse was 100/min regular of good volume. His Rt. ankle was swollen and red, hot & markedly tender. Blood picture revealed hb of 12.7 gm/dl, WBCs count was 13000/mm³, and ESR was 35 mm/hour.

- a. What is your provisional diagnosis?*
- b. What is your differential diagnosis?*
- c. What are investigations would you like to confirm your diagnosis?*

15- A 35 y. old female complained of stitching pain in the right side of the chest with hemoptysis for 2 days. She gave a past history of systemic lupus erythematosus for the last 10 years. She aborted twice last 2 years. Examination revealed swelling of the right leg and foot. Temp. was 37.7°C. Chest examination revealed the presence of right pleural rub.

- a. In your opinion, what is the cause of the chest symptoms?*
 - b. What is your D.D. of her chest condition?*
 - c. What is the relation of SLE to her recent events?*
 - d. What further investigation would you suggest to this patient?*
-

Oral

- 1- DD of polyarthrititis**
 - 2- Variants of scleroderma**
 - 3- Anti-phospholipid syndrome**
 - 4- SLE (c/p – investigation)**
 - 5- Gout (diagnosis – TTT)**
 - 6- Extra-articular manifestation of (RA & ankylosing spondylosis)**
 - 7- DD of low back pain**
 - 8- Compare mechanical and inflammatory disease**
-

Answer

MCQ

1- B	2- D	3- A	4- A	5- C	6- C	7- B	8- B	9- A
10- D	11- D	12- C	13- C	14- D	15- D	16- A	17- A	18- C
19- B	20- B	21- B	22- C	23- C	24- A	25- D	26- C	27- B
28- D	29- B	30- A	31- B	32- A	33- A	34- D	35- C	36- A
37- A	38- D	39- B	40- E	41- C	42- C	43- C	44- A	45- D
46- A	47- A	48- B	49- D	50- A				

Cases

- 1) Systemic sclerosis
- 2) Behcet's disease
- 3) Arthropathy in children
- 4) SLE complicated by lupus nephritis and presented by nephritic syndrome... then polyserositis and finally renal failure
Causes of arthritis & proteinuria e.g. lupus nephritis or rheumatoid arthritis
Connective tissue diseases, most probably SLE
- 5) Seronegative spondyloarthritis, mostly Reiter's syndrome
- 6) Rheumatoid arthritis
- 7) SLE complicated by lupus nephritis & lt. sided pleural effusion
- 8) Raynaud's phenomena
- 9) A case of connective tissue diseases, most probably SLE (systemic lupus erythematosus)
- 10) Rheumatoid arthritis or SLE
- 11) SLE complicated by lupus nephritis & left sided pleural effusion
- 12) GOUT
- 13) Most probably a case of ant phospholipid antibody syndrome

Keywords for rheumatology cases

- 1- Arthritis + morning stiffness = RA
- 2- Alopecia + malar rash + kidney affection = SLE
- 3- GIT disturbance + HTN = Scl crisis
- 4- Recurrent abortion = anti-phospholipid
- 5- Renal stone + pain big toe = gout
- 6- Postmenopausal female + low back pain = osteoporosis

Endocrine 1

(Thyroid & Parathyroid glands diso. – Obesity – Hirsutism – Osteoporosis & Osteomalacia)

- 1- Write on myxedema = hypothyroidism (causes – C/P - TTT)
 - 2- TTT of myxedematous coma
 - 3- Management of thyrotoxicosis
 - 4- Thyroid crisis
 - 5- Discuss clinical presentation of hyperthyroidism (diagn. & TTT)
 - 6- Graves' disease (c/p – management)
 - 7- Grave's' ophthalmopathy
 - 8- Hyper para thyroidism(c/p – invest- diagnosis)
 - 9- Hypercalcemia (causes & TTT)
 - 10-Hypoparathyroidism (investigation)
 - 11- Gynecomastia (causes & investigation)
 - 12-Causes & management of Tetany (hypocalcemia)
 - 13- Discuss TTT of osteoporosis
-

MCQ

- 1- **The following is not a complication of obesity**
a- osteoarthritis
b- renal calculi
c- hypertension
d- varicose vein of leg
- 2- **The common presentation of hashimoto thyroiditis is**
a- hyperthyroidism
b- hypothyroidism
c- euthyroid state with goiter
- 3- **In hashimoto thyroiditis there is**
a- hyperthyroidism
b hypothyroidism
c- both
- 4- **Hypersensitivity type 5 occur in**
a- SLE
b- Graves' disease
c- anaphylatics
- 5- **Graves' disease is an example of**
a- type 1 hypersensitivity
b- type 2 (cytotoxic)
c- type 4 (MI)
d- type 5 (stimulatory)
- 6- **Thyrotoxicosis may be presented by all the following except**
a- myopathy
b- pretibial myxedema
c- hypernatremia
d- atrial fibrillation

a- puffy face b- palpitation
c- sweating d- somnolence

a- pretibial myxedema b- dry skin c- atrophic thyroid gland

a-diarrhea b- scaly skin c- lethargy

a- heat intolerance b- bradycardia c- tremors

a-bradycardia b- tachycardia c- neither

a- T3
c- TSH

b- T4
d- reverses T3

a- tachycardia b- hypothermia c- sweating

a- right sided heart failure
b- nephrotic syndrome
c-liver cell failure
d- myxedema

a- hypertension
b- tachycardia
c- hypoventilation
d- normal body temperature

a- hypocalcemia b-hypercalcemia c-hypophosphatemia

a-rickets	b-CRF
c- mal absorption	d- pseudo hyperparathyroidism

a- hyperparathyroidism b- hypothyroidism c- CRF

a- IV saline
b- Loop diuretics
c- calcitonin
d- all of the above

- a- increase activity of osteoclast
- b- increase osteoclastic bone resorption
- c- increase activity of vit D
- d-increase calcitonin production

a- IV saline
b- loop diuretic
c- calcitonin
d- all of the above

22- Hypocalcemia is characterized by the following except

- a- parathesia
- b- tetany
- c- lenticular cataract
- d- shorting of Q-t interval in the ECG

23- In patient with pseudo hypoparathyroidism serum PTH is

- a- normal
- b- increase
- c- decrease

24- Most common type of carcinoma of thyroid gland is

- a- follicular
- b- papillary
- c- anaplastic
- d- mixed papillary and follicular

Cases

1- Female patient 28 y. old presented with tremors, excessive sweating insomnia, tachycardia & wt loss inspite of good feeding

- a. *What is your diagnosis?*
- b. *What are the investigations?*
- c. *TTT you suggest?*

2- A female patient 45 y. old presented with constipation, wt gain skin & hair changes & bradycardia.

- a. *What is the diagnosis?*
- b. *What are investigations?*
- c. *Treatment you suggest*

3- A 38 y. old lady noticed that she is losing weight and suffers from anxiety & insomnia at night. Examination revealed a temp. 36.3°C pulse 135/mm, and she has warm tremoring hands. ESR is normal.

- a. *What is your diagnosis?*
- b. *What are the investigations?*
- c. *TTT you suggest?*

4- A 45 y. old lady presented to the medical clinic complaining from an increasing sense of general fatigue and intolerance to colds since three months. In the last two weeks her voice became increasingly hoarse. Examination revealed that her temp. is 36.9°C, pulse 60/min, BP 140/80, her skin is dry. Neurological examination was negative but the knee reflexes were slow in their response.

- a. *What is the diagnosis?*
- b. *What are investigations?*
- c. *Treatment you suggest*

- 5- A woman 44 y. old complaining of increasing weakness and constipation over the last 3 months. O/E, she was pale with puffy eyelids. Her pulse was 60/min, regular. Her serum TSH 201 U/ml (N. 1-6)
- What is the possible diagnosis?*
 - How can you explain the clinical and laboratory findings?*
 - What is the treatment and how can you follow up her condition?*
- 6- A 35 y. old woman presented with 6 months history of diarrhea, weight loss and palpitation. She was seen by psychiatrist for feeling anxious & irritable and was given a tranquilizer without good response.
- What is the most important disease you have to exclude?*
 - Mention important clinical signs that can help you in its diagnosis?*
 - Mention important blood tests help you in its diagnosis?*
 - If the diagnosis proved to be correct how to manage?*
- 7- A 45 y. old female patient complained of regular palpitation that used to increase by exertion, but sometimes also occur during rest, she reported that she had marked weight loss in spite of her reasonable appetite.
- What is your provisional diagnosis?*
 - What is your D.D.?*
 - What laboratory tests you need to reach a definitive diagnosis?*
- 8- A 55 y. old female presented with coma. Examination revealed puffy face and mild lower limb edema. Her temperature was 35°C. her pulse was 50 beats/mm & regular. Her relatives stated that she was taking pills regularly for long time and she stopped the ttt. The last 4 weeks, she also noticed that her speech was clumsy and slows in the last weeks.
- What is your diagnosis?*
 - What is your differential diagnosis?*
 - What are you doing to confirm your diagnosis?*
 - What ttt should be given?*
- 9- A 60 y. old female complained of tiredness, constipation, weight gain and tingling in her hands and feet. O/E, she was obese, with pale puffy face. Investigations revealed mild normocytic normochromic anemia and hypercholesterolemia, chest X-ray revealed cardiomegaly.
- What is your diagnosis?*
 - How would you explain the chest X-ray findings?*
 - What is the possible cause for the tingling in her hands?*
 - How would you treat this lady?*

10-A 22 y. old female student suffered from IDDM for the last 7 years. She started a few weeks ago to complain of tachycardia, sweating & nervousness

- a. Enumerate possible causes of her complains
- b. Discuss diagnosis & management of one of them

11- A 72 y. old lady complaining of fatigue has become increasingly lethargic. She also said that her hair is thinning. Her pulse rate is 52/min, regular and BP is 138/90 mmHg.

Examination of her heart chest and abdomen is normal.

- a. What is your diagnosis (explain why)?
- b. How do you investigate this case?
- c. How would you manage the case?

Oral

- 1- Diagnosis of obesity
- 2- Hirsutism
- 3- Tetany
- 4- Myxedematous coma
- 5- Thyrotoxic crisis

Answer

MCQ

			1- B	2- C	3- C	4- B
5- D	6- C	7- B/C	8- A	9- A	10-B	11-A
12-C	13-B	14-D	15-C	16-B	17-B/C	18-A
19-D	20-B	21-D	22-D	23-B	24- B	

Cases

- 1) Thyrotoxicosis
- 2) Myxedema
- 3) Thyrotoxicosis due to Graves'
- 4) Myxedema
- 5) Adult myxedema
- 6) Thyrotoxicosis
- 7) Thyrotoxicosis
- 8) Myxedema coma
- 9) Myxedema
- 10) Thyrotoxicosis either graves or thyroiditis (autoimmune disease) recurrent attacks of hypoglycemia
- 11) Hypothyroidism

Endocrine 2

(Suprarenal & Pituitary – Gland disorders)

- 1- Write on dwarfism on endocrine origin
- 2- Enumerate causes and stunted growth with short note on each
- 3- Describe C/P of acromegaly
- 4- Give an account on neurological manifestation of acromegaly
- 5- (Diagnosis – causes – management) of Cushing syndrome
- 6- Discuss indication and side effect of cortisone
- 7- Addison disease (causes – C/P - management)
- 8- Enumerate syndromes of hyper function of adrenal cortex & discuss one of them
- 9- (Diagnosis – management) of pheochromocytoma
- 10- (C/P – diagnosis) of acute adrenal insufficiency
- 11- disease caused by hyper function of adenohypophysis (C/P- investigation)
- 12- ADH disorders
- 13- DI (causes and C/P)
- 14- C/P of anterior pituitary tumors
- 15- C/P of hyper function of supra renal cortex

MCQ

- Acromegaly is due to a disease in**
 - Ant. Pituitary
 - post. Pituitary
 - neither
- Acromegaly is characterized by all except**
 - big head and feet
 - prognathism
 - moon face
 - separate teeth
- Recognized complications of acromegaly**
 - exophthalmos
 - DM
 - pathological fracture
 - decreased sweating
- In investigations of acromegaly all are true except**
 - failure to suppress GH < 2mg/ml after glucose
 - ↑ IGF 1
 - MRI sellaturica reveals pituitary adenoma
 - ↓ serum prolactin
- All of the following are causes of hyperprolactinaemia except**
 - pregnancy
 - primary hypothyroidism
 - pheochromocytoma
 - prolactinomas

6- Intra-sellar pituitary tumor cause

- a- visual defect
- b- neurological defect
- c- hormonal changes
- d- all of the above

7- Recognized causes of hypopituitarism include all of the following except

- a- postpartum hemorrhage
- b- Cushing's syndrome
- c- acromegaly
- d- auto-immune hypophysitis
- e- sarcoidosis

8- Causes of hypopituitarism all are true except

- a- craniopharyngioma
- b- head injury
- c- Sheehan's syndrome
- d- Caplan's syndrome
- e- none of the above

9- Pan hypopituitarism causes

- a- galactorrhea
- b- skin pigmentation
- c- amenorrhea

10- Causes of DI include all of the following except

- a- craniopharyngioma
- b- sarcoidosis
- c- severe hypocalcemia
- d- severe ↑ in Ca & ↓ in K

11- Causes of nephrotic DI include all of the following except

- a- lithium therapy
- b- heavy metal poisoning
- c- demeclocycline
- d- chlorpromide

12- Manifestation of Cushing syndrome include all of the following except

- a- osteoporosis
- b- hypoglycemia
- c- proximal myopathy
- d- HTN

13- All typical features of central DI except

- a- specific gravity less than 1005 with decreases urine osmolality & ↑ plasma osmolality
- b- onset following bacterial meningitis & hypothalamic trauma
- c- decrease renal responsiveness to ADH

14- In Cushing syndrome all are true except

- a- hypoglycemia
- b- polycythemia
- c- osteoporosis
- d- moon face

15- Hypoglycemia may occur in the following disease except

- a- Sheehan's syndrome
- b- Cushing's syndrome
- c- chlorpropamide therapy

16- Urinary 17 keto-steroids are ↑ in

- a- Cushing's syndrome
- b- Conn's syndrome
- c- adrenagenital syndrome

17- Commonest enzymatic defect for development of CAH

- a- 21 hydroxylase
- b- 11 hydroxylase
- c- 17 hydroxylase
- d- 3β dehydrogenase

18- Auto-immune adrenalitis syndrome commonly presented with

- a- Addison's disease
- b- Cushing syndrome
- c- both

19- Hypo-natremia may occur in

- a- Addison's disease b- Cushing's disease c- Conn's syndrome

20- The following are common features of Addison's disease except

- a- easy fatigability & weakness b- fever
c- skin pigmentation d- hypotension

21- Important signs of Addison disease

- a- hypoglycemia b- weight gain c- peripheral neuritis

22- Adrenal crisis is characterized by

- a- profound asthma b- severe abdominal pain
c- vascular collapse d- low Na & high K
e- all of the above

23- Pheocromocytoma may be associated

- a- anhidrosis b- miosis
c- neurofibromatosis d- paroxysmal hypotension

24- Cortico steroid side effects

- a- hyper-prolactinemia b- hyperglycemia c- hypercalcemia

25- Features of Addison's disease don't include

- a- diarrhea b- dizziness
c- dermatitis d- dehydration

Cases

1. Middle age male patient presented with chronic fatigue & hyperpigmentation on examination his BP was 95/75 mmhg on supine and decreased on standing

- a) *What is your diagnosis?*
b) *What are the investigations?*
c) *TTT you suggest?*

2. 35 years old female suffer from paroxysmal HTN and attack of pallor and sweating , headache , anxiety , palpitation , abdominal pain and vomiting constipation , wt loss and intolerance also evident

- a) *What is your diagnosis?*
b) *What are the investigations?*
c) *TTT you suggest?*

3. Male patient 65 years old recently suffered from trunk obesity, DM, HTN & myopathy and presented with sudden back pain with history trauma.

- a) *DD of this case?*
b) *What are the investigations?*
c) *TTT you suggest?*

4. Female patient complaining of amenorrhea for 2 years following the last difficult labor complicated by severe postpartum hemorrhage, from that time she was told that she is hypotensive. She has a well-marked asthenia & can't tolerate cold weather
- a) *What are other symptoms to be asked for?*
 - b) *What is the diagnosis?*
 - c) *What are the necessary investigations to prove the diagnosis?*
 - d) *What are the main lines of treatment?*
 - e) *What is the prognosis?*
5. A male patient aged 35 years presented with asthenia. He has been under therapy for his low blood pressure for sometimes. His dermatologist had along run of therapy for discoloration on his trunk & for the deep color of skin.
- a) *What do you expect to find in his history and on full clinical examination?*
 - b) *What are the steps needed for the diagnosis?*
 - c) *What do you suggest for treatment?*
6. A female patient 40 years complains from exhaustion & asthenia in the last year, with loss of weight. She noticed that her tongue and cheeks are becoming mottled with dark areas. Examination revealed that BP 90/60, she is underweight, dark pigmented area on cheeks and tongue was confirmed.
7. A 25 years male presented with marked polydipsia since 2 weeks that made him markedly exhausted. There is no family history of diabetes. Examination revealed generalized enlargement of lymph glands, there is erythema nodosum on his legs. Chest X-ray revealed broad superior mediastinum. And sedimentation rate is 40mm/hr. Urine examination revealed pale urine specific gravity 1006 and free from sugar.
8. A 64 years old man noticed that he had gained 8 kg in weight over the past 6 months. He started to get easily bruising, he found difficulty in getting up from his arm chair or to climb stairs, he felt depressed. On examination he is overweight particularly on the abdominal region with purpurish stria on his abdomen and thighs. His blood pressure is 168/100, examination of his heart respiratory system and abdomen is normal. There is some weakness in shoulder and hip regions. His routine examinations are within normal apart from elevated 2 hours post prandial blood sugar.
- a) *What is the provisional diagnosis?*
 - b) *How would you investigate and manage this patient?*

9. A 25 y. old white male consulted his physician for recurrent headache & shortness of breath. The patient was obese and his abdomen was fatty. His BP was 170/100. His face was flushed and greasy, there was some acne on his chest wall. Urine was positive for glucose (++)

- a) *What is your diagnosis?*
- b) *What is your differential diagnosis?*
- c) *What other tests would you like to ask to reach a definitive diagnosis?*

Oral

- 1- DD of short stature
- 2- Addison crisis
- 3- Conn's disease
- 4- Side effects of corticosteroid drugs
- 5- Action of growth hormone

Answer

MCQ

1- A	2- C	3- B	4- D	5- C	6- D	7- C/B
8- D	9- C	10- C	11- D	12- B	13- C	14- A
15- B	16- A	17- A	18- A	19- A	20- B	21- A
22- E	23- C	24- B	25- C			

Cases

- 1) Addison's disease
- 2) Pheochromocytoma
- 3) Cushing syndrome – Cushing disease – cushinoid – ectopic cushion
- 4) Sheehan syndrome
- 5) Addison's disease
- 6) Addison's disease
- 7) Diabetic incipidus d.t. sarcoidosis
- 8) Cushing syndrome
- 9) Cushing syndrome

Endocrine 3

(DM)

- 1- (Management – C/P – causes) of DKA
- 2- Oral hypoglycemic drugs
- 3- TTT of uncomplicated DM
- 4- DD of diabetic coma
- 5- Account on hypoglycemia
- 6- Hyperglycemic coma (PPT factor/ TTT)
- 7- Complication of insulin therapy
- 8- DM (classification and pathogenesis)
- 9- Type 1 DM (pathogenesis & TTT)
- 10- DM (chronic complication)
- 11- DM (neurological complication)
- 12- DKA
- 13- Recurrent hypoglycemia DD
- 14- Type 2 DM (management)
- 15- DD acute complication and discuss one of them
- 16- Antidiabetic drugs

MCQ

- 1- **Secondary DM may be caused by the following except**
a- chronic pancreatitis b- insulinoma c- glucoagonoma
- 2- **Immune mediated DM is present in**
a- type 1 b- type 2 c- both
- 3- **The best TTT of type 1 DM is**
a- oral hypoglycemic drugs b- insulin c- low caloric intake
- 4- **Feature of hypoglycemic don't include**
a- drenching sweating b- tachycardia
c- brisk jerk (tremors) d- tachypnea
- 5- **In type 2 DM is associated with**
a- insulin resistance b- cell defect c- Both

- 6- In type 2 DM insulin resistance is increase by**
 a-exercise b- obesity c- metformin
- 7- The initial TTT of obese Type 2 DM is**
 a-sulfonylurea b- biguanide c- wt reduction
- 8- In DM there is polyurea with**
 a- high specific gravity b- low specific gravity c- Both
- 9- Glycosylated HB assess glucose level during**
 a- last 3 months b-past week
 c- this day d- none of the above
- 10- Diabetic keto-acidosis is charactrized by**
 a- rapid respiration b- dehydration
 c- rapid small pulse d- all of the above
- 11- Diabetic ketoacidosis is treated by**
 a- IV fluid b- K c- both
- 12-Which type of hypoglycemic would on insulinoma case**
 a- reactive b- drug induced
 c- alimentary d- fasting
- 13- Which hormone is responsible for many of symptoms of hypoglycemia**
 a- TSH b- epinephrine
 c- cortisol d- thyroxin
- 14- Hypoglycemia may result of the following except**
 a- glycogen storage disease b- galactosemia
 c- chronic pancreatitis d- post gastrectomy

Cases

- 1- 15 y. old boy known history of IDDM presented to ER with coma**
a. DD of causes?
b. What are the investigations?
c. Management on of the above?
- 2- 35 y. old obese female complain of Wt loss inspite of increase appetite pruritus in her vulva and frequent waking up for urination at night**
a. What is the diagnosis?
b. What are the investigations?
c. Complication of the case?
- 3- A 16 y. old boy, with known insulin dependent D.M., was admitted to hospital in a lethargic drowsy condition, with history of vomiting, polyuria & polydespia 3 days before. He was taking soluble insulin 10 units each morning. His temp. was 37.6 °C, pulse was 140/min, BP 100/60 mmHg, heart, lung & abdomen were clinical normal**
a. What is your diagnosis?
b. What are the possible precipitating factors?
c. What is your immediate management?

- 4- A 12 y. old underweight boy had fever for 2 days. Examination revealed a very tender swelling in his right buttock that was diagnosed as a gluteus abscess. Next day the patient was very confused & his mouth was dry, had also acidotic breathing.
- What is the likely diagnosis for the confusion?*
 - What is your differential diagnosis?*
 - What further tests do you like to perform to this patient to reach a diagnosis?*
- 5- A 60 y. old male presented to ER with disturbed level of consciousness of 1 day duration. His relatives reported that he had weight loss over the past few weeks despite of good appetite. They also gave history of polyuria for the last 2-3 weeks. O/E, the pt. was drowsy and dehydrate with sunken eyes. His BP was 140/65 mmHg, temp. was 37°C & tests revealed a Hb of 16 gm/dl, a hematocrit of 50, WBCs 11.000/mm³ & platelets 380.000/mm³. Serum creat. Was 1.8 mg/dl & BUN was 46 mg/dl. Random blood sugar was 684 mg/dl. Arterial blood gases (ABG) revealed normal serum bicarbonate.
- What is your provisional diagnosis?*
 - What investigation would you like to do to confirm your diagnosis?*
 - What is your suggested line of ttt?*
- 6- A 22 y. old student presented with abdominal pain, repeated vomiting and marked weakness. Over the last few weeks she noticed polydipsia and polyuria. Two days ago she complained of fever, bilateral loin pain and dysuria. O/E, she was dehydrated, tachypneic and feverish. Random blood sugar was 450 mg/dl. Abdominal examination revealed tender both loins.
- What is your probable diagnosis?*
 - What would you investigate such case?*
 - What is your suggested treatment?*
-

Oral

- 1- Management of DKA
- 2- DD of hypoglycemia
- 3- DD of diabetic coma
- 4- Oral hypoglycemic drugs

Answer

MCQ

1- B	2- A	3- B	4- D	5- C
6- B	7- C	8- A	9- A	10- D
11- C	12- D	13- B	14- C	

Cases

- 1) DD of diabetic coma
- 2) type 2 DM
- 3) diabetic keto acidosis
- 4) IDDM complicated by Gluteal abscess leading to DKA
- 5) Diabetic ketoacidosis
- 6) Most probably a case of IDDM complicated by acute pyelonephritis

Key words for endocrine

- 1- Decrease K = Cushing or conns
- 2- Increase K = Addison disease
- 3- Ca > parathyroid
- 4- Increase body Wt = myxedema
- 5- Decrease WT = thyrotoxicosis
- 6- Weight gain + bradycardia + distant heart sound = myxedema
- 7- Young age + HTN+ association = pheochromocytoma
- 8- Post-partum Hge and can't lactate her baby + lack of hair growth = Sheehan syndrome
- 9- Polyuria + polydipsia then abdominal pain , irritation , dehydration & vomiting = DKA

#ولأن التفوق أجمل سأجتهد
مررت بكثير من المعاناة.. وتعلمت الكثير .. لم يتبق إلا القليل على الراحة
قليل من الاجتهاد وستجنون ثمار التعب .. وستفرون
الفرصة قريبة .. اجتهدوا .. ووعواتنا خلفكم .. ووعوات صاوقة من قلوب محبة
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